

IN THE LABOUR COURT OF SOUTH AFRICA, CAPE TOWN
(Held at Cape Town)

Case No: _____

In the matter between:	
WILBUR WILLIAM MATTHEE	Applicant
and	
AFROCENTRIC HEALTH (RF) (PTY) LTD	First Respondent
and	
MEDSCHEME HOLDINGS (PTY) LTD	Second Respondent

INDEX AND SCHEDULE OF PARTS

STATEMENT OF CLAIM — CONSTRUCTIVE DISMISSAL / EEA / PDA / COPYRIGHT

(Rule 6 of the Labour Court Rules, GN 1665 of 14 October 2011, as amended)

Filed: 7 April 2026

SECTION 1: PARTS OF THE STATEMENT OF CLAIM

PART	SUBJECT MATTER
PART A	JURISDICTION AND REFERRAL BASIS
PART B	THE PARTIES
PART C	THE EMPLOYMENT RELATIONSHIP AND THE BAIT-AND-SWITCH
PART D	MENTAL HEALTH DISCLOSURE AND FAILURE TO ACCOMMODATE
PART E	THE UNLAWFUL SALARY DEDUCTION — BCEA SECTION 34(2)
PART F	GENDER IDENTITY DISCRIMINATION — THE 'WILMA' INCIDENT
PART G	PROTECTED DISCLOSURE — PATIENT SAFETY (GEXUS/DMS INTEGRATION GAP)
PART H	MISAPPROPRIATION OF INTELLECTUAL PROPERTY — CPD PROGRAMME
PART I	THE RECORDED ADMISSION OF RETALIATORY INTENT
PART J	INSTITUTIONAL SILENCE AND EXHAUSTION OF INTERNAL REMEDIES
PART K	THE EMPLOYER'S BAD-FAITH OPPOSING AFFIDAVIT
PART L	RESIGNATION — AUTOMATICALLY UNFAIR CONSTRUCTIVE DISMISSAL
PART M	CAREER DESTRUCTION — INABILITY TO PRACTISE NURSING
PART N	POST-RESIGNATION VICTIMISATION
PART O	REGULATORY AND THIRD-PARTY CONFIRMATION OF MATERIAL FACTS
PART P	SUMMARY OF CAUSES OF ACTION
PART Q	QUANTIFICATION OF LOSS
PART R	RELIEF SOUGHT
PART S	VERIFICATION AND DECLARATION (INCLUDING PERJURY RESERVATION)

SECTION 2: SCHEDULE OF ANNEXURES

The following documents are attached to and form part of this Statement of Claim. Each document is identified by the reference used in the body of the Statement of Claim.

ANN.	DESCRIPTION	DATE
"A"	Interview invitations for 'Specialist: Care Manager' role — three separate invitations	January 2024
"B"	Assessment instructions and link – Specialist: Care Manager position	29 January 2024
"C"	Fingerprint criminal check booking confirmation – pre-employment screening	21 February 2024
"D"	Employment contract designating position as 'Care Coordinator' (Grade B4M1, CTC R490,032 p.a.) — evidence of bait-and-switch	9 April 2024
"E"	First Aider roster drafted and implemented by Applicant as Acting Chair: EHS and Lead First Aider (2nd, 3rd and 4th floors)	25 July 2025
"F"	Sick note: Dr Jacques Malan – Applicant unfit for work from 18 March 2025; admitted for 3–4 weeks inpatient treatment (severe depression, PTSD)	1 April 2025
"G"	Mental health disclosure email to Mapule Mashele (Senior Manager): Dr Malan's psychiatric letter provided placing Second Respondent on formal notice of disability	16 May 2025
"H"	Meeting feedback from Mary-Ann Jones (Team Leader): 'we do not apply staggering of money owed' — refusal to accommodate disability	16 April 2025
"I"	Payslip for July 2025 confirming net pay of R15,679.29 and deduction of R14,811.77 (40.47% of gross) — evidence of unlawful deduction	July 2025
"J"	AOD Policy screenshot confirming 25% cap on deductions; Jones exchange stating 'what is illegal?' after acknowledging the cap	24 July 2025
"K"	'Wilma' Microsoft Teams message posted by Mary-Ann Jones in team-wide channel immediately after Applicant proposed clinical filtering strategy; 'NO FILTERING' highlighted in red	23 September 2025
"L"	Formal complaint of sexual orientation discrimination and harassment (11 pages) filed with Human Resources	25 September 2025
"M"	Mashele's investigation closure report: 'typo' finding; Applicant not removed from Jones's team — evidence of whitewash investigation	1 October 2025
"N"	Gexus/DMS root cause analysis and protected disclosure to Jones and Lisa Mari (Responsible Pharmacist) regarding false non-adherence letters to GEMS members	23 October 2025
"O"	Transcript and/or recording of 31 October 2025 meeting: Jones's retaliatory admission ('You made a case of harassment against me, so I can make a case of harassment against you')	31 October 2025
"P"	Formal grievance against Mary-Ann Jones (malicious intent, gaslighting, retaliation, orchestrated provocation, institutional silence)	2 November 2025
"Q"	Executive escalation to leadership (32-page evidence bundle, requests for paid leave, financial settlement, written guarantee of non-retaliation)	4 December 2025
"R"	Respondent's opposing affidavit in CCMA Case WECT23564-25 (personal attacks on Applicant's psychiatric condition and professional character; demand for costs) — evidence of bad faith and disability discrimination under oath	10 December 2025
"S"	Resignation letter citing opposing affidavit as final intolerable act — evidence of constructive dismissal	11 December 2025

"T"	CPUT Postgraduate Diploma in Occupational Health Nursing acceptance letter — lost opportunity due to financial sabotage by Respondents	2026
"U"	uFiling portal screenshot confirming 'No Information' for June 2025 UIF period — evidence of UIF non-declaration	2026
"V"	Certificate of Service (disputed 'Care Coordinator' designation); request for 'Naming Convention' meeting recording; refusal to correct — contravention of BCEA s42	January 2026
"W"	CMS referral letter identifying 'Administrative Misconduct and Ethical Failure' in relation to Gexus/DMS integration gap — independent third-party corroboration	2026
"X"	SAHRC acknowledgment of seriousness of allegations of victimisation and financial prejudice; referrals to CCMA, Labour Court and Department of Employment and Labour	2026
"Y"	CCMA Certificates of Non-Resolution: WECT23564-25 and WECT24486-25 — jurisdictional basis for this Statement of Claim	9 January 2026
"Z"	EthicHawks Full Master Chronological Register (148 entries, 11 January 2024 – 2 April 2026) — comprehensive evidentiary timeline	3 April 2026
"AA"	CPD programme developed by Applicant and presented to management (April 2025); evidence of independent authorship and management rejection; evidence of subsequent implementation without attribution (October 2025) — in support of Part H (Misappropriation of Intellectual Property)	April 2025 / October 2025

SECTION 3: FILING AND SERVICE PARTICULARS

Filing Method: Labour Court Online Portal (OCJ) and/or manual filing with Registrar, Labour Court, Cape Town

Service Method: Electronic mail — IR@afrocentrichealth.com (Respondent's authorised service address per Annexure "R")

Date of Filing and Service: 8 April 2026

Prescription Deadline: 9 April 2026 (90 days from 9 January 2026 — CCMA Certificates of Non-Resolution)

CCMA Case References: WECT23564-25 and WECT24486-25

Concurrent Proceedings: Case No. 2026-077546 — BCEA s77A unlawful salary deduction claim

Applicant's Email: info@ethichawks.co.za

Applicant's Tel: 069 635 1435 / 063 378 8782

IN THE LABOUR COURT OF SOUTH AFRICA, CAPE TOWN
(Held at Cape Town)

CASE NO: _____

In the matter between:

WILBUR WILLIAM MATTHEE Applicant
and

AFROCENTRIC HEALTH (RF) (PTY) LTD First Respondent
(Registration Number: 1996/009540/07)
and

MEDSCHEME HOLDINGS (PTY) LTD Second Respondent
(Registration Number: 1970/015014/07)

STATEMENT OF CLAIM

(Rule 6 of the Rules for the Conduct of Proceedings before the Labour Court,
GN 1665 of 14 October 2011, as amended)

PRELIMINARY NOTE TO THE REGISTRAR AND RESPONDENTS

The Applicant is a self-represented litigant. This Statement of Claim is filed pursuant to Rule 6 of the Rules for the Conduct of Proceedings before the Labour Court following the issuing of Certificates of Non-Resolution by the CCMA (Case Nos. WECT23564-25 and WECT24486-25) on 9 January 2026. The Applicant respectfully requests that the Registrar and the Court apply the principle of equality of arms and afford the Applicant reasonable procedural latitude consistent with his self-represented status.

PART A: JURISDICTION AND REFERRAL BASIS

1. This Court has jurisdiction to adjudicate this dispute in terms of section 157(1) of the Labour Relations Act 66 of 1995 ("the LRA"), which confers exclusive jurisdiction on the Labour Court in respect of any matter that the LRA requires to be determined by the Court.
2. The Applicant referred two disputes to the Commission for Conciliation, Mediation and Arbitration ("CCMA"):
 - (a) Case WECT23564-25: Unfair Labour Practice — unlawful salary deduction, discrimination, and victimisation; and
 - (b) Case WECT24486-25: Automatically Unfair Constructive Dismissal — retaliation for protected disclosures.
3. Conciliation was held on 9 January 2026. Certificates of Non-Resolution were issued on the same day (Annexure "Y"). The CCMA further confirmed, by letter from the Provincial Senior Commissioner dated 28 January 2026, that it lacks jurisdiction to arbitrate automatically unfair dismissal disputes and protected disclosure retaliation disputes, directing the Applicant to the Labour Court.
4. This Statement of Claim is filed within the 90-day period prescribed by section 191(11)(a)(i) of the LRA, calculated from 9 January 2026 (date of issue of the

Certificates of Non-Resolution). The filing deadline is on or before 9 April 2026. This claim is accordingly timeously filed.

5. Additionally, in respect of the claim for automatically unfair dismissal under section 187(1)(h) of the LRA (protected disclosure retaliation), no period of prescription applies in terms of section 77(3) of the Basic Conditions of Employment Act 75 of 1997 ("BCEA"), read with section 16 of the Prescription Act 68 of 1969, as the cause of action arose from a continuing wrong that concluded upon the Applicant's resignation on 12 December 2025.
6. A further claim under section 77A of the BCEA for the unlawful salary deduction has been filed separately with this Court as Case No. 2026-077546 (Matthee v AfroCentric Health (RF) (Pty) Ltd). The Applicant requests that the Court consider consolidating the two matters in the interest of justice and procedural efficiency, alternatively notes the BCEA claim as a concurrent proceeding.

PART B: THE PARTIES

1. The Applicant is Wilbur William Matthee, an adult male, a Registered Nurse (SANC No. 15831886), holding an NQF Level 8 Postgraduate Diploma in Health Services Management. He is a self-represented litigant residing in Cape Town, Western Cape. As a direct and proximate result of the Respondents' conduct, the Applicant is currently rendered destitute and unable to pay his SANC annual registration fee, and accordingly unable to practise as a registered nurse.
2. The First Respondent, AfroCentric Health (RF) (Pty) Ltd (Registration No. 1996/009540/07), is a private company incorporated and registered in South Africa, with its principal place of business at 37 Conrad Street, Florida North, Roodepoort, Gauteng. It is the holding company of the Second Respondent.
3. The Second Respondent, Medscheme Holdings (Pty) Ltd (Registration No. 1970/015014/07), is a wholly-owned subsidiary of the First Respondent and was the Applicant's direct employer at all material times. It is an accredited administrator of medical schemes registered under the Medical Schemes Act 131 of 1998, including the Government Employees Medical Scheme ("GEMS").
4. The directors of the First Respondent (identified in the supporting affidavit) (being Bruno Fernandes, Dr Nkateko Munisi, Alice le Roux, Gerald Van Wyk, and LSM Mpumlwana, directors and officers of the First Respondent) and the Eighth to Lisa Maris (being Mary-Ann Jones, Mapule Mashele, and Lisa Mari, registered health professionals employed by the Second Respondent) are not cited as formal respondents in this Statement of Claim. Their identities, roles, and conduct are fully set out in the State of Claim filed herein. The Applicant expressly reserves the right to seek joinder of any or all of these persons in the event that the Court orders a personal liability enquiry pursuant to section 162 of the Companies Act 71 of 2008, section 60 of the Employment Equity Act 55 of 1998 ("EEA"), or section 4 of the Protected Disclosures Act 26 of 2000 ("PDA"), or in any separate proceedings as the evidence may warrant.

PART C: THE EMPLOYMENT RELATIONSHIP AND THE BAIT-AND-SWITCH

1. With effect from 1 May 2024, the Applicant entered into a contract of employment with the Second Respondent in Cape Town, Western Cape.
2. In January 2024, the Applicant was recruited by the Second Respondent for the position of "Specialist: Care Manager". He received three separate interview

invitations in January 2024 (Annexure "A"), completed an assessment on 29 January 2024 (Annexure "B"), and underwent a fingerprint criminal check on 21 February 2024 (Annexure "C"). All recruitment documentation described the position as a Specialist role.

3. However, the contract presented to the Applicant for signature on 9 April 2024 downgraded his position to "Care Coordinator" (Grade B4M1) at a cost-to-company of R490,032 per annum — without any prior notification of the change in designation (Annexure "D").
4. This deliberate act of title suppression constituted a "bait-and-switch": the Applicant was recruited and utilised as an NQF Level 8 Specialist but was officially designated as a junior "Care Coordinator" to:
 - (a) avoid paying a remuneration package commensurate with a Specialist;
 - (b) suppress his management status in employment records, certificates of service, and UIF declarations; and
 - (c) facilitate potential B-BBEE fronting, in respect of which the B-BBEE Commission has allocated Case No. 5/02/2026.
5. In support of the assertion that the Applicant performed Specialist-level functions, it is noted that during his employment he was appointed as Acting Chairperson of the Employee Health and Safety (EHS) Committee and as Lead First Aider for the 2nd, 3rd and 4th floors of the Second Respondent's premises. On 25 July 2025, the Applicant drafted and implemented a First Aider roster covering all three floors and issued directives to team leaders regarding occupational health and safety compliance — functions that no "Care Coordinator" would perform (Annexure "E"). These responsibilities constitute operational management control and confirm that his true role was that of a Health Services Management Specialist (NQF Level 8).
6. The Senior HR Business Partner, Cheryl-Dawn Modern, confirmed in writing on 19 January 2026 that the "naming convention changes [were] still in progress" — an admission that the Applicant's designation had not been updated to reflect his true role. This constitutes an admission that the Certificate of Service is inaccurate.

PART D: MENTAL HEALTH DISCLOSURE AND FAILURE TO ACCOMMODATE

1. From 18 March 2025, the Applicant was admitted as an inpatient at Akeso Stepping Stones Clinic for severe depression and post-traumatic stress disorder ("PTSD"). A sick note issued by his treating psychiatrist, Dr Jacques Malan, is attached as Annexure "F".
2. **On 16 May 2025, the Applicant provided Dr Malan's psychiatric letter to Mapule Mashele (Senior Manager, Second Respondent), placing the Second Respondent on formal written notice of his disability within the meaning of section 1 of the EEA and his need for reasonable accommodation (Annexure "G").**
3. **Notwithstanding this formal notification, Mary-Ann Jones (Team Leader, Second Respondent), failed to provide reasonable accommodation. Specifically:**
 - (a) On 16 April 2025, Jones stated in writing: "we do not apply staggering of money owed" — a position the Second Respondent later deployed to justify the unlawful deduction (Annexure "H");
 - (b) On 7 May 2025, the Applicant requested staggering of unpaid leave deductions; Jones declined;

- (c) On 20–21 May 2025, the Applicant suffered a medical crisis and was required to leave the workplace to obtain medication; Jones instructed him to log unpaid leave, thereby applying a punitive response to a medical emergency.
- 4. The Respondents' failure to provide reasonable accommodation to an employee with a documented disability, of which they had formal written notice, constitutes:
 - (a) unfair discrimination on the ground of disability in terms of section 6(1) of the EEA, read with section 7(b)(iii) of the Code of Good Practice on the Employment of Persons with Disabilities; and
 - (b) a failure to comply with the duty to take all reasonable steps to prevent unfair discrimination under section 60 of the EEA.

PART E: THE UNLAWFUL SALARY DEDUCTION — BCEA SECTION 34(2)

1. In July 2025, while the Applicant was between psychiatric admissions and while the Respondents were in possession of Dr Malan's letter confirming his medical vulnerability and need for accommodation, the Second Respondent deducted the sum of R14,811.77 (representing 40.47% of his gross monthly salary) in a single pay cycle, leaving him with a net pay of R15,679.29. The payslip is attached as Annexure "I".
2. This deduction was made:
 - (a) without any prior written agreement or written consent as required by section 34(1) of the BCEA;
 - (b) in excess of the 25% cap imposed by section 34(2) of the BCEA;
 - (c) without any prior consultation with the Applicant, despite his express earlier request for staggering; and
 - (d) in breach of the Second Respondent's own HCBP Policy, which also imposes a 25% cap and requires prior consultation.
3. On 24 July 2025, Jones shared the AOD Policy confirming: "monthly deductions must not be more than 25% of the employees CTC as per legislation" (Annexure "J"). When the Applicant pointed out that the deduction was therefore illegal, Jones replied: "Hi Wilbur, what is illegal?" — feigning ignorance of the very statutory cap she had just confirmed. This exchange constitutes direct evidence that the Second Respondent was aware of the legal prohibition and proceeded regardless.
4. The deduction contravenes section 34(2) of the BCEA. The Applicant is accordingly entitled to recover R14,811.77 together with interest at the prescribed rate from 23 July 2025 to date of payment in terms of section 77A of the BCEA. The Applicant notes that a separate civil claim for this amount has been filed in this Court as Case No. 2026-077546.

PART F: GENDER IDENTITY DISCRIMINATION — THE 'WILMA' INCIDENT

1. On 23 September 2025, immediately after the Applicant proposed a clinical filtering strategy to improve the operational management of member adherence cases, Jones posted a message in a team-wide Microsoft Teams channel in which she:
 - (a) referred to the Applicant as "Wilma" — a feminised version of his name, knowing the Applicant is gay; and
 - (b) in the same message, prohibited the Applicant's filtering strategy with "NO FILTERING" highlighted in red.

A copy of this Teams message is attached as Annexure "K".

2. The temporal and contextual nexus between the Applicant's clinical proposal and Jones's response is direct: the feminisation of his name and the simultaneous rejection of his strategy in a team-wide communication constitutes harassment on the grounds of gender identity and sexual orientation within the meaning of section 6(3) of the EEA, read with clause 3.6 of the Code of Good Practice on the Handling of Sexual Harassment Cases in the Workplace.
3. On 25 September 2025, the Applicant lodged a formal complaint of sexual orientation discrimination and harassment with Human Resources, requesting: a neutral investigator; that he not be required to meet with the accused manager; that Jones be suspended pending investigation; and that he be removed from Jones's reporting line (Annexure "L").
4. HR's response was inadequate:
 - (a) The Applicant was told he would not be required to meet Jones, but he was never removed from her reporting line — leaving him directly exposed to further retaliation;
 - (b) **On 29 September 2025 — the same day the formal complaint was filed — Jones loaded a double punitive unpaid leave entry against the Applicant. The Mapule Mashele reversed the entry on 1 October 2025, confirming that it was discretionary and retaliatory;**
 - (c) **On 1 October 2025, Mapule Mashele (Senior Manager, Second Respondent) closed the investigation, concluding that Jones's "Wilma" was a "typo" — a plainly inadequate finding that ignored the Applicant's May 2025 disclosure of Jones's hostile conduct, the immediate context (clinical proposal being simultaneously rejected), and the team-wide visibility of the message. The closure report is attached as Annexure "M".**
5. The Respondents' failure to take reasonable steps to prevent and address the harassment constitutes a contravention of section 60 of the EEA. The discriminatory conduct and the whitewash investigation constitute unfair discrimination in terms of section 6(1) of the EEA on the grounds of sexual orientation and/or gender identity.

PART G: PROTECTED DISCLOSURE — PATIENT SAFETY (GEXUS/DMS INTEGRATION GAP)

1. **On 23 October 2025, the Applicant made a protected disclosure to Jones, copied to Lisa Mari (Responsible Pharmacist, Second Respondent), identifying a systemic structural integration gap between the Gexus and DMS platforms that was generating false non-adherence letters to GEMS members, including a 75-year-old dependent (Annexure "N").**
2. The disclosure constitutes a protected disclosure within the meaning of section 1 of the PDA in that it related to:
 - (a) a failure to comply with a legal obligation, namely the Responsible Pharmacist's obligation under section 49 of the Pharmacy Act 53 of 1974 to ensure the integrity of pharmaceutical care communications; and
 - (b) conduct likely to endanger the health or safety of any individual, namely GEMS members receiving clinically incorrect adherence assessments.
3. **The Applicant's root cause analysis confirmed that the integration gap was structural — not an isolated error — affecting any member transitioning**

between benefit categories. He stated: "Given that the integration gap still exists, I will not contact the member at this time." Jones responded: "The below is noted. I am not in agreement with your feedback though." The Lisa Mari, as Responsible Pharmacist, did not intervene to correct the system error.

4. The Applicant was subsequently:

- (a) labelled as "insubordinate" for declining to contact a vulnerable patient on clinically incorrect grounds;
- (b) excluded from the resolution process for the integration error he had identified; and
- (c) compelled to issue a forced apology to Mary-Ann Jones on 23 October 2025 for "poor communication and failure to comply" — an apology extracted under constructive duress for a legitimate patient safety disclosure.

5. The Applicant's disclosure, and the adverse treatment that followed it, engage both section 3 of the PDA (prohibition on occupational detriment for making a protected disclosure) and section 187(1)(h) of the LRA (automatically unfair dismissal where the reason for dismissal is the making of a protected disclosure).

10. This cause of action is pleaded as an independent head of damage and is not in substitution of any other claim set out in this Statement of Claim.

9. The Applicant's moral rights under section 20 of the Copyright Act — including the right to claim authorship of the work and to object to any distortion, mutilation, or modification of the work — were infringed by the Second Respondent's failure to acknowledge him as the author upon implementation.

(d) injury to his professional dignity and moral rights as the author of the work, recognised in section 20 of the Copyright Act.

(c) deprivation of the opportunity to register the programme in his own name; and

(b) loss of the commercial or market value of the programme as an independently CPD product, including potential accreditation value;

(a) loss of the professional credit, authorship attribution, and reputational recognition to which he was entitled as the originator of the programme;

8. The Applicant suffered the following loss and damage as a direct result:

(d) conduct inconsistent with the Second Respondent's obligations as an accredited medical scheme administrator in terms of sections 24 and 26 of the Medical Schemes Act 131 of 1998, which require governance and compliance standards consistent with professional integrity.

(c) a further act of retaliation and occupational detriment within the meaning of section 1 of the PDA, in that having already penalised the Applicant for his protected clinical disclosures, management proceeded to appropriate his professional intellectual work without recognition — an act of professional erasure that compounds the pattern of victimisation described in this Statement of Claim; and

(b) misappropriation of the Applicant's intellectual property under the common law of South Africa, constituting an unjustified enrichment of the Second Respondent at the Applicant's expense;

(a) copyright infringement under section 23 of the Copyright Act 98 of 1978, read with section 6 thereof, in that the Second Respondent reproduced, adapted, and/or communicated the work to the public without the consent of the copyright holder;

7. The Second Respondent's implementation of the programme without the Applicant's consent constitutes:

(c) management's rejection of the programme in April 2025 itself constitutes an implicit acknowledgment that it was the Applicant's independent work product and not an employment deliverable.

(b) the development was undertaken on the Applicant's own initiative and at his own time and expense; and

(a) the programme was developed independently, outside of the Applicant's contracted duties as a Care Coordinator / Case Manager, and was not part of any assigned work task;

6. The Applicant's CPD programme does not constitute a work made in the course of employment within the meaning of section 21(1)(d) of the Copyright Act because:

5. The Applicant's CPD programme constitutes an original literary and/or educational work within the meaning of section 2(1)(a) of the Copyright Act 98 of 1978 ("the Copyright Act"). Copyright vested automatically in the Applicant upon creation of the programme pursuant to section 21(1)(a) of the Copyright Act, as the Applicant is the author of the work. No written agreement transferring, assigning, or licensing the copyright from the Applicant to the Second Respondent was concluded at any time.

4. The temporal sequence — formal rejection in April 2025, silent implementation in October 2025 — is consistent with a deliberate appropriation of the Applicant's intellectual labour. It mirrors the broader pattern of intellectual extraction documented throughout this Statement of Claim, in which the Second Respondent consistently suppressed the Applicant's specialist-level contributions (the title suppression in Part C; the clinical filtering strategy rejected in Part F; the patient safety disclosure in Part G) while simultaneously benefiting from them.

3. In October 2025 — six months after the formal rejection — the Applicant discovered that the Second Respondent had implemented a CPD programme materially identical in structure, content, and pedagogical framework to the programme he had developed and presented. The implementation was carried out without the Applicant's knowledge or consent, without attribution of authorship, and without any form of remuneration, credit, acknowledgment, or recognition of his intellectual contribution.

2. The programme was rejected by management without written reasons and without any indication that it had been retained, evaluated further, or referred for any future implementation.

1. In approximately April 2025, the Applicant independently conceptualised, researched, and developed a structured Continuing Professional Development ("CPD") programme intended for submission under the South African Nursing Council ("SANC") CPD accreditation

framework. The programme was presented to the Second Respondent's management in April 2025 as a bespoke professional development offering for nursing staff employed within the Second Respondent's clinical operations.

PART H: MISAPPROPRIATION OF INTELLECTUAL PROPERTY — CPD PROGRAMME

PART I: THE RECORDED ADMISSION OF RETALIATORY INTENT

1. On 31 October 2025, in a meeting that the Applicant lawfully recorded, Jones stated:

"You made a case of harassment against me, so I can make a case of harassment against you. You misspelled my name in an email — go look."

Jones further warned the Applicant to "think of the ripple effect" of his actions as a person who had made formal complaints and protected disclosures.

A transcript of the recording is attached as Annexure "O".

2. This statement constitutes direct contemporaneous evidence of a causal nexus between the Applicant's protected activities (formal discrimination complaint, patient safety disclosure) and Jones's retaliatory intent. It satisfies the causal test for automatically unfair dismissal under section 187(1)(h) and for occupational detriment under section 3 of the PDA.
3. Jones's warning about "the ripple effect" of the Applicant's whistleblowing activities constitutes a veiled threat within the meaning of "any act or omission that prejudices the employee" as contemplated in the definition of "occupational detriment" in section 1 of the PDA.

PART J: INSTITUTIONAL SILENCE AND EXHAUSTION OF INTERNAL REMEDIES

1. On 2 November 2025, the Applicant filed a formal grievance against Jones, detailing malicious intent, gaslighting, weaponisation of past issues, orchestrated provocation, and a sustained campaign for his removal. He demanded an investigation, removal from Jones's team, and a written guarantee against retaliation (Annexure "P"). The Respondents met this grievance with over 30 consecutive days of institutional silence — no acknowledgment, no investigation, no response.
2. **On 4 December 2025, the Applicant escalated the matter to executive leadership — including Gerald Van Wyk (CEO, First Respondent) — with a 32-page evidence bundle, requesting paid recuperation leave, a financial settlement, and a written guarantee of non-retaliation (Annexure "Q").**
3. The Chief Risk Officer (Monwabisi Kula) responded on 9 December 2025, acknowledging receipt but stating that interim feedback would only be provided in January 2026 due to the "festive break". This response, given the humanitarian urgency of the Applicant's situation (two months in rental arrears, facing eviction, no food or medication funds), constituted a deliberate delay tactic rather than a good-faith engagement. This after the de-escalation of my complaint by Executive management back then the department I complained about on the 5th of December 2025 which officially closed the door with the executive team..
4. **On 9 December 2025, the Applicant escalated to the Board of Directors (directors of the First Respondent (as identified in the supporting affidavit) with 38 attachments documenting discrimination, retaliation, King IV governance**

breaches, and material legal risks (constituting a Priority 1 Material Risk Report). The Board did not respond.

5. On 17 December 2025, the CRO sent a further communication asking the Applicant to "stop sending these e-mails to our directors", stating that continued escalation "only gives them an indication of impatience and unreasonableness on your part." The Applicant declined, asserting his right to escalate material risks to oversight committees. This gatekeeping response constitutes evidence of the Board's deliberate shielding from governance accountability.
6. The Applicant had exhausted all reasonable internal remedies prior to his resignation. The 30+ days of institutional silence following the November 2025 formal grievance, the "festive break" delay, and the CRO's gatekeeping communications collectively demonstrate the futility of further internal engagement — a relevant consideration under section 186(1)(e) of the LRA for purposes of establishing constructive dismissal.

PART K: THE EMPLOYER'S BAD-FAITH OPPOSING AFFIDAVIT

1. On 10 December 2025, the Respondents, through their legal representative Xolile Vincent Manyati, filed an opposing affidavit in the CCMA (Case WECT23564-25) (Annexure "R"). The affidavit contained the following personal attacks made under oath:
 - (a) The Applicant's diagnosed depression and PTSD were dismissed as a "cynical excuse" and an "afterthought";
 - (b) The Applicant was labelled as having an "untruthful character";
 - (c) His clinical and professional conduct was described as "sloppiness" and a "fishing expedition"; and
 - (d) The Respondents demanded that his application be dismissed "with cost".
2. This affidavit was filed the day after the Applicant's board-level escalation. The characterisation of a SANC-registered NQF Level 8 nurse's documented psychiatric condition as a "cynical excuse" in a sworn document constitutes:
 - (a) discrimination on the ground of disability under section 6(1) of the EEA;
 - (b) a violation of the right to dignity under section 10 of the Constitution of the Republic of South Africa, 1996; and
 - (c) evidence of the bad faith that pervaded the entire employment relationship and that renders the employer's conduct unlawful for purposes of the costs prayer.
3. The filing of this affidavit, combined with the cumulative conduct described above, eliminated any objective possibility of restoring the employment relationship and constituted the final act rendering continued employment intolerable.

PART L: RESIGNATION — AUTOMATICALLY UNFAIR CONSTRUCTIVE DISMISSAL

1. On 11 December 2025, the Applicant gave formal notice of resignation, effective 12 December 2025, citing the employer's sworn affidavit as the final intolerable act. The resignation letter is attached as Annexure "S".
2. The employment situation had become objectively intolerable for the purposes of section 186(1)(e) of the LRA due to the cumulative effect of the following:

- (a) The unlawful salary deduction of 40.47%, made without consultation and in breach of the statutory cap, while the employer was in possession of the Applicant's psychiatric assessment confirming medical vulnerability;
 - (b) Discrimination and harassment on the grounds of gender identity and sexual orientation ('Wilma' incident), followed by a whitewash investigation;
 - (c) Retaliation and insubordination labelling following a bona fide protected disclosure of a patient safety risk;
 - (d) The recorded admission of retaliatory intent by Jones;
 - (e) Over 30 days of institutional silence on formal grievances, demonstrating the futility of internal remedies;
 - (f) The executive 'festive break' delay and the CRO's gatekeeping of Board-level escalation;
 - (g) The sworn, defamatory affidavit attacking the Applicant's psychiatric condition and professional character; and
 - (h) The Board's failure to respond to the Priority 1 Material Risk Report.
3. The Applicant's resignation constitutes a constructive dismissal within the meaning of section 186(1)(e) of the LRA. The dominant reason for the dismissal — pervading all of the conduct described above — is the Respondents' retaliation against the Applicant for making protected disclosures as defined in the PDA. The dismissal is accordingly automatically unfair in terms of section 187(1)(h) of the LRA.
4. Alternatively, if the Court finds that the dismissal does not fall within section 187(1)(h), the Applicant contends that the dismissal was automatically unfair under section 187(1)(f) of the LRA in that the reason for the dismissal relates to the Applicant's disability (diagnosed psychiatric condition) — an enumerated ground under section 6 of the EEA.
5. Further alternatively, if the Court finds that the dismissal is not automatically unfair, the Applicant submits that it was substantively and procedurally unfair within the meaning of section 185 of the LRA, for all the reasons set out above.

PART M: CAREER DESTRUCTION — INABILITY TO PRACTISE NURSING

1. As a direct and proximate result of the unlawful 40.47% salary deduction in July 2025, the Applicant was rendered financially destitute. He could not afford to pay his annual registration fee to the South African Nursing Council ("SANC") and consequently his nursing registration was not renewed.
2. As a result, the Applicant is currently barred from practising as a registered nurse — a permanent deprivation of his professional livelihood, constituting career destruction.
3. The Applicant had been accepted into the Postgraduate Diploma in Occupational Health Nursing at the Cape Peninsula University of Technology ("CPUT") for the 2026 academic year. Due to the Respondents' financial sabotage, he could not pay the registration fee and lost that professional development opportunity. The acceptance letter is attached as Annexure "T".
4. The loss of nursing registration and the lost CPUT opportunity constitute recoverable consequential damages caused by the unlawful deduction. They are pleaded as damages under section 4 of the PDA (occupational detriment) and under the general damages jurisdiction of this Court.

PART N: POST-RESIGNATION VICTIMISATION

1. After the Applicant's resignation, the Respondents continued to cause him harm through the following acts of post-resignation victimisation:
 - (a) UIF Non-Declaration: UIF contributions were deducted from the Applicant's June 2025 salary but the Respondents failed to declare his employment for that month. The uFiling portal continues to reflect 'No Information' for June 2025 (Annexure "U"). This constitutes a contravention of section 5 of the Unemployment Insurance Contributions Act 4 of 2002, as well as potential fraud under section 68 of the Unemployment Insurance Act 63 of 2001;
 - (b) Misrepresentation of Certificate of Service: The Certificate of Service issued to the Applicant designates him as 'Care Coordinator' rather than 'Care Manager' or 'Health Services Management Nurse Specialist'. The Respondents have refused to correct this despite the Applicant's formal demand and have withheld the recording of the internal 'Naming Convention' meeting that would confirm his true title (Annexure "V"). This constitutes a contravention of section 42 of the BCEA; and
 - (c) Withholding of Exit Documents: The Respondents delayed providing the UI-19 form and retirement fund claim forms, causing additional financial prejudice to an already destitute applicant.

PART O: REGULATORY AND THIRD-PARTY CONFIRMATION OF MATERIAL FACTS

1. The following regulatory and institutional acknowledgments independently corroborate the material facts set out in this Statement of Claim:
 - (a) Council for Medical Schemes (CMS): Formally identified 'Administrative Misconduct and Ethical Failure' in relation to the Gexus/DMS integration gap and referred the matter to HPCSA and SANC (Annexure "W");
 - (b) South African Human Rights Commission (SAHRC): Acknowledged the 'seriousness of the allegations' regarding victimisation and financial prejudice, and made referrals to the CCMA, Labour Court, and Department of Employment and Labour (Annexure "X");
 - (c) JSE Issuer Regulation Division: Acknowledged receipt of the Applicant's 325-page evidentiary bundle and has engaged the First Respondent for a response regarding potential SENS disclosure failures;
 - (d) B-BBEE Commission: Allocated Case No. 5/02/2026 for investigation of the B-BBEE fronting allegation;
 - (e) Department of Employment and Labour: Allocated an integrated inspection for UIF non-compliance (DEL IES Ref, 16 February 2026); and
 - (f) SAPS: Criminal case CAS 705/3/2026 registered at SAPS Delft on 19 March 2026 and transferred to SAPS Woodstock on 25 March 2026 for investigation of statutory fraud under the UIF Contributions Act and BCEA.
2. A full Master Chronological Register containing 148 entries from 11 January 2024 to 2 April 2026, summarising all material events, institutional responses, and patterns of deflection, is attached as Annexure "Z".

PART P: SUMMARY OF CAUSES OF ACTION

1. The Applicant's causes of action are as follows:

- (a) Primary claim: Automatically unfair constructive dismissal under sections 186(1)(e) and 187(1)(h) of the LRA — the dominant reason for the dismissal was retaliation for protected disclosures as defined in the PDA;
- (b) Alternative claim A: Automatically unfair dismissal under sections 186(1)(e) and 187(1)(f) of the LRA — the dominant reason relates to the Applicant's disability;
- (c) Alternative claim B: Constructively and substantively/procedurally unfair dismissal under sections 185 and 186(1)(e) of the LRA;
- (d) Unfair discrimination and harassment under sections 6(1), 6(3) and 60 of the EEA — on the grounds of sexual orientation, gender identity and disability;
- (e) Occupational detriment for making protected disclosures under sections 3 and 4 of the PDA;
- (f) Contravention of section 34(2) of the BCEA — unlawful salary deduction (also pleaded in Case No. 2026-077546);
- (g) Contravention of section 42 of the BCEA — failure to provide a true and accurate Certificate of Service; and
- (h) Contravention of section 5(1) of the UIF Contributions Act 4 of 2002 — failure to declare contributions for June 2025.
- (i) Misappropriation of intellectual property and copyright infringement under section 23 of the Copyright Act 98 of 1978 — the Applicant developed an original CPD programme in April 2025 which was rejected by the Second Respondent, then implemented without attribution or consent in October 2025; the Applicant's moral rights under section 20 of the Copyright Act were simultaneously infringed; and the Second Respondent was unjustifiably enriched at the Applicant's expense under the common law.

PART Q: QUANTIFICATION OF LOSS

1. The Applicant's loss is calculated as follows:

Head of Damage / Claim	Statutory Basis	Amount (ZAR)
Automatically unfair dismissal compensation (24 months × R42,796.13)	LRA s194(3)	R1,027,107.12
Unlawful salary deduction — principal amount	BCEA s34(2) / s77A	R14,811.77
Interest on unlawful deduction (prescribed rate from 23 Jul 2025)	Prescribed Rate Act	To be calculated
EEA — solatium for injury to dignity (sexual orientation / disability discrimination)	EEA s50(2)	R350,000.00
EEA — compensation for failure of reasonable accommodation	EEA s50(2)	To be assessed
PDA — compensation for occupational detriment (financial loss, SANC registration, CPUT opportunity)	PDA s4	To be assessed
Consequential damages: lost CPUT programme (registration fees and associated costs)	PDA s4 / general damages	To be quantified
Career destruction damages: loss of nursing registration and future earning capacity	PDA s4 / general damages	To be assessed

Head of Damage / Claim	Statutory Basis	Amount (ZAR)
Copyright infringement / IP misappropriation — CPD programme developed April 2025, implemented without credit October 2025 (market value of programme, moral rights, loss of authorship credit, unjustified enrichment)	Copyright Act s23 / s20 / common law	To be assessed
TOTAL (known components)		R1,391,918.89 plus assessed amounts

Note: Items marked 'to be assessed' or 'to be quantified' will be supplemented by expert evidence and/or further particulars as the matter develops. The Applicant reserves the right to amend the quantum of the claim upon receipt of further documentation from the Respondents.

PART R: RELIEF SOUGHT

The Applicant respectfully prays for the following relief:

A. DECLARATORY RELIEF

- Q.1.** A declaration that the Applicant's dismissal constitutes an automatically unfair constructive dismissal within the meaning of sections 186(1)(e) and 187(1)(h) of the LRA, on the grounds that the dominant reason for the dismissal was retaliation by the Respondents for the Applicant making protected disclosures as defined in the PDA.
- Q.2.** Alternatively to Q.1, a declaration that the dismissal was automatically unfair within the meaning of sections 186(1)(e) and 187(1)(f) of the LRA on the grounds that a dominant reason for the dismissal relates to the Applicant's disability.
- Q.3.** Further alternatively, a declaration that the dismissal was unfair within the meaning of section 185 of the LRA.

B. COMPENSATION AND DAMAGES

- Q.4.** An order that the First and/or Second Respondents pay the Applicant compensation equal to 24 months' remuneration as provided for in section 194(3) of the LRA for an automatically unfair dismissal, calculated at his monthly cost-to-company of R42,796.13 per month, amounting to R1,027,107.12, alternatively the maximum permissible compensation amount.
- Q.5.** An order that the First and/or Second Respondents reimburse the Applicant the unlawfully deducted amount of R14,811.77 together with interest at the prescribed rate calculated from 23 July 2025 to the date of final payment, alternatively that this relief be granted in Case No. 2026-077546.
- Q.6.** An order that the First and/or Second Respondents pay the Applicant compensation in terms of section 50(2) of the EEA for unfair discrimination on the grounds of sexual orientation, gender identity and disability, including solatium for injury to dignity in the amount of R350,000, alternatively such amount as this Court deems just and equitable.
- Q.7.** An order that the First and/or Second Respondents pay the Applicant compensation for occupational detriment under section 4 of the PDA, including compensation for all financial loss, career destruction, loss of nursing

registration, and the lost opportunity to pursue the CPUT Postgraduate Diploma in Occupational Health Nursing, in amounts to be assessed or proved.

C. CORRECTIVE ORDERS

Q.8. An order directing the First and/or Second Respondents to issue a corrected Certificate of Service reflecting the Applicant's true designation as Health Services Management Nurse Specialist / Case Manager (NQF Level 8) in compliance with section 42 of the BCEA, within 10 business days of the grant of this order.

Q.9. An order that the First and/or Second Respondents pay the Applicant damages for copyright infringement and misappropriation of the CPD programme developed in April 2025 and implemented without consent or attribution in October 2025, including: (a) compensation for the market value of the programme and loss of its commercial potential; (b) compensation for the Applicant's moral rights infringement under section 20 of the Copyright Act 98 of 1978; (c) compensation for injury to professional reputation and deprivation of authorship credit; and (d) disgorgement of unjustified enrichment under the common law — in amounts to be assessed or proved.

Q.10. An order directing the First and/or Second Respondents to remit the outstanding UIF contributions for June 2025 to the Unemployment Insurance Fund and to provide the Applicant with a corrected UI-19 form within 10 business days, in compliance with the Unemployment Insurance Contributions Act 4 of 2002 and section 37 of the Unemployment Insurance Act 63 of 2001.

NOTE ON PERSONAL LIABILITY OF DIRECTORS AND NAMED INDIVIDUALS:

The Applicant expressly reserves the right to institute separate proceedings against Bruno Fernandes, Dr Nkateko Munisi, Alice le Roux, Gerald Van Wyk, LSM Mpumlwana (directors and officers of the First Respondent), Mary-Ann Jones, Mapule Mashele, and Lisa Mari, in their personal capacities, for liability arising from their individual conduct as set out in the Statement of Claim herein. The Applicant further reserves the right to apply for joinder of any or all of these persons in the event that the Court orders a personal liability enquiry under section 162 of the Companies Act 71 of 2008, section 60 of the EEA, or section 4 of the PDA, or in any related proceedings.

D. COSTS

Q.11. Costs of suit on an attorney-and-client scale, in terms of section 162 of the LRA and the inherent jurisdiction of this Court, having regard to:

- (a)** the Respondents' bad faith conduct, including the filing of a defamatory opposing affidavit attacking the Applicant's diagnosed mental illness under oath;
- (b)** the systematic institutional cover-up, whitewash investigations, and 30+ days of deliberate institutional silence;
- (c)** the post-resignation victimisation through the misrepresentation of Certificate of Service and UIF non-declaration; and
- (d)** the CRO's gatekeeping communications designed to prevent Board-level accountability.

E. FURTHER AND/OR ALTERNATIVE RELIEF

Q.12. Further and/or alternative relief as this Honourable Court deems just and equitable in the circumstances.

PART S: VERIFICATION AND DECLARATION

PERJURY RESERVATION: The Applicant hereby places on record that he reserves the right to institute proceedings for perjury and/or contempt against any deponent who has knowingly provided false information under oath in any opposing affidavit filed in these proceedings, in the CCMA proceedings forming part of the record hereof, or in any related regulatory or judicial proceedings. In particular, the Applicant reserves his rights in respect of the opposing affidavit deposited by the Second Respondent's legal representative (Annexure "R"), which characterised the Applicant's documented psychiatric condition as a "cynical excuse" and labelled him as having an "untruthful character" — allegations that the Applicant submits are themselves false and made under oath. This reservation is made in terms of section 319 of the Criminal Procedure Act 51 of 1977 and the Applicant's rights under section 34 of the Constitution of the Republic of South Africa, 1996.

I, WILBUR WILLIAM MATTHEE, the Applicant herein, declare that the facts set out in this Statement of Claim are within my personal knowledge, save where indicated to the contrary, and are to the best of my knowledge true and correct.



Signature of Applicant: _____

Name: WILBUR WILLIAM MATTHEE

Identity Number: 890903 5151 087

Capacity: Applicant (Self-Represented)

Email: info@ethichawks.co.za

Tel: 069 635 1435 / 063 378 8782

Address for service: 15 Bellevue Street Voorbrug, Cape Town, 7001 (*service accepted by email/as indicated as the case evolve*)

Signed at CAPE TOWN on this: Tuesday

Date: 07th day of APRIL 2026

SCHEDULE OF ANNEXURES

The following documents are attached to and form part of this Statement of Claim. Each document is identified by the annexure reference used in the body of this Statement of Claim.

Annexure	Description	Date
"A"	Interview invitations for 'Specialist: Care Manager' role	January 2024
"B"	Assessment instructions and link – Specialist: Care Manager position	29 January 2024
"C"	Fingerprint criminal check booking confirmation – pre-employment screening	21 February 2024
"D"	Employment contract designating position as 'Care Coordinator' (Grade B4M1, CTC R490,032 p.a.)	9 April 2024
"E"	First Aider roster drafted and implemented by Applicant as Acting Chair: EHS and Lead First Aider (2nd, 3rd and 4th floors)	25 July 2025
"F"	Sick note: Dr Jacques Malan – Applicant unfit for work from 18 March 2025; admitted for 3–4 weeks inpatient treatment (severe depression, PTSD)	1 April 2025
"G"	Mental health disclosure email to Mapule Mashele (Mapule Mashele); Dr Malan's psychiatric letter provided	16 May 2025
"H"	Meeting feedback from Mary-Ann Jones (Mary-Ann Jones): 'we do not apply staggering of money owed'	16 April 2025
"I"	Payslip for July 2025 confirming net pay of R15,679.29 and deduction of R14,811.77 (40.47% of gross)	July 2025
"J"	AOD Policy screenshot confirming 25% cap on deductions; exchange in which Jones states 'what is illegal?' after acknowledging cap	24 July 2025
"K"	'Wilma' Microsoft Teams message posted by Mary-Ann Jones in team-wide channel immediately after Applicant proposed filtering strategy; 'NO FILTERING' in red	23 September 2025
"L"	Formal complaint of sexual orientation discrimination and harassment (11 pages) filed with HR	25 September 2025
"M"	Mashele's investigation closure report: 'typo' finding; Applicant not removed from Jones's team	1 October 2025
"N"	Gexus/DMS root cause analysis and protected disclosure to Jones and Lisa Mari (Lisa Mari) regarding false non-adherence letters to GEMS members	23 October 2025
"O"	Transcript and/or recording of 31 October 2025 meeting: Jones's retaliatory admission ('You made a case of harassment against me, so I can make a case of harassment against you')	31 October 2025
"P"	Formal grievance against Mary-Ann Jones (malicious intent, gaslighting, retaliation, institutional silence)	2 November 2025

Annexure	Description	Date
"Q"	Executive escalation to leadership (32-page evidence bundle, requests for paid leave, financial settlement, written guarantee)	4 December 2025
"R"	Employer's opposing affidavit in CCMA Case WECT23564-25 (personal attacks on Applicant's character and mental health; request for costs)	10 December 2025
"S"	Resignation letter citing opposing affidavit as final intolerable act	11 December 2025
"T"	CPUT Postgraduate Diploma in Occupational Health Nursing acceptance letter (lost opportunity due to financial sabotage)	2026
"U"	uFiling portal screenshot confirming 'No Information' for June 2025 UIF period	2026
"V"	Certificate of Service (disputed designation); request for 'Naming Convention' meeting recording	January 2026
"W"	CMS referral letter identifying 'Administrative Misconduct and Ethical Failure'	2026
"X"	SAHRC acknowledgment of seriousness of allegations of victimisation and financial prejudice	2026
"Y"	CCMA Certificates of Non-Resolution: WECT23564-25 and WECT24486-25	9 January 2026
"Z"	EthicHawks Full Master Chronological Register (148 entries, 11 January 2024 – 2 April 2026)	3 April 2026
"AA"	CPD programme developed by Applicant and presented to management (April 2025); evidence of independent authorship and rejection; evidence of subsequent implementation (October 2025) without attribution — in support of Part H (Misappropriation of Intellectual Property)	April 2025 / October 2025

NOTE ON FILING AND SERVICE

1. This Statement of Claim is to be filed with the Registrar of the Labour Court, Cape Town, in accordance with Rule 6 of the Rules for the Conduct of Proceedings before the Labour Court (GN 1665 of 14 October 2011).
2. **Copies of this Statement of Claim and the attached annexures are to be served on the First and Second Respondents in accordance with Rule 7 of the Labour Court Rules.**
3. In accordance with section 191(11) of the LRA, the filing of this Statement of Claim within 90 days of the issue of the CCMA Certificates of Non-Resolution (9 January 2026) is confirmed. The deadline for filing is 9 April 2026.

4. This document has been filed on the OCJ Court Online system (Case No: 2026-077546 refers to the concurrent BCEA claim). A new case number for this Statement of Claim is to be allocated by the Registrar.

ANNEXURE TABS

STATEMENT OF CLAIM — MATTHEE v AFROCENTRIC HEALTH & MEDSCHEME HOLDINGS

Labour Court, Cape Town | Filed: 8 April 2026

Each following page constitutes a separator tab for the corresponding annexure. Annexures should be inserted immediately after their respective tab.

ANNEXURE

"A"

Interview invitations for 'Specialist: Care Manager' role — three separate invitations

Date: January 2024

*Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*



Interview: Specialist Care Manager (Wilbur William Matthee)

2 messages

Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com> Thu, 11 Jan 2024 at 15:47
To: wilburmat@gmail.com <wilburmat@gmail.com>, Nthabiseng Nkatane
<nthabisengn@medscheme.co.za>, Gretchen Veldtman <Gretchen.Veldtman@afrocentrichealth.com>

Good day Wilbur

We look forward to meeting with you for a face-to-face interview for the **Specialist: Care Manager** Role at Medscheme. J

- o **Date:** 15 January 2024
- o **Time:** 12:00

- o **Venue:** [Searle street, The Boulevard](#), 4th Floor, Cape Town.

You are to please enter **4th Floor**, you will be meeting with **Nthabiseng Nkatane**.

If you have any questions, please do not hesitate to contact me. Moreover, kindly please accept or decline this invitation.

Kind Regards

Microsoft Teams meeting

Join on your computer, mobile app or room device

[Click here to join the meeting](#)



Interview: Specialist Care Manager (Wilbur William Matthee)

2 messages

Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com> Fri, 12 Jan 2024 at 08:07
To: wilburmat@gmail.com <wilburmat@gmail.com>, Nthabiseng Nkatane
<nthabisengn@medscheme.co.za>, Gretchen Veldtman <Gretchen.Veldtman@afrocentrichealth.com>

Good day Wilbur

We look forward to meeting with you for a **face-to-face** interview for the **Specialist: Care Manager** Role at Medscheme. J

- o **Date:** 15 January 2024
- o **Time:** 12:00
- o **Venue:** [Searle street, The Boulevard](#), 4th Floor, Cape Town.

You are to please enter **4th Floor**, you will be meeting with **Nthabiseng Nkatane**.

If you have any questions, please do not hesitate to contact me. Moreover, kindly please accept or decline this invitation.

Kind Regards

Microsoft Teams meeting

Join on your computer, mobile app or room device

[Click here to join the meeting](#)

Meeting ID: 392 978 788 382

Passcode: SAiaqa

[Download Teams](#) | [Join on the web](#)

Join with a video conferencing device

889941933@t.plcm.vc

Video Conference ID: 127 029 428 0

[Alternate VTC instructions](#)

Or call in (audio only)

[+27 21 836 0608,,755542986#](tel:+27218360608755542986) South Africa, Cape Town

Phone Conference ID: 755 542 986#

[Find a local number](#) | [Reset PIN](#)

[Learn More](#) | [Meeting options](#)

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Wilbur William Matthee.pdf

976 KB

Wilbur William Matthee <wilburmat@gmail.com>

Mon, 15 Jan 2024 at 12:41

To: Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com>

Good afternoon Mr Neo Sekgoka,

It is with great regret that I have to inform you minutes after the supposed interview today that I will not be able to make it i worked the night shift the entire weekend and underestimated how tired I was resulting in me oversleeping. I would appreciate it if we could reschedule the interview. I am sorry for the inconvenience I may have caused.

Kind regards

Sr. W. Matthee

[Quoted text hidden]



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ANNEXURE

"B"

Assessment instructions and link – Specialist: Care Manager position

Date: 29 January 2024

*Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*



Vacancy - Specialist Care Manager

6 messages

Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com>

Mon, 29 Jan 2024 at 08:36

Good day

Thank you for showing interest in the abovementioned position. As a part of our hiring process, we send out assessments related to the position to test your skills.

You will find an attachment to this mail covering all the instructions related to the assessment. You will also find the link to the assessment in the attachment. In order to complete the assessment successfully, you will need to go through the instructions carefully.

The due date of the assessment is **30 January 2024 @ 12am/Midday**.

Don't forget to send pivotal assessment document.

Wishing you all the best!



Neo Sekgoka
Human Capital Intern

- +27 (011) 671 4819
- neo.sekgoka@afrocentrichealth.com
- [37 Conrad St, Florida North, Roodepoort](#) | PO Box 1101, Florida Glen, 1709
- www.afrocentric.za.com



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-  **image005.png**
29 KB
-  **PT Assessment Instructions_ Medscheme (002) (Case manager etc.).docx**
15 KB

Wilbur William Matthee <wilburmat@gmail.com> Mon, 29 Jan 2024 at 09:33
To: Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com>

Thank you, will do.
[Quoted text hidden]

Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com> Mon, 29 Jan 2024 at 11:06
To: Wilbur William Matthee <wilburmat@gmail.com>

May you kindly send me everything I have requested for before the end of business day

[Quoted text hidden]
[Quoted text hidden]

Wilbur William Matthee <wilburmat@gmail.com> Mon, 29 Jan 2024 at 12:02
To: Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com>

Thank you, will do.
[Quoted text hidden]

Wilbur William Matthee <wilburmat@gmail.com> Mon, 29 Jan 2024 at 14:14
To: Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com>

Hello Neo

I hope you are well. I have just completed the online assessment as requested. I will submit the other required documents now.

My MIT fingerprint test was done in December 2023, i have the report and will send that to you on request. I have to update my CV with regards to the statement I made "criminal record" clear, in the report it states that i have a pending trial for pending blood results for the level of alcohol in my system. I do not want this statement to determine the success of my application so I have provided you with the updated CV.

thank you so much for your assistance with this.

Regards,

Sr. W. Matthee

[Quoted text hidden]



WillCvJan update.pdf

752 KB

Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com>
To: Wilbur William Matthee <wilburmat@gmail.com>

Mon, 29 Jan 2024 at 14:20

May you kindly fill in the consent form as we do our own fingerprint checks. We do not allow any fingerprint checks that were done by the candidate themselves unless otherwise specific.

[Quoted text hidden]

[Quoted text hidden]

ANNEXURE

"C"

Fingerprint criminal check booking confirmation – pre-employment screening

Date: 21 February 2024

*Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*



Vacancy - Specialist Care Manager

4 messages

Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com>
To: Wilbur William Matthee <wilburmat@gmail.com>

Mon, 29 Jan 2024 at 14:39

Good day

May you kindly fill this in

Warm regards



Neo Sekgoka

Human Capital Intern

- +27 (011) 671 4819
- neo.sekgoka@afrocentrichealth.com
- [37 Conrad St, Florida North, Roodepoort](#) | PO Box 1101, Florida Glen, 1709
- www.afrocentric.za.com



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 **MIE Processing Notification.pdf**

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Wilbur William Matthee <wilburmat@gmail.com>

Mon, 29 Jan 2024 at 14:54

To: Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com>

<https://app.luminpdf.com/viewer/65b79b5d4b7608a70677500b>

I have completed the form and left out the blanks for your perusal.

Thank you

[Quoted text hidden]

Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com>

Mon, 29 Jan 2024 at 15:25

To: Wilbur William Matthee <wilburmat@gmail.com>

I cannot access the form

[Quoted text hidden]

[Quoted text hidden]

Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com>

Mon, 29 Jan 2024 at 15:58

To: Wilbur William Matthee <wilburmat@gmail.com>

May you kindly send me the form without any link

From: Wilbur William Matthee <wilburmat@gmail.com>

Sent: Monday, January 29, 2024 2:55 PM

To: Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com>

Subject: Re: Vacancy - Specialist Care Manager

<https://app.luminpdf.com/viewer/65b79b5d4b7608a70677500b>

[Quoted text hidden]

[Quoted text hidden]

[Quoted text hidden]



Processing Notification - Background Screening Request



COMPANY DETAILS "Company"

To be completed by Company Agent

Company Name: _____ Email: _____
Agent Name: _____ Mobile No: _____

CANDIDATE PERSONAL INFORMATION

To be completed by the Candidate

Surname: **MATTHEE**
Full Names: **WILBUR WILLIAM** Maiden Name: _____
Mobile Number: **069 490 9914** Date of Birth: **03/09/1989**
ID Number / Identifier:

8	9	0	3	0	9	5	1	5	1	0	8	7
---	---	---	---	---	---	---	---	---	---	---	---	---

Description of Identifier: **SOUTH AFRICA ID NUMBER**
E.g. South African ID Number, Zimbabwean Passport Number, etc.
Physical Address: **15 BELLEVUE STREET, VOORBRUG, DELFT, 7100**
Previous Convictions: Yes ☐ No ☒

If Yes, Please Provide the Conviction Details:

Date Convicted: _____
Offence: _____
Sentence: _____

BACKGROUND SCREENING CHECKS

To be completed by the Company Agent

☐	Credit Check	☐	Sanctions
☐	Qualification	☐	Identity Verifications
☐	Employment References	☐	Insurance Regulations
☐	Criminal Checks	☐	Drivers license & Vehicles
☐	Fraud Check	☐	Social Media Screening Checks

DEFINITIONS

- ✓ "Candidate" means the person on whom the Company will process background screening checks for lawful purposes including but not limited to employment/ continuation of employment;
- ✓ "Company" refers to MIE Client;
- ✓ "Consumer Credit Information" shall have the meaning ascribed to it in section 70 of the NCA;
- ✓ "FAIS Act" shall mean the Financial Advisory and Intermediary Services Act of 2002;
- ✓ "FSCA" refers to the Financial Sector Conduct Authority;
- ✓ "NCA" shall mean the National Credit Act, No. 34 of 2005, as amended from time to time, including any regulations made under the Act;
- ✓ "Personal Information" shall have the meaning ascribed to it in Chapter 1 of POPI and includes, but is not limited to a name, address, email address, telephone or fax number, fingerprints, criminal history and education or other personal credentials provided, or which is collected from the candidate or other third parties, before and/or during the background screening process and/or thereafter;
- ✓ "POPI" shall mean the Protection of Personal Information Act, No. 4 of 2013, as amended from time to time, including any regulations made under the Act;
- ✓ "Privacy and Data Protection Conditions" refers to the 8 (eight) statutory prescribed conditions for the lawful Processing of Personal Information;
- ✓ "Responsible Parties" have meaning to the Company and MIE together, and "Responsible Party" any one of them;
- ✓ "Verification Information Suppliers" shall mean third parties acting on behalf of MIE, including, but not limited to, criminal record bureaus, credit bureaus, governmental bodies, and any educational, training, and fraud prevention organisations;

ACKNOWLEDGMENT OF THE USE OF PERSONAL INFORMATION

- I acknowledge
- ✓ that the Company's duly authorized verification agent, Managed Integrity Evaluation (Pty) Ltd ("MIE"), will need to process my Personal Information to conduct background screening checks as indicated above which are required by the Company.
 - ✓ that verification requests form part of the background screening process and that:
 - ✓ requests for credit information from Credit Bureaus will only be conducted under the regulations defined as per the NCA and for the below prescribed purposes only
 - ✓ For employment in a position of trust and honesty and entails the handling of cash or finances;
 - ✓ Fraud prevention or detection.
 - ✓ data obtained from the FSCA serve only for the purpose to determine the fitness and propriety as envisaged in the FAIS Act.
 - ✓ that any Personal Information supplied to the Company is provided voluntarily and is accurate and current as the Company may not be able to comply with its obligations if the correct Personal Information is not supplied to the Company, I further agree to correct and update such information when necessary;
 - ✓ that privacy is important to the Responsible Parties and the Responsible Parties will use reasonable efforts in order to ensure that any Personal Information in their possession or processed on their behalf is kept confidential, stored in a secure manner, and processed in terms of South African law and or applicable Data Protection Legislation, for the purposes I have authorised.
 - ✓ that all information, including Personal Information, supplied to the Company is accurate and current and agree to correct and update such information when necessary.
 - ✓ may be shared by the Company with MIE and may be further shared by MIE with the Verification Information Suppliers for verification or other legitimate purposes;
 - ✓ may be shared by the Verification Information Suppliers with MIE and be further stored and shared by MIE with the Company and Fraud databases or services when the information provided for verification is deemed fraudulent by the Verification Information Suppliers for purposes of continued or future employment or for other legitimate purposes as per the NCA or other National or Provincial Legislation;
 - ✓ may be stored by MIE, for legislated retention periods and where such periods are not legislated then my personal information may be stored by MIE for as long as the information may be needed for verification purposes, or any other period as I may agree with the MIE. and
 - ✓ Personal Information may be transferred cross-border to countries, for verification or storage purposes. In any cross-border transfer of personal information MIE will comply with the security safeguards as provided for in the POPIA including but not limited to ensuring that the information is secured when transported to or from the recipient.
 - ✓ I take note that if the Responsible Party has utilised the Personal Information contrary to the Privacy and Data Protection Conditions, I may first resolve any concerns with that Responsible Party. If I am not satisfied with such process, I have the right to lodge a complaint with the Information Regulator.
 - ✓ A copy of Personal Information kept by the Responsible Parties will be furnished to me upon request in terms of the provisions of POPI or the NCA and I understand that I may dispute any information in the record provided.
 - ✓ "I" agree that MIE, to the extent permitted by law, will not be liable for any complaint, claim or action brought by me, arising from any action or omission by MIE, to the extent that such action or omission resulted from MIE complying with the terms of this agreement and I shall indemnify, hold harmless and defend MIE from and against any such claims or actions brought against MIE."

SIGNATURE	CANDIDATE	COMPANY AGENT
DATE	29/01/2024	

All signatories to this document agree that MIE will NOT be held liable for the content, factual correctness or accuracy of any Supplier Data supplied to MIE for the Company by MIE's suppliers. The Company and the Candidate hereby indemnifies and holds MIE harmless against any loss arising from neglect or damage in procuring, communicating, or failing to communicate information to the Company.

ANNEXURE

"D"

Employment contract designating position as 'Care Coordinator' (Grade B4M1, CTC R490,032 p.a.) — evidence of bait-and-switch

Date: 9 April 2024

*Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*

09 April 2024

W Matthee
C/O AFROCENTRIC HEALTH (RF) (Pty) Ltd

Dear Wilbur,

INTERNAL APPOINTMENT: Care Coordinator

We are pleased to offer you the position **Care Coordinator (Reference IRC78739)** within AfroCentric Health (RF) (Pty) Ltd with effect from 01 May 2024. This position is situated at Cape Town.

The Key elements of your duties and responsibilities will be as discussed with you and may be revised from time to time thereafter.

CONDITIONS OF EMPLOYMENT

1. PROBATION

You will be required to successfully complete a probationary period of **six (6)** continuous months in this new position. In terms of the Group Talent Acquisition Policy, you will not be permitted to apply for any vacant position internally during this probation period.

2. REMUNERATION

We are pleased to advise you that your total package on a cost to company basis will be **R490,032.00 per annum**, payable by bank transfer into your Bank account.

The above cost to company figure includes all your contributions towards Medical Aid, Pension Fund, Group Life Assurance and Disability Assurance. If you are required to use your cell phone for business purposes, this will be deemed to be part of your cost to company package.

The Grade for your position based on our Grading Model is that of **B4M1**.

3. RETIREMENT FUNDING INCOME

www.afrocentric.za.com



AfroCentric Investment Corporation Limited Reg #1988/000570/06
T +27 11 671 2000
PO Box 1101, Florida Glen 1708 | 37 Conrad Street | Florida North | Roodepoort 1709



DIRECTORS:

Dr ATM Mokgokong (Chairman), MJ Madungandaba (Deputy Chairman), GN Van Wyk (CEO), JW Boonzaaier (CFO), M Chauke, MK Dippenaar, JB Fernandes, PB Hanratty, AM le Roux, KN Mkhize, Dr ND Munisi

Group Company Secretary: LSM Mpumlwana



A percentage between **78** and **90** (78 / 80 / 85 / 90) of your total cost to company figure, as selected by yourself, will constitute your "**Retirement Funding Income**". All contributions, both employer and employee, to the Pension / Provident Fund, Group Risk Benefits such as Life and Disability Schemes will be based on this figure.

4. LEAVE ENTITLEMENT

Your annual leave entitlement will be **seventeen (17)** working days per annum.

5. HOURS OF WORK

You will be required to work **forty-five (45) hours per week**. Your working hours are flexible and will be scheduled by the Business Unit and may include a Saturday which is deemed part of your normal working week. You will be entitled to a thirty (30) minute lunch break as scheduled. Any work performed on a Saturday will be paid at normal rates. Your working hours will be determined by the requirements of the Business Unit to which you are assigned to from time to time and may be amended to suit the operational requirements of the Business Unit.

It is a condition of employment that, as and when the need arises, you will be required to work overtime. Payment, or time off in lieu of overtime worked, hours more than forty-five (45) working hours per week, will be made in accordance with the Human Capital Payroll and Benefits Management Policy.

6. OTHER CONDITIONS OF EMPLOYMENT

All other conditions of employment not specifically mentioned in this letter remain unchanged.

7. PACKAGE RESTRUCTURING

Please feel free to have a package structuring calculation/discussion with your Line Manager or the HC Business Partner responsible for your Business Unit before accepting this offer of employment. Package restructuring takes place during June/July.

Candidate Initial _____ GM Initial BQ

8. OFFER ACCEPTANCE

Kindly sign and return the copy of this letter to your HC Business Partner. Also note that this offer is valid for three (3) days from receipt.

Please initial every page and sign in full on the last page of this document before returning to your HC Business Partner.

We would like to take this opportunity to wish you well in your new position.

Yours sincerely,



Barbara Johnson
General Manager: GEMS Operations.5
AfroCentric Health (RF) (Pty) Ltd

18 April 2024

Date Signed

Candidate Initial _____ GM Initial 

I ACCEPT OFFER extended to me for the position of Care Coordinator within AfroCentric Health (RF) (Pty) Ltd at Roodepoort. I will take up the position on 01 May 2024.

I hereby acknowledge that I have read the contents of this document and fully understand and agree to abide by the provisions thereof. I also acknowledge that the contents of this document are confidential, and I will not disclose the contents to any party within the working environment.

I acknowledge that I have not been induced to complete this contract by any representations made to me by any person/s and hereby confirm that I have entered into this contract of my own free will.

By accepting this offer of appointment, I undertake to keep abreast with updates pertaining to all company policies and procedures regularly and adhere to them.

W Matthee

DATE SIGNED

Candidate Initial _____ GM Initial BG

ANNEXURE

"E"

First Aider roster drafted and implemented by Applicant as Acting Chair: EHS and Lead First Aider (2nd, 3rd and 4th floors)

Date: 25 July 2025

*Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*



Updated First Aid Daily Roster & Backup Plan

1 message

Wilbur William Matthee <wilburmat@gmail.com>
To: Wilbur Matthee <wilbur.matthee@medscheme.co.za>

Mon, 05 May 2025 at 10:24

Dear First Aid Team,

I hope this message finds you well.

Please be informed that we have implemented a new daily roster for all First Aiders to ensure fair distribution of duties across all floors. This change comes in response to recent observations that only certain First Aiders are being contacted during emergencies. The new roster aims to ensure equal participation and support throughout the building.

Each day will have a designated First Aider, as well as a backup person, in case the primary is not available.

Important:

If you are scheduled for leave this month, please indicate your leave dates on the roster.

Kindly share the updated roster with your team leaders so they are aware of your allocated days.

If you are the backup, please make sure the scheduled First Aider is on duty. If not, be prepared to step in.

Your cooperation is greatly appreciated in making sure that all areas are covered and that we respond swiftly in the event of an emergency.

Thank you for your commitment and continued support.

Best regards,

First Aider Allocation Roster: July 2025 (Weekdays Only)

Second Floor

First Aiders: Nigel Abrams, Adeline Appolles, Wilbur Matthee, Ntokozo Nxumalo (Ntokozo Nxumalo is WFH indefinitely)

Date (July 2025)	Primary First Aider	Backup First Aider	Notes
Wednesday, 2	Nigel Abrams	Adeline Appolles	Ntokozo Nxumalo WFH
Thursday, 3	Wilbur Matthee	Nigel Abrams	Ntokozo Nxumalo WFH
Friday, 4	Adeline Appolles	Wilbur Matthee	Ntokozo Nxumalo WFH
Monday, 7	Nigel Abrams	Adeline Appolles	Ntokozo Nxumalo WFH
Tuesday, 8	Wilbur Matthee	Nigel Abrams	Ntokozo Nxumalo WFH
Wednesday, 9	Adeline Appolles	Wilbur Matthee	Ntokozo Nxumalo WFH
Thursday, 10	Nigel Abrams	Adeline Appolles	Ntokozo Nxumalo WFH
Friday, 11	Wilbur Matthee	Nigel Abrams	Ntokozo Nxumalo WFH
Monday, 14	Wilbur Matthee	Nigel Abrams	Ntokozo Nxumalo WFH
Tuesday, 15	Adeline Appolles	Wilbur Matthee	Ntokozo Nxumalo WFH
Wednesday, 16	Nigel Abrams	Adeline Appolles	Ntokozo Nxumalo WFH
Thursday, 17	Wilbur Matthee	Nigel Abrams	Ntokozo Nxumalo WFH
Friday, 18	Adeline Appolles	Wilbur Matthee	Ntokozo Nxumalo WFH
Monday, 21	Nigel Abrams	Adeline Appolles	Ntokozo Nxumalo WFH
Tuesday, 22	Wilbur Matthee	Nigel Abrams	Ntokozo Nxumalo WFH
Wednesday, 23	Adeline Appolles	Wilbur Matthee	Ntokozo Nxumalo WFH
Thursday, 24	Nigel Abrams	Adeline Appolles	Ntokozo Nxumalo WFH
Friday, 25	Wilbur Matthee	Nigel Abrams	Ntokozo Nxumalo WFH
Monday, 28	Wilbur Matthee	Nigel Abrams	Ntokozo Nxumalo WFH
Tuesday, 29	Adeline Appolles	Wilbur Matthee	Ntokozo Nxumalo WFH
Wednesday, 30	Nigel Abrams	Adeline Appolles	Ntokozo Nxumalo WFH
Thursday, 31	Wilbur Matthee	Nigel Abrams	Ntokozo Nxumalo WFH

Third Floor

First Aiders: Tarryn Newman, Lucian Geswindt, Robin Lang, Zakee Ebrahim (Tarryn

Newman on annual leave from July 7th to 11th)

Date (July 2025)	Primary First Aider	Backup First Aider	Notes
Wednesday, 2	Tarryn Newman	Lucian Geswindt	
Thursday, 3	Lucian Geswindt	Robin Lang	
Friday, 4	Robin Lang	Zakee Ebrahim	
Monday, 7	Lucian Geswindt	Robin Lang	Tarryn Newman on AL (Mon-Fri)
Tuesday, 8	Robin Lang	Zakee Ebrahim	Tarryn Newman on AL (Mon-Fri)
Wednesday, 9	Zakee Ebrahim	Lucian Geswindt	Tarryn Newman on AL (Mon-Fri)
Thursday, 10	Lucian Geswindt	Robin Lang	Tarryn Newman on AL (Mon-Fri)
Friday, 11	Robin Lang	Zakee Ebrahim	Tarryn Newman on AL (Mon-Fri)
Monday, 14	Tarryn Newman	Lucian Geswindt	Tarryn Newman returns from AL
Tuesday, 15	Lucian Geswindt	Robin Lang	
Wednesday, 16	Robin Lang	Zakee Ebrahim	
Thursday, 17	Zakee Ebrahim	Tarryn Newman	
Friday, 18	Tarryn Newman	Lucian Geswindt	
Monday, 21	Zakee Ebrahim	Tarryn Newman	
Tuesday, 22	Tarryn Newman	Lucian Geswindt	
Wednesday, 23	Lucian Geswindt	Robin Lang	
Thursday, 24	Robin Lang	Zakee Ebrahim	
Friday, 25	Zakee Ebrahim	Tarryn Newman	
Monday, 28	Robin Lang	Zakee Ebrahim	
Tuesday, 29	Zakee Ebrahim	Tarryn Newman	
Wednesday, 30	Tarryn Newman	Lucian Geswindt	
Thursday, 31	Lucian Geswindt	Robin Lang	

Fourth Floor**First Aiders: Maureen Mudau, Precious Nkonde, Sandz Dyantyi-Nzunga, Audrey Rinquest, Goduletoweng Polori (Goduletoweng Polori WFH from July 7th to 21st)**

Date (July 2025)	Primary First Aider	Backup First Aider	Notes
Wednesday, 2	Maureen Mudau	Precious Nkonde	
Thursday, 3	Precious Nkonde	Sandz Dyantyi-Nzunga	
Friday, 4	Sandz Dyantyi-Nzunga	Audrey Rinquest	
Monday, 7	Maureen Mudau	Precious Nkonde	Goduletoweng Polori WFH (Mon-Fri)
Tuesday, 8	Precious Nkonde	Sandz Dyantyi-Nzunga	Goduletoweng Polori WFH (Mon-Fri)
Wednesday, 9	Sandz Dyantyi-Nzunga	Audrey Rinquest	Goduletoweng Polori WFH (Mon-Fri)

Date (July 2025)	Primary First Aider	Backup First Aider	Notes
Thursday, 10	Audrey Rinqest	Maureen Mudau	Goduletoweng Polori WFH (Mon-Fri)
Friday, 11	Maureen Mudau	Precious Nkonde	Goduletoweng Polori WFH (Mon-Fri)
Monday, 14	Audrey Rinqest	Maureen Mudau	Goduletoweng Polori WFH (Mon-Fri)
Tuesday, 15	Maureen Mudau	Precious Nkonde	Goduletoweng Polori WFH (Mon-Fri)
Wednesday, 16	Precious Nkonde	Sandz Dyantyi-Nzunga	Goduletoweng Polori WFH (Mon-Fri)
Thursday, 17	Sandz Dyantyi-Nzunga	Audrey Rinqest	Goduletoweng Polori WFH (Mon-Fri)
Friday, 18	Audrey Rinqest	Maureen Mudau	Goduletoweng Polori WFH (Mon-Fri)
Monday, 21	Sandz Dyantyi-Nzunga	Audrey Rinqest	Goduletoweng Polori WFH (Last day of WFH for Polori)
Tuesday, 22	Goduletoweng Polori	Maureen Mudau	Goduletoweng Polori returns to office
Wednesday, 23	Maureen Mudau	Precious Nkonde	
Thursday, 24	Precious Nkonde	Sandz Dyantyi-Nzunga	
Friday, 25	Sandz Dyantyi-Nzunga	Audrey Rinqest	
Monday, 28	Maureen Mudau	Precious Nkonde	
Tuesday, 29	Precious Nkonde	Sandz Dyantyi-Nzunga	
Wednesday, 30	Sandz Dyantyi-Nzunga	Audrey Rinqest	
Thursday, 31	Audrey Rinqest	Goduletoweng Polori	

ANNEXURE

"F"

Sick note: Dr Jacques Malan – Applicant unfit for work from 18 March 2025; admitted for 3–4 weeks inpatient treatment (severe depression, PTSD)

Date: 1 April 2025

*Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*

Dr Jacques Malan

SPECIALIST - PSYCHIATRIST
MBChB(UCT) FCPSYCH(SA)
PR. NO. 022 000 0449113



Akeso Kenilworth Clinic
32 Kenilworth Road
Kenilworth 7708
Tel: 021 763 4502

drmalanpsychiatrist@gmail.com

The employer

15 May 2025

Re: Mr Wilbur Matthee
Date of Birth: 03/09/1989

Mr Matthee was admitted to Akeso Stepping Stones Clinic on 18 March 2025 due to significant mental health difficulties. He required in patient treatment for severe depression and PTSD. Due to the severity of his illness his initial 3 week treatment period was extended to 6 weeks in order to provide optimal treatment and restore his occupational function to premorbid levels.

However due to major personal and social problems (theft from his home and loss of his pets) and a lack of any remaining sick leave days Mr Matthee was forced to leave treatment early to deal with matters at home. Now that this has settled Mr Matthee should return to hospital to complete his treatment properly. He would ideally be re-admitted to Stepping Stones to finish the remainder of his therapy and complete his recovery program including comprehensive discharge planning. Currently he is at risk and struggles to cope with ordinary daily tasks and occupational requirements and he may relapse further in his mental health if not afforded the time to finish his treatment. He is expected to need 2 further weeks to complete the treatment program and should be admitted as soon as possible.

If you have any queries regarding this request please contact the office.

Kind regards

Dr Jacques Malan

ANNEXURE

"G"

Mental health disclosure email to Mapule Mashele (Senior Manager): Dr Malan's psychiatric letter provided placing Second Respondent on formal notice of disability

Date: 16 May 2025

*Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*

Cc:
Bcc:
Date: Fri, 16 May 2025 12:37:35 +0000
Subject: Emailing Wilbur Matthee - letter to employer.pdf

Dear Mapule,

Please find letter from my treating Dr attached as requested for further information feel free to speak to my Dr directly via details provided or speak to me if you require additional information.

Best regards

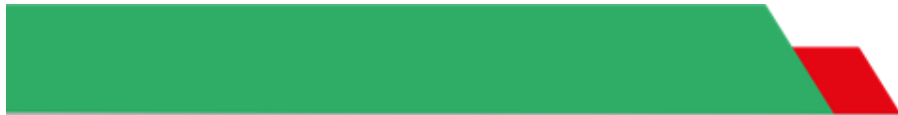


Sr. Wilbur W. Matthee

(Nurse Specialist PGrad HSM, Btech Gen Pn (Midw,Psych,Comm))

Care manager: GEMS Active Disease Risk Management

- +27 21 466 1332
- wilbur.matthee@medscheme.co.za
- The Boulevard Building F & G, Searle street, Woodstock,7925
- www.medscheme.com



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----- Forwarded message -----

ANNEXURE

"H"

Meeting feedback from Mary-Ann Jones (Team Leader): 'we do not apply staggering of money owed' — refusal to accommodate disability

Date: 16 April 2025

*Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*

Mary-Ann Jones <Mary-AnnJ@medscheme.co.za>

Wed, 16 Apr 2025 12:59:04 +0200
Subject: Meeting Feedback 16 April 2025

Wendy Wilbur

Hi Wendy,

Please find a summary of our meeting yesterday.

Welcome back-I wish you all the best with your new path. Thanks for what you shared and the changes you are making to your life.
5 days unpaid leave-I have reached out to HR via email regarding this matter but just want to mention that we do not apply staggering of money owed. Email has been sent to HR and feedback will be provided as soon as received.
Sick note or letter from doctor required to assist with accommodations.
Accommodations made upon return to work:

1. Allocation to LACE cases only for 2 weeks/until end of the month.
2. Working hours will be from 08h00-17h00
3. 7 working days QA grace period-email sent to QA with request.
4. 3 hours may be worked in in advance per month for doctor's visit. Department only allow 2 hours per month but part of the accommodation made now is due to follow ups with doctor. Process will be reviewed as time goes on (3 months-6 months).



ANNEXURE

"I"

Payslip for July 2025 confirming net pay of R15,679.29 and deduction of R14,811.77 (40.47% of gross) — evidence of unlawful deduction
Date: July 2025

*Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*

PO Box 1101, Florida Glen, 1708
 37 Conrad Street, Florida North,
 Roodepoort, 1709
 t +27 11 671 2000
 www.medscheme.com

Employee Identity Number	Matthee, Wilbur William(Wilbur),Mr. 8909035151087	Employer Name	Medscheme Holdings (Pty) Ltd
Employee Number	7074649	Grade	B4M1
Location	The Boulevard	Position	Care Coordinator.25
Hire Date	01-May-2024	Original Hire Date	01-May-2024
Assignment Number	7074649	Payroll	Medscheme Holdings (Permanent)
Employee Address	Street Number: 15 Street or Name of Farm: Bellevue Street Suburb or District: Voorbrug, Postal Code: 7100	Employer Address	The Boulevard 4th Floor Searle Street Cape Town 7925

Payroll Information		
Period End Date	Tax Period	Payment Frequency
31-Jul-2025	5	Calendar Month

Tax Information		
Tax Status	Tax Reference No	Pay Date
Normal	0694546243	23-JUL-2025

Summary of Payment		
Total Earnings	Total Deductions	Total Net Pay
21,777.04	6,097.75	15,679.29

Earnings	
Description	Amount
Normal Earnings	36,588.81
Unpaid Leave Recovery	(13,824.32)
Unpaid Leave	(987.45)

Deductions	
Description	Amount
Sanlam Funeral Cover	52.00
Bonitas EE	1,471.33
Sanlam Pension Fund EE	2,503.57
UIF Employee	177.12
PAYE	1,743.73
Access Tags Loss	150.00

Fringe Benefits and Additional Info	
Description	Amount
Medical Aid Bonitas ER	2,942.67
PHI Disability ER	469.00
Sanlam Pension Fund ER	2,536.95
UIF Employer Contribution	177.12
Group Life	258.70
Medical Aid Fringe Benefit	2,942.67
Medical Aid Tax Credit	728.00
Monthly Salary Info - CTC	42,796.13
PHI FBT	469.00
Retirement Funding Income %	78.00

YTD Accumulations & Balances	
Description	Amount
Medical Aid Tax Credit YTD	3,640.00
Medical Aid ER YTD	14,713.35
Medical Aid EE YTD	7,356.65
PAYE YTD	23,491.73
Normal Earnings YTD	181,133.44
Total Taxable Income YTD	161,080.84
UIF Employee Contribution YTD	885.60

Net Pay Distribution			
Payment Method	Bank Branch Code	Account Number	Amount
ACB	679000	10961611474	15,679.29

Message(s)
No Message(s) Found

ANNEXURE

"J"

AOD Policy screenshot confirming 25% cap on deductions; Jones exchange stating 'what is illegal?' after acknowledging the cap

Date: 24 July 2025

*Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*

Got it!

Thursday, 24 July

Sent unannounced!

Mary-Ann Jones 07/24 10:35

MJ

Acknowledge of Debt (AOD)**Payroll:**

- Payroll to provide a recon on benefits paid during **waiting period**.
- In cases of a recovery, HCBP must have a consultation meeting with the employee to make arrangements to recover the owed amount.
- Declined cases – payroll to recover unpaid leave and benefits paid
- AOD monthly deductions must not be more than 25% of the employees CTC as per legislation.

total deduction= >50% in total

PTSD&Depression ΔDisability

07/24 10:54

This doesn't make sense to me right now so they are allowed to deduct more than 50% of my salary at a time?

Mary-Ann Jones 07/24 11:19

MJ

This is for disability

You can go back to the policy and read the caption and the extract-that will give more context

07/24 11:20

but according to BCEA its illegal,

Edited

But im just glad its settled

Mary-Ann Jones 07/24 12:46

MJ

Hi Wilbur, what is illegal?



Comm... Highlig... Draw Text Fill & S... Edit PDF More t...

ANNEXURE

"K"

'Wilma' Microsoft Teams message posted by Mary-Ann Jones in team-wide channel immediately after Applicant proposed clinical filtering strategy; 'NO FILTERING' highlighted in red

Date: 23 September 2025

*Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*

Afternoon Belinda, Charles, Wilbur and Shuryda. We received a better report which we will filter and assign to you each for the members to be contacted. Same process apply. Just using a better list

Mary-Ann Jones 9/23 5:18 PM

Afternoon Wilma, Shuryda, Belinda and Charles. Please find updated lists added to your sheets. **NO NEED TO FILTER THESE LISTS. THIS REPORT COMES FROM HIU AND ARE THE MEMBERS WHO HAVE NOT HAD THEIR TESTS DONE YET.**
PLEASE JUST CALL EACH MEMBER AND UPDATE YOUR SHEET AS WE WOULD NEED THE TOTALS FOR WEEKLY REPORTING.
NO FILTERING PLEASE

Status of Wilbur Matthee: Social Style: Expressive; with a flair for bold, attention-grabbing communication. Learning style: Reflective & Auditory Learner: often seeking out educational insights appreciating structured advice that builds understanding step by step.

Type a message



Search

SAMSUNG

ANNEXURE

"L"

Formal complaint of sexual orientation discrimination and harassment (11 pages) filed with
Human Resources

Date: 25 September 2025

*Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*



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Always aligning our leaders & people with the business purpose.

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From: Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>

Sent: 25 September 2025 16:08

To: Cheryl-Dawn Modern <Cheryl-DawnM@afrocentrichealth.com>; Sipokazi Makaula <Sipokazi.Makaula@afrocentrichealth.com>; Thando Mandondo <ThandoM@medscheme.co.za>

Subject: Formal Complaint: Handling of Harassment and Discrimination Case

Importance: High

Dear Cheryl, and team

I am writing to raise a serious concern regarding how my recent complaint of sexual orientation discrimination and harassment has been managed.

After I reported this matter, I was informed that I should have a one-on-one meeting with the individual I reported, and that she would then provide feedback to her manager. I believe this approach is inappropriate and does not align with the principles of impartiality, confidentiality, or protection from retaliation, which are essential when dealing with sensitive cases of discrimination and harassment.

Key Concerns with Current Process

Meeting privately with the accused places me in a vulnerable position and creates the risk of further harassment or intimidation.

Having the accused “report back” on the outcome of the meeting undermines impartiality and fairness in the process.

Forwarding my report directly to the accused without first ensuring neutral oversight compromises confidentiality and objectivity.

Specific Incident of Sexual Orientation Discrimination and Harassment

On 23 September 2025, my manager deliberately posted in our work teams group a version of my name from “Wilbur” to “Wilma,” clearly referencing a female name, knowing that I am gay.

This was not a typo or accidental error, but a deliberate act intended to mock and embarrass me.

The post was made after working hours and immediately before the Heritage Day public holiday on 24 September 2025.

The manager then took two consecutive days off on 25 and 26 September 2025, ensuring that I could not address or respond to the issue immediately. She is only returning on Monday.

This action caused me significant upset, humiliation, and distress, and publicly undermined my dignity and identity.

The post also affected many of my colleagues, who expressed disgust and concern over my manager’s deliberate actions.

As a result of this incident, I found myself unable to focus or be productive at work. I requested from my manager's manager if I could leave the office for the day to manage my mental health. I was referred to another team leader, who informed me that my manager had instructed that if I leave, she will log it as a half-day unpaid leave, despite my willingness to work the two hours back to support my mental health. When I objected, the team leader told me I should consult with my manager on Monday.

This clearly demonstrates how the manager's deliberate actions directly impacted my ability to perform my work, caused undue stress, and created an environment that was hostile to my mental health.

Under South African law, this constitutes sexual orientation discrimination and harassment, violating both company policy and the legal protections afforded under the Employment Equity Act 55 of 1998 and the Protection from Harassment Act 17 of 2011.

Inconsistent Application of Disciplinary Policy

Our company's disciplinary policy clearly states that any form of harassment warrants dismissal.

At a minimum, best practice requires that the accused be suspended on full pay pending investigation.

Despite this, the manager I reported has been allowed to continue in her role, while I remain in a compromising position.

This situation suggests:

The company is safeguarding the accused instead of applying its own policy consistently.

I remain exposed to further harassment, intimidation, or retaliation.

The credibility of the disciplinary process and grievance procedures is being undermined.

Requests and Expectations

I respectfully request that:

1. This matter be handled by a neutral HR representative or independent investigator, rather than the individual I reported.
2. I not be required to meet privately with the accused.
3. The manager be suspended pending investigation, in accordance with company policy.
4. The process be conducted in a way that ensures impartiality, confidentiality, and protection from retaliation.
5. I be immediately removed from this manager's team to prevent further exposure to harassment or a hostile work environment.

Legal Reminder

Under the Employment Equity Act 55 of 1998, harassment on the basis of sexual orientation constitutes unfair discrimination.

The Labour Relations Act 66 of 1995 requires fair labour practices.

The Protection from Harassment Act 17 of 2011 safeguards against conduct that could reasonably cause harm or distress.

Please confirm in writing how the company intends to proceed to ensure this complaint is managed fairly, lawfully, and in compliance with company policy and South African labour legislation.

If the matter is not addressed appropriately, I may have no option but to escalate this complaint to the CCMA or other relevant authorities.

Thank you for your urgent attention to this matter.

Sincerely,

Wilbur Matthee

ANNEXURE

"M"

Mashele's investigation closure report: 'typo' finding; Applicant not removed from Jones's team
— evidence of whitewash investigation

Date: 1 October 2025

*Mathee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*



Fw: Follow-Up on Formal Complaint and Meeting Outcome

2 messages

Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>
To: wilburmat@gmail.com <wilburmat@gmail.com>

Fri, 05 Dec 2025 at 17:53

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If you would prefer, the disclaimer can be emailed to you by clicking:
mailto:Meds_disclaimer@medscheme.co.za .

----- Forwarded message -----

From: Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>
To: Cheryl-Dawn Modern <Cheryl-DawnM@afrocentrichealth.com>, Thando Mandondo <ThandoM@medscheme.co.za>, Tebogo Dikobe <Tebogo.Dikobe@afrocentrichealth.com>, Sipokazi Makaula <Sipokazi.Makaula@afrocentrichealth.com>
Cc: Nthabiseng Nkatane <nthabisengn@medscheme.co.za>, Mapule Mashele <Mapule.Mashele@medscheme.co.za>
Bcc:
Date: Wed, 1 Oct 2025 11:09:00 +0200
Subject: Follow-Up on Formal Complaint and Meeting Outcome

Dear [Chery-DI/Team/Investigation Team],

I would like to provide a formal update following my meeting with the investigation officer, Senior Manager Ms. Mapule Mashele.

During our discussion, we acknowledged that while errors can occur, there are also instances where actions are deliberate. After reviewing all relevant evidence and context, we navigated toward the latter and reached a shared understanding of the seriousness of the incident.

As a constructive step forward, I have decided to speak directly with Mary-Ann in the presence of Ms. Mashele to express how the situation truly affected me and to seek a way to make amends.

I would like to sincerely thank the team for your swift response to my formal complaint, and for the support, professionalism, and guidance you provided throughout the reporting process.

Kind regards,



Sr. Wilbur W. Matthee

(Nurse Specialist PGrad HSM, Btech Gen Pn (Midw,Psych,Comm))

Care coordinator: GEMS Active Disease Risk Management

- +27 12 466 1332
- wilbur.matthee@medscheme.co.za
- The Boulevard Building F & G, Searle street, Woodstock,7925
- www.medscheme.com



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 **Follow-Up on Formal Complaint and Meeting Outcome.eml**
52 KB

Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>
To: Wilbur William Matthee <wilburmat@gmail.com>

Sat, 06 Dec 2025 at 22:01

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mailto:Meds_disclaimer@medscheme.co.za .

----- Forwarded message -----

From: Mapule Mashele <Mapule.Mashele@medscheme.co.za>

To: Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>, Cheryl-Dawn Modern <Cheryl-DawnM@afrocentrichealth.com>, Tebogo Dikobe <Tebogo.Dikobe@afrocentrichealth.com>, Sipokazi Makaula <Sipokazi.Makaula@afrocentrichealth.com>

Cc: Nthabiseng Nkatane <nthabisengn@medscheme.co.za>, Thando Mandondo <ThandoM@medscheme.co.za>

Bcc:

Date: Wed, 1 Oct 2025 13:28:09 +0200

Subject: RE: Follow-Up on Formal Complaint and Meeting Outcome

Dear Wilbur,

I would like to take this moment to thank you once again for bringing this issue to our attention and, most importantly, for your willingness to engage and resolve this amicably.

@HC Team, I have engaged with both Wilbur and Mary-Anne on the complaint raised. Mary-Anne has confirmed that the incorrect spelling of Wilbur's name in the Teams chat was a typo on her part, which she regretted and has acknowledged the seriousness of. She has apologised to Wilbur, and the apology has been accepted. The issue is now considered resolved, and Mary-Anne has committed to being extra careful in the future.

Furthermore, we have agreed that the unpaid half-day leave logged for the 25th of September on Wilbur's behalf will be cancelled – Mary Anne will action and reach out to HC if she needs any support.

May Disclosures regarding Jones: "We not a loan company, You won't feel the chunk of money missing because you receive weekly online deliveries"

Wilbur has confirmed that there have been no prior issues with Mary-Anne that suggested any harassment based on sexual orientation and that this was a first occurrence. He further expressed that he does not wish to be removed from Mary-Anne's reporting line, as they work well together.

Next Steps:

- We will continue to monitor the working relationship to ensure there are no further concerns.
- Wilbur will continue to be in Mary Annes hierarchy – no changes to the reporting lines will be made.

This matter is now closed, but should any similar issues arise in future, they will be addressed promptly in line with the Prevention and Elimination of Harassment Policy.

Kind Regards

Mapule

Thank you again, Wilbur, for raising your concern in good faith and working with us towards a constructive resolution.

[Quoted text hidden]



RE_ Follow-Up on Formal Complaint and Meeting Outcome.eml

58 KB

ANNEXURE

"N"

Gexus/DMS root cause analysis and protected disclosure to Jones and Lisa Mari (Responsible Pharmacist) regarding false non-adherence letters to GEMS members

Date: 23 October 2025

*Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*

COMPLAINT RECEIVED FROM CMM HLQ TEAM

Good day Team

Hope my email finds you well.

Complaint details:

"Hi I have a 75 yr old Mom who is my one and only dependent. I religiously ensure she honours her blood collection months when due and not end there. Also collect her treatment every month from Nite Doc pharmacy. You sent her an email that you don't get records of her repeat. I request that you get your house in order and update your records. Stop confusing old lady.

SP Mahlangu

000655789"

Comment from the Medscheme CMM HLQ Team:

No email correspondence was sent to the member via GEXUS from the Medscheme CMM department.

Upon receipt of the complaint, the Medscheme CMM CMM HLQ Team contacted the main member to obtain details regarding the email that was sent. The following was advised:

FROM EMAIL ADDRESS: NOCGEMSBHRM@MEDSCHEME.CO.ZA

TO EMAIL ADDRESS: SBONGILEPR@GMAIL.COM

DATE AND TIME: 19/10/2025 13:28PM

SUMMARY OF EMAIL CONTENT:

- DID NOT RECEIVE REGULAR MONTHLY CLAIMS FOR CHRONIC MEDICATION

- WANT TO KNOW REASONS, THEN STIPULATES REASONS E.G. ERROR, SICK, OR FEELING WELL
NO LONGER NEEDS MEDS

The member has indicated that the communication received is incorrect, as they are claiming monthly for chronic medication.

Could you kindly confirm the following:

1. Is this email address associated with the ADRM space?
2. Was this communication sent from DMS?

If both are confirmed, I would like to request input from ADRM to enable me to respond on your behalf, as there is currently no HLQ agent assigned to our space.

As per my investigation, the last visible claim was in August 2025.

Could you also confirm whether this letter was part of a bulk communication or if it was sent by an agent?

Thank you in advance for your assistance.

Kind regards

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MATTHEE FEEDBACK TO COMPLAINT

1. Email Address Verification

The email address NOCGEMSBHRM@MEDSCHEME.CO.ZA does not appear under the current possible email addresses accessible via Gexus.

It is an automated email address, not linked to ADRM space but the DMS system itself.

2. Source of Communication

The communication originated from DMS.

Confirmed that it was not sent by an agent but generated automatically.

3. Root Cause Analysis

Miscommunication between Gexus and DMS systems.

Member collected cholesterol medication from acute benefit from 21/11/2017 until 17/12/2024.

From January 2025, medication was claimed under chronic benefit.

Although Gexus shows a chronic claim on 14/04/2023 onwards for cholesterol medication, DMS did not receive this update.

DMS continued tracking adherence based on acute claims only, ignoring chronic claims after the benefit change.

This resulted in the member being incorrectly flagged as non-adherent, despite being compliant.

4. Impact

Automated correspondence was sent based on incorrect source information.

Member received misleading communication, causing dissatisfaction.

5. Resolution Steps

Confirmed issue stems from system integration gaps between Gexus and DMS.

Recommended actions:

Review and update communication between systems.

Ensure chronic claims are recognized by DMS for adherence tracking.

Input from the ADRM space to assist with member correspondence

Thank you for bringing this matter to our attention. We have identified that the communication you received was the result of a system error, and we appreciate your patience while we investigated.

Please be assured that your adherence status will be updated correctly going forward, and this will not affect your record.

We sincerely apologize for any inconvenience caused and thank you for your understanding. Should you have any further questions, please feel free to reach out to us.

I hope that this is sufficient feedback.

Kind regards,

Please find final feedback below.

The comms went out correctly, this member is non-adherent.

The non-adherent methodology is you need claim twice in a rolling three months member only claimed once in the past 3 months.

Sep – No claims = 0

Aug – claims = 1

Jul – no claims =0

June – claims = 1

May – claims = 1

INSTRUCTIONS LISA MARI

Please note a member could have claimed this month but the previous two months have not claims the member will be flagged as non-adherent – and clinically and behaviourally there is a problem that requires attention, therefore the intervention and should please be managed as such in the ADRM calls too.

Kind Regards



Mary-Ann Jones

Team Leader: GEMS Active Disease Risk Management



(021) 466 1191



Mary-annj@medscheme.co.za



37 Conrad St, Florida North, Roodepoort, 1709 | PO Box 1101, Florida Glen, 1709



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From: Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>
Sent: Thursday, October 23, 2025 12:40 PM
To: Mary-Ann Jones <Mary-AnnJ@medscheme.co.za>
Subject: RE: general complaint received - GEMS 000655789 Dependent 02

Dear Mary-Ann,

I wanted to provide an update on the recent complaint investigation. Based on the information received, the team indicated that the non-adherence correspondence was sent due to the member not claiming twice within a three-month period, which aligns with the non-adherence methodology for the Jul-Sep 2025 period.

However, after reviewing the evidence, I believe the complaint stems from a different issue:

- DMS began flagging the member as non-adherent in Dec 2024, after the last acute claim.
- From Jan 2025 onward, the medication was claimed under the chronic benefit, but DMS did not recognize this change.
- As a result, automated non-adherence letters were sent on 19 May 2025 and 19 Oct 2025, despite the member being compliant.

This confirms the root cause is a miscommunication between Gexus and DMS systems, not the methodology itself. Evidence supporting this timeline has been attached for reference.

Given that the integration gap still exists, I will not contact the member at this time. Instead, I recommend that the team review this feedback and consult on the best way forward to prevent similar issues.

Thank you very much for your assistance.

Kind regards,



Sr. Wilbur W. Matthee

(Nurse Specialist PGrad HSM, Btech Gen Pn (Midw,Psych,Comm))

Case Manager: GEMS Active Disease Risk Management

- +27 12 466 1332
- wilbur.matthee@medscheme.co.za
- The Boulevard Building F & G, Searle street, Woodstock,7925
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From: Mary-Ann Jones <Mary-AnnJ@medscheme.co.za>

Sent: Thursday, 23 October 2025 12:02

To: Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>

Cc: Mary-Ann Jones <Mary-AnnJ@medscheme.co.za>

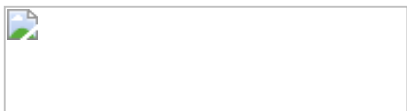
Subject: FW: general complaint received - GEMS 000655789 Dependent 02

Good day Wilbur

Please call member for enrollment and bring the non-adherence into the conversation with gentle probing as to why the medication was not claimed.

Kindly provide feedback when done.

Thank you



Mary-Ann Jones

Team Leader: GEMS Active Disease Risk Management



(021) 466 1191



Mary-annj@medscheme.co.za



37 Conrad St, Florida North, Roodepoort, 1709 | PO Box 1101, Florida Glen, 1709



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From: Mary-Ann Jones <Mary-AnnJ@medscheme.co.za>
Sent: Thursday, 23 October 2025 12:54
To: Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>
Cc: Mary-Ann Jones <Mary-AnnJ@medscheme.co.za>
Subject: RE: general complaint received - GEMS 000655789 Dependent 02

Good day Wilbur

The below is noted. I am not in agreement with your feedback though.

Kind Regards



Mary-Ann Jones

Team Leader: GEMS Active Disease Risk Management



(021) 466 1191



Mary-annj@medscheme.co.za



37 Conrad St, Florida North, Roodepoort, 1709 | PO Box 1101, Florida
Glen, 1709



www.medscheme.com

RE: general complaint received - GEMS 000655789 Dependent 02

1 message

Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>
To: Mary-Ann Jones <Mary-AnnJ@medscheme.co.za>

Thu, 23 Oct 2025 at 15:21

Hello Mary-Ann,

This investigation outcome doesn't sit well with me, knowing that I will be **excluded from further feedback or the final resolution**. I revisited our communication prior to your response and believe I understand why you were not in agreement with my feedback:

You explicitly instructed me to call the member and address non-adherence, and I can see how my tone in the email, where I stated I would not contact the member at this time, may have come across as contradicting your directive. I also understand that complaints require timely resolution, and my decision may have delayed the process, potentially causing further dissatisfaction, which you were trying to avoid.

After looking at this from a neutral perspective, I realize that I should have contacted the member as requested while still escalating the integration gap for review.

Ultimately, I appreciate **te the opportunity you gave me to work on this case**, it opened a valuable learning **experience, and I truly enjoyed investigating it**.

I hope that this instance of poor communication and failure to comply does not influence future requests for assistance.

Thank you for your guidance and support.

Kind regards,



Sr. Wilbur W. Matthee

(Nurse Specialist PGrad HSM, Btech Gen Pn (Midw,Psych,Comm))

Case Manager: GEMS Active Disease Risk Management

- +27 12 466 1332
- wilbur.matthee@medscheme.co.za
- The Boulevard Building F & G, Searle street, Woodstock,7925
- www.medscheme.com

ANNEXURE

"O"

Transcript and/or recording of 31 October 2025 meeting: Jones's retaliatory admission ('You made a case of harassment against me, so I can make a case of harassment against you')

Date: 31 October 2025

*Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*

ANNEXURE P: RECORD OF HOSTILE CONDUCT AND PROFESSIONAL MISCONDUCT – MARY-ANN JONES

This Annexure provides a summary of the hostile, malicious, and unprofessional conduct of Team Leader Mary-Ann Jones. The following highlights and verbatim quotes are extracted from the consultation on 31 October 2025 and the subsequent formal grievance (FG2). This conduct evidence points to a calculated pattern of behavior designed to undermine, isolate, and provoke the employee, creating an intolerable work environment intended to force constructive dismissal.

1. Hostile and Critical Communication

Ms. Jones initiated the interaction with immediate criticism and accusations, setting a tone of conflict.

- *"So I think there's a huge communication problem between me and you at the moment."*
- *"I don't think that what you are doing is really supporting what we need to achieve as a team and as a level two."*
- *"...the noise that you're creating today is another thing."*

2. Systematic Gaslighting and Denial of Facts

Ms. Jones repeatedly denied verifiable factual reality, such as the receipt of follow-up emails, to destabilize the employee's confidence.

- *"Well, but you did not inform me."*
- *"No, you didn't. You didn't. You did not."*
- *"I don't know what your intention is... but it's not right. I'm not standing for that."*

3. Retaliatory Threats and Weaponizing Past Issues

Ms. Jones maliciously referenced past resolved grievances and issued retaliatory threats of harassment claims based on minor clerical errors.

- *"I thought so. I thought you made a case of harassment against me last time. Isn't that so?"*
- *"And you must misspelled my name in an email. Go check. So I can make a case of harassment against you too."*

4. Accusations of Undermining and Insubordination

Ms. Jones utilized 'Double Binds' to label initiative as undermining while simultaneously demanding respect for her authority.

- *"You have the recording, if you need it.."*
- *"Yes, that was insubordination. Because you are telling me you're not going to do what I'm asking you to do."*

5. Pressure for Resignation / Transfer (Constructive Dismissal)

Ms. Jones explicitly solicited the employee's removal from the team, suggesting they were unhappy and should resign or transfer.

- *"If you're not happy in this team, ask to be removed from the team."*
- *"I'd rather do that. Send Mapule the email asking if you can be removed from the team because you do communicate like that."*
- *"Maybe it is like you say, you're being targeted... tomorrow it can be victimization and that state can be whatever."*
- *"I cannot continue on the way that we are continuing on as a team leader with somebody in my team."*

00:00:05 Wilbur Matthee
Okay.

00:00:07 Mary-Ann Jones
Okay.

00:00:08 Mary-Ann Jones
Great stuff.

00:00:11 Mary-Ann Jones
So I think there's a huge communication problem between me and you at the moment.

00:00:21 Mary-Ann Jones
I don't think that what you are doing is really supporting what we need to achieve as a team and as a level two.

00:00:38 Mary-Ann Jones
There is the request that I asked you to attend to.

00:00:41 Mary-Ann Jones
You said that you will not attend to it.

00:00:45 Mary-Ann Jones
You're sending escalations to CBNR without us knowing what it's about.

00:00:52 Mary-Ann Jones
We need to do reporting on those.

00:00:55 Mary-Ann Jones
If I have to get any questions, I wouldn't know.

00:00:58 Mary-Ann Jones
Maybe the intention is that I should not be knowing.

00:01:00 Mary-Ann Jones
But as a team leader.

00:01:02 Wilbur Matthee
Okay, so sorry.

00:01:03 Wilbur Matthee
Sorry.

00:01:03 Wilbur Matthee
Something that is not the intention.

00:01:06 Wilbur Matthee
First of all, immediately after I sent that message to some peewee, I informed you saying that I am sorry, but I forgot to cc you in that complaint.

00:01:20 Wilbur Matthee
Do you remember me sending that email to you?

00:01:23 Mary-Ann Jones
Sorry.

00:01:23 Mary-Ann Jones
Well, but you did not inform me.

00:01:26 Wilbur Matthee
I did.

00:01:27 Wilbur Matthee
I did inform you.

00:01:28 Wilbur Matthee
I did inform you.

00:01:29 Wilbur Matthee
As soon as.

00:01:30 Wilbur Matthee
As soon as I send that CMBR referral, I send you an email.

00:01:35 Wilbur Matthee
I just want to open my email quickly.

00:01:39 Mary-Ann Jones
And if I speak again, if you can just give me the opportunity to finish speaking, I

would appreciate that first.

00:01:46 Wilbur Matthee
All right, no problem.

00:01:48 Mary-Ann Jones
If there is.

00:01:49 Mary-Ann Jones
Because we can't all speak at the same time and then.

00:01:55 Mary-Ann Jones
Yeah, I don't know what your intention is with.

00:01:58 Mary-Ann Jones
With what is happening at the moment, but it's not right.

00:02:03 Mary-Ann Jones
I'm not standing for that.

00:02:06 Mary-Ann Jones
Hold on, let me finish.

00:02:08 Mary-Ann Jones
I was in Mapule's office.

00:02:10 Mary-Ann Jones
I asked if you now want to go to the other team.

00:02:13 Mary-Ann Jones
You said no, no, no, no, no.

00:02:15 Mary-Ann Jones
But what you are doing now, you are never minding me.

00:02:19 Mary-Ann Jones
You're undermining what I'm trying to do in the team.

00:02:22 Mary-Ann Jones
And I would really appreciate if you think about that.

00:02:25 Mary-Ann Jones

00:03:04 Wilbur Matthee

I didn't consult you first or CCU in the email.

00:03:08 Wilbur Matthee

Please let me know if you'd like me to forward the details so keep you updated on the outcomes.

00:03:12 Wilbur Matthee

You reply to me on that email, just good day, Wilbur.

00:03:17 Wilbur Matthee

That's right.

00:03:18 Wilbur Matthee

And then I said thank you for the sop.

00:03:20 Wilbur Matthee

So I did reply on it and I did get back to you.

00:03:23 Wilbur Matthee

So you should also take it from a manager's point of view that I am also learning these stuff still and I am going to make steps along the way.

00:03:31 Mary-Ann Jones

So.

00:03:33 Mary-Ann Jones

So why don't you reach out to me when you have a question, when you wondering about something, why don't you reach out to me first?

00:03:40 Wilbur Matthee

I normally do too.

00:03:41 Wilbur Matthee

Don't I normally do reach out?

00:03:43 Mary-Ann Jones

No.

00:03:43 Mary-Ann Jones

No, you didn't.

00:04:22 Mary-Ann Jones

I think maybe you need maybe take a step back and the noise that you're creating today is another thing.

00:04:34 Mary-Ann Jones

I spoke to you this morning.

00:04:35 Mary-Ann Jones

When I want to speak to you, I reach out to you and I speak to you.

00:04:39 Wilbur Matthee

All right.

00:04:40 Wilbur Matthee

I just didn't think it was fair.

00:04:42 Mary-Ann Jones

No.

00:04:42 Wilbur Matthee

Tell me this morning before I started to work that I need to be back in office when the agreement wasn't that.

00:04:49 Wilbur Matthee

That wasn't the initial agreement we had.

00:04:52 Wilbur Matthee

And that's why I asked the team if anyone else received that information.

00:04:57 Wilbur Matthee

And no one received that information.

00:04:59 Wilbur Matthee

What does that tell me then?

00:05:00 Wilbur Matthee

At the end of the day, what.

00:05:03 Mary-Ann Jones

Does it tell you?

00:05:04 Wilbur Matthee

Maybe I'm being targeted.

00:05:05 Wilbur Matthee
Maybe I was going to be.

00:05:07 Mary-Ann Jones
I thought so.

00:05:09 Mary-Ann Jones
I thought you made a case of harassment against me last time.

00:05:13 Mary-Ann Jones
Isn't that so?

00:05:15 Mary-Ann Jones
And you must misspelled my name in an email.

00:05:18 Mary-Ann Jones
Go check.

00:05:19 Mary-Ann Jones
So I can make a case of harassment against you too.

00:05:22 Wilbur Matthee
I'm not.

00:05:23 Wilbur Matthee
I don't want to go back and forth with you about.

00:05:24 Wilbur Matthee
About things that's really not valid about or to talk about.

00:05:28 Wilbur Matthee
So I want us to focus on what is really important.

00:05:31 Mary-Ann Jones
Yes.

00:05:32 Mary-Ann Jones
No, you want to.

00:05:33 Mary-Ann Jones

00:12:19 Mary-Ann Jones

But there is quite a bit of challenges and I mentioned that also when.

00:12:23 Mary-Ann Jones

When I reach out to you guys with all of this of the pilot that everybody wants.

00:12:30 Mary-Ann Jones

So yeah.

00:12:32 Mary-Ann Jones

So please just keep an open mind.

00:12:35 Mary-Ann Jones

I would appreciate if you take a step back.

00:12:37 Mary-Ann Jones

If you do decide that you do ask Mapule that you want to be out of the team.

00:12:43 Mary-Ann Jones

But I also believe that me and you as a team, I was supporting you for months on here.

00:12:48 Mary-Ann Jones

But I cannot continue on the way that we are continuing on as a team leader with somebody in my team.

00:12:57 Mary-Ann Jones

Another question I want to ask you because you're in.

00:13:00 Mary-Ann Jones

In my team, on my team, in our team, are you having meeting with the team?

00:13:06 Mary-Ann Jones

Are you scheduling meetings with the teams in the department?

00:13:12 Wilbur Matthee

So to be honest, yes.

00:13:15 Mary-Ann Jones

Yeah.

00:13:15 Mary-Ann Jones

00:14:00 Mary-Ann Jones

Should it be a union meeting or is it just a nice chill department kind of meeting?

00:14:06 Mary-Ann Jones

Should we then cancel the.

00:14:09 Mary-Ann Jones

These chilling sessions and things that we're having?

00:14:12 Mary-Ann Jones

What is the effect of this at the end of the day?

00:14:19 Wilbur Matthee

So the team discussion on the work from Home policy was a meeting that I created prior to sending this email to Mapule requesting for the policy.

00:14:33 Wilbur Matthee

It was just a short meeting that we had to discuss on what we want to put in on that.

00:14:40 Wilbur Matthee

On the email and stuff.

00:14:41 Wilbur Matthee

Wasn't a long meeting.

00:14:44 Wilbur Matthee

Yeah.

00:14:45 Mary-Ann Jones

So was that put on the timesheets of the team?

00:14:48 Mary-Ann Jones

Did I approve that?

00:14:50 Mary-Ann Jones

Was it now in October or September?

00:14:52 Wilbur Matthee

It was in a second.

00:15:00 Wilbur Matthee

Ninth month in September.

00:15:03 Mary-Ann Jones
September.

00:15:04 Mary-Ann Jones
Was that put on the time sheet?

00:15:05 Mary-Ann Jones
The time?

00:15:06 Wilbur Matthee
No.

00:15:06 Wilbur Matthee
No, I don't.

00:15:08 Mary-Ann Jones
Don't think so.

00:15:09 Mary-Ann Jones
Okay.

00:15:10 Mary-Ann Jones
Don't think so.

00:15:12 Mary-Ann Jones
Okay.

00:15:13 Mary-Ann Jones
Because if I did approve that, then that is so wrong.

00:15:16 Mary-Ann Jones
If I approve that time on that thingy timesheet.

00:15:20 Mary-Ann Jones
If it was something that untoward.

00:15:28 Mary-Ann Jones
Wilbur, I think when you come into office we need to have a person face to face.

00:15:34 Mary-Ann Jones
I like first to face to face in such instances.

00:20:32 Mary-Ann Jones
Whatever.

00:20:33 Mary-Ann Jones
We haven't spoken since we've been in the office there.

00:20:36 Mary-Ann Jones
I think it's time for us to sit and face to face.

00:20:41 Mary-Ann Jones
Besides, before or without Mapule them, if you decide you're still gonna go there, yes, you go there.

00:20:48 Mary-Ann Jones
But I think it's important that we must have some other, healthier something going on.

00:20:56 Mary-Ann Jones
But you do have the recording as proof if.

00:20:58 Mary-Ann Jones
If you need it.

00:21:00 Mary-Ann Jones
So, Wolba, I will chat to you later again.

00:21:03 Mary-Ann Jones
I did call you because I felt what was happening was a bit uncooled for you.

00:21:13 Mary-Ann Jones
She wanted clarity and I'm glad that somebody in the chat said maybe you must rediscuss it with your team leader then.

00:21:21 Mary-Ann Jones
So.

00:21:23 Mary-Ann Jones
Yeah, I did appreciate that.

00:21:26 Mary-Ann Jones
So I will speak to you again this afternoon.

ANNEXURE

"P"

Formal grievance against Mary-Ann Jones (malicious intent, gaslighting, retaliation,
orchestrated provocation, institutional silence)

Date: 2 November 2025

*Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*



Formal Grievance - Evidence of Malicious Intent, Constructive Dismissal, and Hostile Work Environment by Mary-Ann Jones

1 message

Wilbur Matthee <Wilbur.Matthee@medscheme.co.za> Sun, 02 Nov 2025 at 22:28
To: Cheryl-Dawn Modern <Cheryl-DawnM@afrocentrichealth.com>, Sipokazi Makaula <Sipokazi.Makaula@afrocentrichealth.com>, Thando Mandondo <ThandoM@medscheme.co.za>, Tebogo Dikobe <Tebogo.Dikobe@afrocentrichealth.com>
Cc: Wilbur William Matthee <wilburmat@gmail.com>

Dear HR Department and Union,

I am writing to lodge an urgent formal grievance against my team leader, Mary-Ann Jones. A review of the attached transcript from 31 October 2025 (SPK1=Mary-Ann SPK2 Wilbur) reveals a calculated pattern of behavior designed to undermine, isolate, and provoke me, creating an intolerable work environment with the apparent intent of forcing my resignation.

This is not a simple personality conflict. The evidence points to malicious intent and a strategic effort to build a false case against me.

Evidence of Malicious and Bad-Faith Conduct:

1. Strategic Entrapment and "Double Binds":
 - My team leader creates no-win situations. When I show initiative (e.g., the CNBR referral), I am accused of "undermining" her and operating in secrecy, even when I proactively correct the oversight. When I seek clarity from peers on a sudden, unilateral change to my work arrangement, I am accused of "creating noise" and failing to "respect authority." This is a deliberate tactic to ensure I am always in the wrong.
2. Systematic Gaslighting:
 - She directly denies factual reality. In the transcript, I state I informed her of the CNBR issue via a follow-up email, which she explicitly denies ("No, you didn't"). This is a clear attempt to destabilize my perception of events and erode my confidence.
3. Weaponizing Past Resolved Issues:
 - She inappropriately and maliciously referenced a past, resolved grievance I lodged, using it to suggest a pattern of behavior on my part which she can do to now by opening a case of harassment against me. This is an obvious attempt to pre-emptively discredit me and intimidate me from raising current valid concerns.
4. Orchestrated Provocation and Narrative Building:

- The sudden, out-of-hours demand to return to the office—issued only to me, contrary to agreed-upon plans—appears designed to provoke a reaction. My reasonable efforts to understand this directive were then used as the "evidence" to support her accusations of insubordination. She is actively manufacturing incidents to support a false narrative.

5. Overt Campaign for My Removal:

- She repeatedly and unprompted suggested I "ask Mapule to be removed from the team," stating our dynamic is "unhealthy" and that she "cannot continue." Given that she is the source of the conflict, these statements are not constructive but are a clear indication of her desired outcome: my removal from her team, by any means necessary.

6. Retaliation and Mockery of prior Complaint:

.Mary-Ann referenced the harassment complaint I previously lodged, and said: "You made a case of harassment against me, now I can make a case of harassment against you. You misspelled my name in an email—go look." Given that we communicate via email multiple times daily, this comment was delivered in a manner that was dismissive and condescending. This threaten a counter-complaint appears retaliatory, and it served to belittle and undermine my prior legitimate complaint rather than address it confidentially and professionally.

Conclusion and Urgent Request:

The cumulative effect of these actions is a textbook hostile work environment. The pattern of entrapment, gaslighting, and provocation indicates bad faith and malicious intent that goes far beyond poor management. I am experiencing significant stress and anxiety, and I have a genuine fear that my position is being deliberately made untenable to facilitate a constructive dismissal.

I therefore demand that HR:

1. Launch an immediate and formal investigation into the conduct of Mary-Ann Jones, focusing on the patterns of malicious behavior and intent outlined above.
2. Take immediate interim measures to remove me from her direct authority pending the outcome of the investigation.
3. Provide me with a written guarantee against any form of retaliation.
4. Confirm in writing that the unilateral change to my work arrangement is suspended until a full and fair consultation is conducted.

This situation is critical. I have attached the transcript as evidence and am prepared to provide all further documentation. I expect your immediate response to arrange a meeting.

Sincerely,



Sr. Wilbur W. Matthee

(Nurse Specialist PGrad HSM, Btech Gen Pn (Midw,Psych,Comm))

Case Manager: GEMS Active Disease Risk Management

- +27 12 466 1332
- wilbur.matthee@medscheme.co.za
- The Boulevard Building F & G, Searle street, Woodstock,7925
- www.medscheme.com



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transcript1.pdf

136 KB

ANNEXURE

"Q"

Executive escalation to leadership (32-page evidence bundle, requests for paid leave, financial settlement, written guarantee of non-retaliation)

Date: 4 December 2025

*Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*

RE: Urgent intervention required

1 message

Monwabisi Kula <Monwabisi.Kula@afrocentrichealth.com>

Tue, 09 Dec 2025 at 10

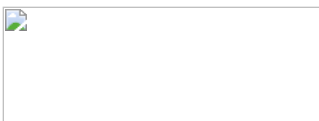
To: Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>, Gerald Van Wyk <Gerald.VanWyk@afrocentrichealth.com>, Lindiwe Miyambu <Lindiwe.Miyambu@afrocentrichealth.com>, Andrew Schwulst <Andrew.Schwulst@medscheme.co.za>, Aklaaq Mahmood <aklaaqm@afrocentrichealth.com>
Cc: wilburmat@gmail.com <wilburmat@gmail.com>

Dear Wilbur,

Thank you for raising the below concerns with us , this mail serves to confirm receipt of your escalation.

While we get the urgency you're highlighting, please also appreciate that your note is quite lengthy, with 32 attachments annexed to it, so please allow us sufficient time to investigate the matter comprehensively and give you feedback as soon as possible. With the festive break upon us, I can realistically say you will get interim feedback during January.

Regards,



Monwabisi Kula

Chief Risk Officer

Office: +27 11 671 6447 | Mobile: +27 72 690 2140

Monwabisi.Kula@afrocentrichealth.com

37 Conrad St, Florida North, Roodepoort, 1709 | PO Box 1101, Florida Glen, 1709

www.afrocentric.za.com



From: Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>

Sent: Thursday, December 4, 2025 12:29 PM

To: Gerald Van Wyk <Gerald.VanWyk@afrocentrichealth.com>; Lindiwe Miyambu <Lindiwe.Miyambu@afrocentrichealth.com>; Monwabisi Kula <Monwabisi.Kula@afrocentrichealth.com>; Andrew Schwulst <Andrew.Schwulst@medscheme.co.za>; Aklaaq Mahmood <aklaaqm@afrocentrichealth.com>

Cc: wilburmat@gmail.com

Subject: Urgent intervention required

Importance: High

Dear Executive Leadership,

This correspondence serves as my final and good-faith attempt to resolve, internally and without escalation, the severe and ongoing harm I have been subjected to as a direct result of the company's repeated institutional failures.

Over the past months, I have experienced serious and documented misconduct, including:

1. Unlawful salary deductions exceeding 40% of my monthly income without my consent or consultation. This during a motivated mental health admission, awaiting a bed informed by the HRBP that the deduction was taken on the principle of no work no pay which did not require consultation.

2. Continuous intimidation, retaliation, and victimisation by my direct manager despite a lodged grievance. After a formal request to be temporarily removed from her team. No action.

3. Awaiting feedback from manager as discrepancies found in up to 20% reduction in some cases of my monthly actual performance metrics that led to poor outcomes even missed opportunities in performance bonuses.

4. Prolonged silence and inaction from HR and senior leadership and union despite formal complaints. Currently +30days to date no response.

These actions have caused significant financial strain, forced me into a cycle of unexpected debt which i am struggling to manage, and resulted in severe psychological distress. My capacity to work in an environment that continues to permit such conduct has reached its limit.

Given the seriousness of these failures, I am formally requesting the restorative measures:

1. Paid Recuperation Leave & Guaranteed Reintegration

A period of fully paid special leave, allowing adequate time for psychological recovery.

Upon return, a permanent and immediate transfer to a new department.

2. Financial Settlement Determined by Risk & Compliance

I request a comprehensive financial settlement, independently assessed by the Head of Risk & Compliance, covering:

Financial losses caused by the unlawful deduction.

Debt incurred as a direct result of these actions.

Compensation for emotional distress, harassment, and institutional negligence.

The company's full legal exposure for:

Unfair labour practice

Discrimination

Failure to provide a safe working environment

Reputational and operational risks should the matter escalate externally.

3. Institutional Commitments

A written, irrevocable guarantee of no retaliation, present or future.

Next Steps

I am willing to meet with this leadership group and a neutral mediator within 14 days from date 04 December 2025.

If a signed agreement in principle is not provided by 14 working days, I will immediately proceed with the following simultaneous actions: As I feel I have exhausted all avenues this far to have the incidents resolved and this is my final resort.

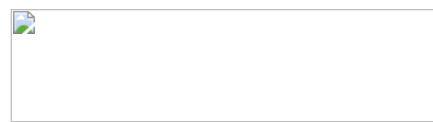
1. File cases with the CCMA for unfair labour practices, and discrimination.
2. Lodge a formal complaint with the Department of Employment and Labour regarding the unlawful salary deduction.
3. Submit a complaint to the South African Human Rights Commission.

I am really at my breaking point now no motivation to come to work.

All evidence is already compiled and ready for submission.

I trust that the company will choose a constructive, internal resolution over a protracted and public legal dispute. I await your urgent response.

Kind regards,

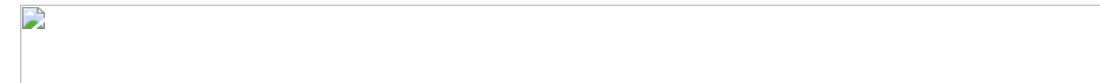


Sr. Wilbur W. Matthee

(Nurse Specialist PGrad HSM, Btech Gen Pn (Midw,Psych,Comm))

Case Manager: GEMS Active Disease Risk Management

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ANNEXURE

"R"

Respondent's opposing affidavit in CCMA Case WECT23564-25 (personal attacks on
Applicant's psychiatric condition and professional character; demand for costs) — evidence of
bad faith and disability discrimination under oath

Date: 10 December 2025

*Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*

IN THE COMMISSION FOR CONCILIATION MEDIATION AND ARBITRATION

Case Ref. Number: **WECT23564-25**

In the matter between

WILBUR MATTHEE

Applicant

And

MEDSCHEME HOLDINGS

Respondent

NOTICE OF OPPOSITION OF CONDONATION APPLICATION

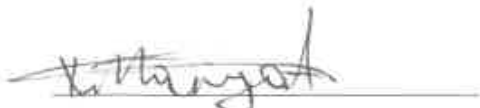
Your condonation application dated **05 December 2025**, is hereby acknowledged and refers.

In response to the condonation application by the applicant **BE PLEASED TO TAKE NOTE THAT** the Respondent hereby files its Notice of Opposition.

PLEASE TAKE FURTHER NOTE THAT the answering affidavit of **Mr XOLILE VINCENT MANYATI** will be used in support of this notice of opposition.

The respondent will accept all documents in this matter at the following email address:
IR@afrocentrichealth.com.

Signed at **Johannesburg** on the **10th** day of **December 2025**.



XOLILE VINCENT MANYATI

Respondent representative

To: Wilbur Matthee
Per email: wilburmat@gmail.com

And to: Case Management Officer
CCMA
Per email: CMS@CCMA.org.za

**IN THE COMMISSION FOR CONCILIATION MEDIATION AND ARBITRATION
HELD AT CAPE TOWN**

Case Ref. Number: **WECT23564-25**

In the matter between

WILBUR MATTHEE

APPLICANT

And

MEDSCHEME HOLDINGS

RESPONDENT

RESPONDENT'S ANSWERING AFFIDAVIT

I the undersigned **Mr. Xolile Vincent Manyathi** do hereby make an oath and say:

1. I am an adult male and the respondent in this matter. I am duly authorized to make this affidavit as a Senior Employee Relations Specialist employed by Afrocentric Health (Pty) Ltd, the holding company.

Never have I met, worked, or discuss, be observed by this individual



2. I am duly authorized to depose of this affidavit. The facts contained herein are both true and correct and fall within my personal knowledge, unless otherwise indicated or the contrary appears from the facts.

WITNESS

3. The applicant in this matter is **Mr. Wilbur Matthee**, who is employed as a care coordinator at Gems business unit under Medscheme Holdings.

WITNESS

4. LEGAL FRAMEWORK:

- 4.1 In terms of the Labour Relations Act 66 of 1995, as amended, if there is a dispute about unfair labour practice, the employee may refer the dispute in writing, however the referral must be made within **90 days** of the date of the act or omission which allegedly constitutes the unfair labour practice. If the employee shows good cause at any time, the council or the commission may permit the employee to refer the dispute after the 90-day time limit has expired.

4.2 Section 191 (2) stipulates that if the applicant shows good cause at any time, the council or CCMA may permit the applicant to refer the dispute after the 90-day time limit has expired. In the case of **Shoprite Checkers (Pty) Ltd CCMA & Others (1998) 19 ILJ 892 (LC) p 21-22, (1998) 5 BLLR 510 (LC)** the honourable judge summarised the law in this regard as follows: "Section 191(2) enjoins an applicant to show good cause before a commission may permit her to refer the dispute after the time limit has expired. The section makes it clear that condonation is not there merely for the asking. The applicant must tender an adequate explanation for the delay. The Commissioner must also consider this explanation. Due regard must be had to the other generally accepted requirements for the grant of condonation as contemplated in the words good cause...."

4.3 From the above, it is clear that before the commissioner can grant condonation, the applicant must furnish **justifiable reasons** for the late request. The fundamental question is, therefore, whether the applicant has good grounds for not referring the dispute in time and does his explanation satisfy the factors as mentioned below.

4.4 To that end, in the case of **Dlamini v Progress Housekeeping CC (2001) 22 ILJ 1024 (CCMA) 1028** the commissioner indicated that in determining whether to grant condonation or not the following factors needs to be taken into account: the **degree of lateness**, the **degree of fault**, the **reasonableness of the explanation**, the **prejudice to the parties**, the **prospect of success in the main dispute**. The aforementioned factors were re-iterated in the case of **Job L. Boshop v Mac Donalds Transport NC5139 7 July 2000 CCMA**.

5. BACKGROUND:

5.1 The Applicant is employed as the care coordinator in Medscheme and is based at the employer's offices in Cape Town.

5.2 On the or about **July 2025** the Applicant was absent due to health reasons, the employees leave days got exhausted due to this prolonged absence and because of previous intermittent sick leaves taken. All the employees' leave days were exhausted, including sick leave and annual leave were exhausted at the time.

5.3 The Respondent's process is that before it can apply a no work no pay principle, it firstly exhausts the employees annual leave in leu of sick leave when sick leave is finished, it also assists employees with special leave which in this case it also did in order to accommodate the employee. This is the case in this matter where the respondent firstly used annual leave of the

employee in lieu of sick leave absence. The employee was not paid as he was not at work and had no leave days, however this was not a deduction, but an employee did not render any services and therefore was not paid rightfully so.

6. DEGREE OF LATENESS:

6.1 Disputes of Unfair labour practice must be referred to CCMA within 90 days of the date of the act or omission.

6.2 It is interesting to note that the Applicant deliberately leaves out the information of how late his referral is. This is consistent with his untruthful character, he is aware how long is his matter out of the prescribed time frames of the CCMA, yet he does not state that information. The Applicant's referral is more than 40 days late; I submit that this is excessive, especially for someone who had all the time in the world to refer the matter to the CCMA.

WITNESS

6.3 According to the Applicant he got to be aware of what he needed to do in November or December due to mental issues, but the employee has been working all along since August without any mental issues, but the applicant failed to explain how he got to know of his oversight 40 days later. The employee was addressed by his management team and HR on the issue of unpaid leave. It is also difficult to accept the argument of mental issues from the applicant, as he raised the issue of the alleged victimization in August or September, maybe that was also an oversight. So, when the matter of victimisation was dealt with in August or September why did the employee not refer the matter of unfair deduction then, as clearly there were no mental problems then. The mental issues are an excuse and this whole referral is an afterthought matter. The applicant's failure to take accountability of his lateness and lack of proper pleadings about the degree of lateness on his part, shows his lack of regard for the commission, its processes, and resources.

WITNESS

6.4 The rationale for referring the matter within the required time frames is *inter alia*, to avoid potential prejudice that the Applicant may suffer at the hands of the Respondent. In this case it cannot be argued that the delay was occasioned by the Respondent but by the sloppiness and lack of regard of the most reasonable periods placed by the legislature.

WITNESS

7 REASON FOR LATENESS:

7.1 It is the employer's contention that the Applicant has not provided a compelling explanation for the lateness of his referral to the CCMA. It is interesting to note that the Applicant in his referral claims that he only became aware of what he needed to do in November 2025 but that is not

true as the employee raised another matter with Cheryl-Dawn who is a HR business partner in August/September. Applicant still does not explain how he became aware of his rights in this matter or the lack thereof, this is difficult to accept as the truth as the applicant is the member of the union and if he needed advice he would have gotten it from union representative. It is further interesting that the employee chose not to include this information in his referral affidavit to the CCMA but just makes references that he was ignored by the company.

7.2 The Respondent made the Applicant aware of the reasons why he was not fully paid on the **23rd of July 2025**, when the applicant enquired about the shortfall on his salary, however the applicant only referred the matter to CCMA on the **5th of December 2025** which was more than 40 days late and more than 120 days later from the date of the act or omission.

7.3 The reasons advanced by the Applicant are not truthful, and for that reason alone the CCMA must not entertain this application. In the matter between **Molla v Shal & NO others (2007) 28 ILJ 1028 (LAC)**: Zondo JP as he then was with McCall AJA and Kruger AJA. Concurring held: [34] "I do not have the slightest hesitation in concluding that this is a case where the period of delay is excessive and the appellant's purported explanation for the delay is no explanation at all. If ever there was a case in which one can conclude that good cause has not been shown for condonation without considering the prospects of success, then this is the one. Where in an application for condonation, the delay is excessive and no explanation has been given for that delay or an "explanation" has been given but such explanation amounts to no explanation at all, I do not think it is necessary to consider the prospects of success."

7.4 The Applicant has failed to proffer plausible reasons for the delay. On this ground alone the application should be dismissed, and this abuse of the CCMA services should be condemned with the contempt that it deserves.

WITNESS

8 PROSPECTS OF SUCCESS:

8.1 I submit the Applicant has no prospects of success to the main case. The Respondent acted in line with the law and its own policy that when an employee is sick it will utilize sick leave, once sick leave is exhausted, and employee continues to be absent it will utilize annual leave in lieu of sick leave and once all leaves are exhausted it will then apply unpaid leave. There is nothing illegal about that and the lack of explanation from the applicant as to how the respondent acted unlawfully shows clearly that the employee does not know what he is referring to. The contractual obligation between the respondent and the applicant is that the employer will pay an employee after the employee has rendered his services and not merely for being an employee. It is very clear on what the Applicant did, that to breach the rule as he did, the

consequences thereof of are a dismissal. There is no evidence nor reasons that the applicant will provide to show that he was supposed to be paid a full salary for the month of **July**.

WITNESS

- 8.2 The Applicant's submission lacks explanation on the prospect of success, and a mere claim of unfair labour practice cannot be seen as a prospect of success, and the matter has been wrongly referred as well.
- 8.3 The applicant did not even follow an internal process of formally logging a grievance regarding the alleged unlawful deduction after it was addressed in August but decided to only communicate about it in November which is after the prescription period and then on the 4th of December wrote to the company executive members just before he referred the matter to the CCMA. The respondent submits that this is just a ploy by an applicant to force the company to pay him salary that he is not legally entitled to.
- 8.4 The respondent submits that it did everything to help the employee with his absence to an extent that there was special leave approved for him at the time because the respondent was acting as a responsible employer. The issue of unpaid leave was addressed by his management team (Mapule Mashele) who is the senior manager and HR team that supports the Gems business unit. The respondent will argue that the applicant was addressed on the issue of unpaid leave several times.
- 8.5 The respondent submits that the applicant has really no prospects of success as his reasons given around victimization and intimidation have no relevance but are used to sensationalize his dispute and have nothing to do with main dispute. It further makes it difficult even to understand as to what the real dispute of the applicant is.
- 8.6 The applicant has failed to discharge compelling reasons that are convincing that he has prospects of success other than mere accusations of victimization and intimidation. Therefore, the responded submits that the condonation application of the applicant should be dismissed as there are no prospects of success should the matter go to the main case. The applicant has not even demonstrated that he has a reasonable chance of succeeding nor has he provided facts and evidence to support his claims.

WITNESS

9 PREJUDICE:

9.1 Having regard to submissions made above it is my view that the Respondent will suffer severe prejudice should the application be granted because the Respondent will be required to use resources that are Joburg based to fly to Cape Town to deal with this unsubstantiated dispute, considering that the delay has been caused by the applicant in this instance. Respondents will also have to take management and HR out of the office to bring them to the CCMA as witnesses for something that the applicant is aware of why he was not paid and that management assisted him to not be further negatively affected by his absence through approving special leave for him.

9.2 It would be unfair to now hold the Respondent to ransom given that the Applicant has erred in this regard.

WITNESS

9.3 It will be prejudicial for the respondent to be expected to go through the main case without any good cause shown by the applicant in terms of the prospects of success in the main.

In conclusion:

The Applicant's affidavit is riddled with untruthful submissions, meaning he has not been truthful or forthcoming and expects the CCMA to grant him condonation even though his application is just a mere ask without factual basis. This is just a fishing expedition and abuse of the system. It is therefore my considered view that this application should be dismissed with cost.

WITNESS

The applicant will not succeed in the main case if the reasons is just a claim of unlawful deduction and victimisation.

It is further submitted that the Applicant does not have prospects of success in the main application as it the Respondent's view that there is no unfair labour practice that the applicant has been subjected to.

It is further submitted by the respondent that whether there is good cause shown on the lateness but that cannot be the only determining factor in granting of condonation, but the applicant should satisfy all the elements which I believe he has not given sufficient evidence or explanation in terms of prospects of success and prejudice.

It is clear that the applicant will not succeed in the main if the reasons is just a claim of unlawful deduction or invalid no work no pay claim.

~~Intarjal~~

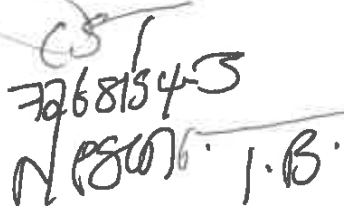
Deponent

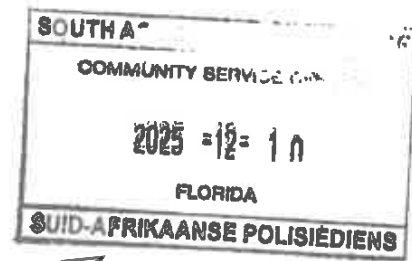
I certify that on this 10 day of December 2025, the deponent signed the declaration in my presence and declared that:

- a) I know and understand the contents hereof;
- b) I have no objection to taking this oath;
- c) I consider this oath to be binding on my conscience and uttered the words: "I swear that the contents of the declaration are true, so help me God."

WITNESS


Commissioner of Oaths


CS
7268134-3
Nelson J.B.



Statement made under oath at a police station. Making a false claim to the state

WITNESS

**IN THE COMMISSION FOR CONCILIATION MEDIATION AND ARBITRATION HELD AT
CAPE TOWN**

CASE NO.: **WEC123564-25**

In the matter between:

WILBUR MATTHEE **APPLICANT**

and

MEDSCHEME HOLDINGS (A Division of AfroCentric Health)
RESPONDENT

**APPLICANT'S REPLYING AFFIDAVIT TO THE RESPONDENT'S OPPOSITION TO
CONDONATION**

I, the undersigned, WILBUR WILLIAM MATTHEE, do hereby make oath and state:

1. INTRODUCTION AND AUTHORITY

1.1. I am an adult male, the Applicant in this matter, and a Health Services Management Nurse Specialist (Care Manager) employed by the Respondent at its GEMS business unit in Cape Town.

1.2. The facts herein are within my personal knowledge, true and correct, unless otherwise stated. I have personal and direct knowledge of all events and communications referenced.

1.3. I am opposing the Respondent's application to dismiss my condonation request and will demonstrate that: (a) the harm was continuous, placing my referral within time; and alternatively, (b) compelling good cause exists for condonation.

2. CORRECTION OF FACTUAL INACCURACIES AND UNFOUNDED ATTACKS

2.1. I must correct material errors and reject unfounded attacks in the affidavit of Mr. Manyathi, which undermine its credibility.

2.2. **Correction of My Position:** I am not a "care coordinator" as dismissively stated, but a **Health Services Management Nurse Specialist (Care Manager)**. This reflects a pattern of the Respondent's **inattention to accuracy**.

2.3. Rejection of the Irrelevant "Dismissal" Argument: My case concerns unfair labour practices, not dismissal. The Respondent's unsolicited argument that my actions warranted dismissal is highly prejudicial and attempts to improperly influence the Commissioner.

2.4. Rejection of the Inappropriate Prayer for Costs: The demand for costs in a CCMA condonation application is punitive and underscores the Respondent's aggressive posture to intimidate me, given my documented financial distress.

2.5. Rejection of Character Attacks: I categorically reject allegations of an "untruthful character." My submissions are backed by documentary evidence. The Respondent's personal attacks are a strategy to distract from their lack of substantive defence.

SECTION 3: THE RESPONDENT'S AFFIDAVIT AS ESCALATING VICTIMISATION & INSTITUTIONAL MALICE

3.1 Aggravated Attack on Protected Characteristics & Medical Condition

The Applicant is compelled to address the content and tone of the Respondent's Affidavit by Mr. Xollie Vincent Manyathi. This document does not constitute a legal defence but represents a continuation and escalation of the very unfair labour practices at issue, providing compelling grounds for enhanced, punitive damages.

3.1.1 The affidavit deliberately and maliciously dismisses my medically documented psychiatric condition—a direct consequence of the Respondent's own discriminatory and victimising conduct—as a cynical "excuse" and "afterthought". This is not legal argument; it is a direct attack on a protected characteristic (disability/medical condition) that compounds the initial discrimination.

3.1.2 By weaponising my mental health crisis—caused by their actions—against me in a formal legal proceeding, the Respondent demonstrates institutional contempt for employee wellbeing and a shocking disregard for the duty of care. This transforms a health condition into a tool for further victimisation.

3.2 Financial Intimidation & Weaponisation of Process

3.2.1 The Respondent's aggressive and unwarranted demand for costs in a routine condonation application, made with full knowledge of my documented financial destitution (as detailed in previous communications), serves no legitimate legal purpose.

3.2.2 This demand is a deliberate act of financial intimidation, calculated to exert psychological pressure and deter me from pursuing my rights. It exploits my vulnerable financial position—a position created by the Respondent's own unlawful deductions—to threaten further ruin.

3.3 Bad Faith, Personal Attacks & Abuse of Process

3.3.1 The affidavit is riddled with gratuitous personal attacks, labelling me as “untruthful” and characterising my application as an “abuse of the CCMA services”. These are not legal submissions but ad hominem assaults intended to poison the well and discredit me personally.

3.3.2 This demonstrates the Respondent's utter lack of remorse, systemic bad faith, and commitment to continued hostility. It confirms that the initial misconduct was not an aberration but reflective of an organisational culture that attacks rather than addresses complaints.

3.3.3 The affidavit itself thereby eliminates any conceivable possibility of restoring the employment relationship. It proves the victimisation is not historical but ongoing and institutionalised, perpetrated now through the Respondent's legal representatives.

3.4 This Affidavit as Independent Grounds for Maximum & Punitive Damages

3.4.1 The Respondent's choice to respond to serious discrimination allegations with further aggression in a sworn affidavit constitutes an independent, post-referral unfair labour practice (victimisation). It is a conscious decision to escalate rather than remediate.

3.4.2 This conduct warrants a significant punitive damages premium in any award. It proves the need for a deterrent sanction to signal that using legal process to further victimise complainants is unacceptable.

3.4.3 The Commissioner is urged to consider this affidavit not merely as a defence to condonation, but as powerful evidence of the Respondent's character and the imperative for a maximum compensatory and punitive award.

4. THE TRUE NATURE OF THE DISPUTE: A CONTINUOUS PATTERN OF UNFAIR LABOUR PRACTICES

4.1. This dispute encompasses multiple, interconnected unfair labour practices forming a continuous course of conduct from July 2025 to December 2025:

- a. Unfair Labour Practice – Unlawful Deduction & Failure to Accommodate (July 2025).
- b. Unfair Discrimination & Harassment (the "Wilma" incident, 23 September 2025).
- c. Victimisation & Retaliation (doubled unpaid leave, punitive management).
- d. Institutional Bad Faith & Procedural Failure (30-day silence on grievances, whitewashing of complaints, bad-faith de-escalation).

5. THE UNLAWFUL NATURE OF THE JULY 2025 SALARY DEDUCTION

5.1. The Respondent misrepresents the July 2025 incident as a lawful "no work, no pay" application. In reality, its implementation was procedurally unfair and discriminatory.

5.2. Failure to Consult and Lack of Procedural Fairness: The Respondent deducted over 40% of my salary without any prior consultation, despite my explicit request to stagger the amount to prevent destitution. This refusal to engage constituted a lack of procedural fairness.

5.3. Failure to Provide Reasonable Accommodation: My absence was due to a documented psychiatric condition. The employer has a duty to make reasonable accommodation. My request to stagger the deduction was a minimal adjustment. The refusal, while I was in a vulnerable post-hospitalization state, constitutes discrimination based on my medical condition.

5.4. Derogatory Comments and Victimization: The manager's comment that "The Respondent is not a loan company" followed by I "would not feel the missing chunk... because I receive online deliveries weekly" was derogatory and proves the refusal to accommodate was motivated by malicious intent, converting a deduction into an act of victimisation.

6. THE CONTINUOUS HARM AND INSTITUTIONAL BAD FAITH THAT FOLLOWED

6.1. The July incident established a pattern of unfairness that continued unabated, demonstrating a continuous course of conduct:

- a. September 2025: I experienced public discrimination (the "Wilma" incident) and immediate retaliation (doubled unpaid leave).
- b. October 2025: Senior management whitewashed the discrimination complaint as a "typo," HR and union accept demonstrating institutional bad faith.
- c. November 2025: I filed a formal grievance about the hostile environment. The Respondent met this with 30 days of institutional silence. HR and union.
- d. Early December 2025 – The Escalation and De-escalation:
 - i. On 4 December 2025, after 30 days of silence from HR and the union followed by +20 days silence by senior management, I escalated the entire matter to executive leadership.
 - ii. The executive leadership de-escalated the matter back to the very department and manager accused of causing the harm.
 - iii. Faced with being returned to the source of the problem, I had no reasonable alternative and filed the CCMA referral.
 - iv. The de-escalated department contacted me to schedule a meeting notifying them of the case is now with the CCMA. I stipulated basic safeguards: a clear agenda and a recorded meeting to ensure transparency.
 - v. The department refused these basic terms, creating a procedural deadlock.

vi. I therefore formally notified the department that I could not attend without safeguards and would seek oversight elsewhere.

vii. As the next logical step, and true to my word, I formally notified the Board's Social & Ethics and Risk Committees the following day. This was a good-faith report of a material governance failure.

6.2. This sequence proves my actions were sequential, reasonable, and forced by the Respondent's conduct. The "delay" encompasses the period I was cycled through this broken internal process.

7. PRIMARY LEGAL ARGUMENT: THE HARM WAS CONTINUOUS, RESETTING THE REFERRAL CLOCK

7.1. Legal Principle: Where unfair labour practices constitute a continuing wrong, the 90-day period for referral resets for as long as the conduct persists. The last act in the continuum sets the clock *Gauteng Department of Health v Tsomakae* (GAJB 5554-16) [2017].

7.2. Application: The Respondent's misconduct was a continuous course of conduct that only culminated with the institutional bad faith and refusal to engage transparently in early December 2025. My referral is therefore within 90 days of the last confirming act of the continuing wrong.

8. ALTERNATIVE ARGUMENT: COMPELLING GOOD CAUSE FOR CONDONATION

Should the Commissioner find the continuous harm principle not fully applicable, I demonstrate compelling good cause.

8.1. Reason for Delay – Exhaustion of Remedies: Any delay was due to my good-faith exhaustion of every internal and governance channel, a process systematically thwarted by the Respondent. The CCMA referral was a last resort made promptly once this became clear.

8.2. Prospects of Success are Overwhelming: I possess direct evidence for each allegation (payslips, screenshots, emails, medical certificates). The Respondent's affidavit avoids engaging with this substance.

8.3. Prejudice: The prejudice to me—destitution, health crisis—if denied a hearing is catastrophic. Any administrative inconvenience to the Respondent is negligible.

9. THE RESPONDENT'S CLAIM OF "PREJUDICE" IS WITHOUT MERIT AND DEMONSTRATES ITS BAD FAITH

9.1. The Respondent claims in its affidavit (paragraph 9.1) that it will suffer "severe prejudice" should my application proceed because it would need to use "resources that are Joburg based to fly to Cape Town to deal with this unsubstantiated dispute."

9.2. This claim is disingenuous and reveals the Respondent's true disregard for the seriousness of this matter and the imbalance of power:

- a. ****Relative Prejudice is Wholly Mismatched:**** The Respondent, a large JSE-listed corporate entity, complains of the administrative and financial inconvenience of travel. I, an individual employee, am facing ****actual destitution, inability to afford food and medication, and severe health deterioration****. To equate corporate travel logistics with the threat to an individual's basic survival and health is not only irrational but morally indefensible. Any prejudice to the Respondent is purely financial and administrative; the prejudice to me is existential.
- b. ****The Respondent Created This Dispute Through Its Own Conduct:**** The Respondent's suggestion that it is being held to ransom is an inversion of reality. The dispute exists ****solely because of the Respondent's own actions****: its unlawful deduction, its discriminatory conduct, its victimisation, and its systemic failure to address my grievances. Having created the harm, it cannot now claim prejudice when called to account for it.
- c. ****Travel is a Normal Business Cost in a National Company:**** The Respondent, as a national healthcare administrator, operates across the country. Requiring managers to travel for a formal legal proceeding regarding serious misconduct within its ranks

is a normal cost of doing business and upholding the law. Framing this as an undue burden is an admission that it views its own legal and ethical accountability as an optional inconvenience.

D. ****Modern Technology Provides Alternatives:**** Should travel genuinely be a concern, the Respondent could have explored or proposed virtual mediation or arbitration. Its failure to do so and immediate leap to claiming “severe prejudice” underscores its strategy to use cost as a barrier to justice.

- d. ****The Claim Undermines the CCMA’s Authority:**** Implicit in the Respondent’s argument is the suggestion that an employee in one city should be barred from justice if the employer’s head office is in another. This strikes at the heart of the CCMA’s national mandate to provide accessible dispute resolution for all employees, regardless of geography.

9.3. Conclusion on Prejudice: The Respondent’s claim of prejudice is not a valid legal objection but a tactical one, designed to portray me as an inconvenience rather than a wronged employee seeking redress. The overwhelming and documented prejudice lies with me. To deny condonation based on the Respondent’s purported travel costs would be a grave miscarriage of justice, prioritising corporate convenience over an individual’s right to be heard on matters of discrimination, victimisation, and survival.

10. CONCLUSION

10.1. My referral is, on the correct legal analysis, within time due to the continuous nature of the harm. Alternatively, more than sufficient good cause exists.

10.2. To dismiss this application on a technicality would sanction the Respondent's strategy of institutional bad faith and deny justice.

11. PRAYER FOR RELIEF

I respectfully request that the Honourable Commissioner:

- SIGNED at DELFT on this 11TH day of December 2025.

SWORN TO AND SIGNED BEFORE ME AT DELFT ON THIS 11th DAY OF
DECEMBER 2025, THE DEPONENT HAVING ACKNOWLEDGED THAT HE KNOWS AND
UNDERSTANDS THE CONTENTS OF THIS AFFIDAVIT, THAT HE HAS NO OBJECTION TO
TAKING THE PRESCRIBED OATH, AND THAT HE CONSIDERS THE OATH BINDING ON HIS
CONSCIENCE.

APPOINTMENT EXPIRES:



11 DEC 2025

ANNEXURE

"S"

Resignation letter citing opposing affidavit as final intolerable act — evidence of constructive dismissal

Date: 11 December 2025

*Mathee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*



FORMAL NOTICE OF RESIGNATION DUE TO AUTOMATICALLY UNFAIR CONSTRUCTIVE DISMISSAL

4 messages

Wilbur William Matthee <wilburmat@gmail.com>
To: lebohang.mpumlwana@afrocentrichealth.com
Cc: ferfam.bruno@gmail.com, alice@aliceleroux.co.za, Gretchen.veldman@afrocentrichealth.com, gerald.vanwyk@afrocentrichealth.com, munisinf@global.co.za
Bcc: Sr Wilbur William Matthee <wilburmat@gmail.com>

Fri, 12 Dec 2025 at 03:29

TO:
The Board of Directors (via Company Secretary)
AFROCENTRIC HEALTH (RF) (PTY) LTD
37 Conrad Road, Florida North, 1709
CC:
Mr./Ms. Gerald Van Wyk (Chief Executive Officer)
Chair of the Audit & Risk Committee
Chair of the Social & Ethics Committee
Group Human Resources Department
DATE: 11 December 2025

Dear Board of Directors,

I, Wilbur Matthee, hereby give notice of the immediate termination of my contract of employment with Medscheme Holdings, effective 12 December 2025.

This resignation is not voluntary. I am terminating my employment because the employer, Medscheme Holdings(A Division of AfroCentric Health), has rendered the continuation of the employment relationship irretrievably intolerable through its malicious, calculated, and institutional conduct.

This termination constitutes an Automatically Unfair Constructive Dismissal in terms of the Labour Relations Act (LRA).

1. Grounds for Intolerability: Institutional Malice and Victimisation

The conditions rendering my continued employment intolerable are proven by the employer's formal legal submission in the matter of Wilbur Matthee and Medscheme Holdings (CCMA Case Ref. Number: WECT23364-25), specifically the Respondent's Answering Affidavit signed under oath on 10 December 2025.

The Affidavit contains specific statements that constitute severe and ongoing occupational detriment, victimisation, and unfair labour practice, including:

Malicious Character Attacks: The Affidavit labels me as having an "untruthful character," engaging in a "fishing expedition," and acting with "sloppiness and lack of regard" for the Commission. This ad hominem attack, under oath, fundamentally destroys the trust and confidence necessary for any employment relationship.

Abuse of Process: The document constitutes an abuse of the CCMA's services by employing tactics designed to discredit and cause further financial harm, rather than engaging in a good faith legal defence.

Institutionalised Victimisation: The Affidavit explicitly dismisses my claims of victimisation and mental health issues as having "no relevance" and being used to "sensationalize" the dispute (Sections 6.3 and 8.5). This demonstrates that the victimisation is ongoing, officially sanctioned, and institutionalised through the Respondent's legal representation.

2. Legal Basis: Automatically Unfair Constructive Dismissal

The employer's conduct, as documented in the sworn Affidavit, makes this termination an Automatically Unfair Constructive Dismissal in terms of the following sections of the Labour Relations Act (LRA):

Section 186(1)(e): The employee terminated the contract of employment because the employer made continued employment intolerable.

Section 187(1)(h): The reason for the employer's conduct is directly linked to an act of retaliation against a Protected Disclosure (the P1 Material Risk Report and subsequent updates regarding discrimination and victimisation).

3. Conclusion and Formal Notice

The content of the Respondent's Opposing Affidavit eliminates any conceivable possibility of restoring the employment relationship. I consider myself to have been dismissed by the employer's malicious conduct.

I have already formally informed the Board of the governance failure related to this P1 risk and the ongoing victimisation. All further matters relating to this dispute, including the claim for maximum compensation and a demand for punitive costs (Attorney-and-Client scale), will be handled at the CCMA/Labour Court, and via my External Whistleblower disclosures to the JSE, FSCA, CMS, DEPARTMENT OF EMPLOYMENT & LABOUR - INSPECTION & ENFORCEMENT + B-BBEE, . Pending complete governance failure.

Yours sincerely,

Wilbur Matthee

Mail Delivery Subsystem <mailer-daemon@googlemail.com>
To: wilburmat@gmail.com

Fri, 12 Dec 2025 at 03:30



Address not found

Your message wasn't delivered to
Gretchen.veldman@afrocentrichealth.com
because the address couldn't be found, or is
unable to receive mail.

ANNEXURE

"T"

CPUT Postgraduate Diploma in Occupational Health Nursing acceptance letter — lost
opportunity due to financial sabotage by Respondents

Date: 2026

*Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*

Dear Applicant

**ANNEXURE: 2026 ACCEPTANCE
POSTGRADUATE DIPLOMA IN OCCUPATIONAL HEALTH NURSING**

Greetings on behalf of the Department of Nursing Science at the Cape Peninsula University of Technology (CPUT) and congratulations on your 2026 acceptance into the Post Graduate Diploma in Occupational Health Nursing.

Kindly note that this programme is offered face-to-face; it is not a distance learning course, and you will therefore be required to attend sessions on campus. This annexure provides important information regarding admission and orientation requirements to be met prior to your commencement of orientation at the Bellville Campus on Monday, 26 January 2026.

Should you accept the offer of study for this programme, kindly complete *Addendum D: Reply Slip and Personal Details* in full on page 14 and email to Ms Natalie Copeling at CopelingN@cput.ac.za within the stipulated timeframe. Failure to do so will result in your place being offered to the next suitable applicant. As per the South African Nursing Council (SANC) accreditation, only **60** students can be accommodated for this programme and this will be done on a first-come, first serve basis.

1. GENERAL INFORMATION

At CPUT, which is a University of Technology, students will be required to play an active role by using mobile learning throughout their studies, using their own data. It is therefore important that you have a device that allows you to connect to the Learner Management System (LMS) used to facilitate online sessions, specifically "My Classroom/Black Board Learning". Further to this, written assignments must be submitted in a typed format, and it is thus also necessary to have access to a device equipped with Microsoft Word, preferably a laptop.

2. COMMENCEMENT INFORMATION PART ONE

A **compulsory** orientation programme will take place from Monday, 26 January 2026 to Friday, 30 January 2026, commencing daily at 09:00.

PLEASE TAKE NOTE OF THE FOLLOWING STEPS TO BE TAKEN:

2.1 Step 1: Submission of Documents

Please submit hard copies of the documents below to the Department of Nursing Science between 26 January 2026 and 19 January 2026 (certification not older than three months):

- 2.1.1 Certified copy of ID (x 1)
- 2.1.2 Completed Application Form (Addendum A) for submission to SANC
- 2.1.3 Certified copy of 2025 SANC receipt as proof of registration with SANC as a professional nurse and midwife
- 2.1.4 Certified copy of Nursing Qualification(s) obtained (x 1)
- 2.1.5 Certified copy of National Senior Certificate (x 1)
- 2.1.6 Copy of 2025 Professional Indemnity (DENOSA, NEHAWU, SASOHN, etc.).
- 2.1.7 Certified copy of Marriage Certificate (if applicable) (x 1)
- 2.1.8 Completed CPUT Indemnity Form (Addendum B)
- 2.1.9 Completed Western Cape Government Indemnity Form (Addendum C)
- 2.1.10 Evidence of Hepatitis B Vaccination or Hepatitis B Immunity
- 2.1.11 Proof of payment of the SANC registration fee to register as a learner **payable to the university**; fees are subject to change therefore, kindly refer to the Government Gazette.

2.1.12 Upon confirmation of SANC FEES, please pay this amount into the CPUT account as indicated below:

Account Name: Cape Peninsula University of Technology
Bank Name: ABSA Bank
Branch: Public Sector Cape Town
Branch Code: 632005
Account Code (South Africans): 405 354 8487
International Students: 01 202 660521
Swift Code (for payment outside SA): ABSA ZA JJ (for payments outside South Africa)
Deposit Reference: Student Number, Surname and J091/46012.

NOTE: Please provide the Proof of Payment to the Department of Nursing Science for the SANC registration by no later than 30 January 2025. Your place in the programme will regrettably be forfeited should you not timeously pay the SANC registration fees; only one application will be submitted within 30 days after registration, as per the SANC guideline/policy.

2.2 Step 2: University Registration

2.2.1 The 2026 Registration Booklet with the applicable registration dates for the Post Graduate Diplomas will be available on the CPUT website (www.cput.ac.za) by the end of 2025. It is imperative to note that students will not be allowed to attend classes if they have not registered for the 2026 academic year.

2.2.2 Full-time Studies

Full-time students will register for the full load in the 2026 academic year i.e. all ten (10) subjects in this clinical programme, *Occupational Health Nursing* as indicated in Table 1 below:

TABLE 1: 2026 SUBJECTS FOR FULL-TIME STUDENTS		
Semester	Subject Description	Subject Codes
Semester 1 (January – June)	Fundamentals in Research	FIR582S
	Professional Practice	PRP582S
	Research Methodology	RMD584S
	Management and Leadership	MLS582S
	Dynamics of Occupational Health Nursing Management	DOH580S
	Practice of Occupational Health Nursing	POH580S
Semester 2 (July – December)	Research Project in Occupational Health Nursing	RPM581S
	Professional Development	PDT582S
	Trends in Occupational Health Nursing	TOH580S
	Risk Management in Occupational Health	RMN580S

2.2.3 Part-time Studies (Year 1: 2026)

Part-time students will complete the programme over two (2) years and must only register for the six (6) subjects in this clinical programme, *Occupational Health Nursing* as indicated in Table 2 below:

TABLE 2: 2026 YEAR 1 SUBJECTS FOR PART-TIME STUDENTS		
Semester	Subject Description	Subject Codes
Semester 1 (January – June)	Fundamentals in Research	FIR582S
	Professional Practice	PRP582S
	Management and Leadership	MLS582S
	Dynamics of Occupational Health Nursing Management	DOH580S
Semester 2 (July – December)	Professional Development	PDT582S
	Trends in Occupational Health Nursing	TOH580S

2.2.4 Part-time Studies (Year 2: 2027)

Part-time students will complete the programme over two (2) years and must only register for the four (4) subjects in this clinical programme, *Occupational Health Nursing* as indicated in Table 3 below:

TABLE 3: 2027 YEAR 2 SUBJECTS FOR PART-TIME STUDENTS		
Semester	Subject Description	Subject Codes
Semester 1 (January – June)	Practice of Occupational Health Nursing	POH580S
	Research Methodology	RMD584S
Semester 2 (July – December)	Research Project in Occupational Health Nursing	RPM581S
	Risk Management in Occupational Health	RMN580S

2.2 Step 3: Expenses and Payments

Financial provision should be made for the necessities used in the programme, including prescribed textbooks as indicated in Table 4 below:

TABLE 4: PRESCRIBED TEXTBOOKS				
Author(s)	Title	ISBN Number	Edition	Publisher
Creswell, JW & Creswell JD, 2018	Research Design: Qualitative, Quantitative, and Mixed Methods Approaches	9781506386690	5 th Ed	Sage Publications
Polit, DF & Beck, CT 2020	Essentials of Nursing Research: Appraising Evidence for Nursing Practice	9781496351296	10 th Ed	Wolters Kluwer
LoBiondo-Wood, G, Haber, J & Tittler, MG 2019	Evidence-Based Practice for Nursing and Healthcare Quality Improvement	9780323480055		Elsevier
Jooste, K, Bezuidenhout, MC & Muller, M 2021	Healthcare Service Management	9780702188671 9781485129127	3 rd Ed	Juta
Jooste, K 2025	Leadership in Health Service Management	9781485113829	4 th Ed	Juta
Jooste, K 2025	The Principles and Practice of Nursing and Health Care: Ethos and Professional Practice, Management, Staff Development and Research	9780627041730	3 rd Ed	Van Schaik
Acutt, J & Hattingh, S (Eds) 2024	Occupational Health: Management and Practice for Health Practitioners	9780702186745	6 th Ed	Juta
Michell, KE (Ed) 2011	A Practical Approach to Occupational Health Nursing	9780620515726		SASOHN
Juta's Statutes (Eds) 2019	Occupational Health and Safety Act and Regulations. 20 th Ed	9781485134756	20 th Ed	Juta
Juta's Statutes (Eds) 2018	Mine Health and Safety Act and Regulations	978148513302	8 th Ed	Juta
Viljoen, MJ & Sibiyi, N 2009	History Taking and Physical Examination.	9781868919765	2 nd Ed	Pearson

3. COMMENCEMENT INFORMATION PART TWO

3.1 Communication

- 3.1.1 **Prior to registration**, it is essential that your most recent personal contact details are captured on the system i.e., cell phone number and email address to ensure that you receive the necessary information from the institution and the department. Please contact the postgraduate administrator, Mr Adinaan Lakay at lakaya@cput.ac.za OR the Call Centre at info@cput.ac.za or 021 959 6767 should you require assistance.
- 3.1.2 **After registration**, it is of vital importance to activate your CPUT student email to receive communication from the institution and department; please only use the CPUT student email when communicating with the institution going forward i.e., kindly refrain from using your personal email once you are registered.
- 3.1.3 Recordings of sessions will be uploaded to *Black Board*, the e-learning platform also known as the *Learner Management System*; the link of sessions will be sent to the CPUT student email, and it is thus imperative that you activate your student email. Please contact the Department of Computer and Telecommunications Services (CTS) at ctsservicedesk@cput.ac.za or 021 959 6407 should you require assistance in this regard.
- 3.1.4 Please ensure that you know how to access the *Student Online Services (SOS)* portal as you will use it extensively during your time at the institution; please contact the Call Centre at info@cput.ac.za or 021 959 6767 should you require assistance.
- 3.1.5 The programme will commence officially with orientation on Monday, 26 January 2026, and the number of face-to-face versus online sessions will be confirmed during orientation.

3.2 Exemptions

- 3.2.1 Registered students are eligible to apply for exemption for *Fundamentals in Research*, if completed on a **National Qualifications Framework (NQF) Level 8** or higher Nursing Degree programme; the content of the module completed will first be evaluated.
- 3.2.2 Only a relevant module completed within the past 7 years will be considered.
- 3.2.3 Completed *Application for Exemption Forms* to be handed to the postgraduate administrator between 26 January and 30 January 2026; late applications will not be accepted.
- 3.2.4 An application for exemption does not constitute automatic approval and students must register and attend these classes until they have been notified otherwise. (See CPUT General handbook: Academic and Student Rules and Regulations).
- 3.2.5 Legacy modules of the qualifications phased out at the end of 2019 will not be exempted in this new Post Graduate Diploma.
- 3.2.6 The *Application for Exemption Form* must be accompanied by the following:
 - 3.2.6.1 Proof of Payment of R65 (per subject) for recognition of **internal** subjects (i.e. CPUT or other University of Technology) and R105 (per subject) for recognition of subjects passed at a university (e.g. UWC); **fees are subject to change**.
 - 3.2.6.2 Certified copies (not older than three months) of official documents from previous institutions i.e. Qualification(s), **full** Academic Transcript(s) (i.e. year passed, and marks obtained) as well as a **Subject Syllabus (Content of the Subject)**; the applicant cannot be evaluated without this vital information.
- 3.2.7 It remains the student's responsibility to follow-up regularly with the respective lecturer(s) regarding the outcome of the application for exemption.

3.3 Programme Structure

- 3.3.1 The Post Graduate Diploma in Occupational Health Nursing consists of compulsory work-integrated learning (WIL) sessions to attend as indicated in Tables 5 below.
- 3.3.2 Each subject in the programme has a theory and WIL component; both components must be passed to complete and pass the subject at the end of the respective semester.
- 3.3.3 There are **720** hours allocated to WIL which are spread over the subjects.
- 3.3.4 As part of the WIL component, full time students will be placed for two (2) days a week on a weekly basis; this will be applicable to each academic year.
- 3.3.5 Part time students will be placed one (1) day a week on a weekly basis in accredited facilities; this will be applicable to each academic year.
- 3.3.6 Compulsory simulation laboratory sessions will take place on Tuesdays as indicated in Table 6 and Table 7 on page 5.

TABLE 5: WORK-INTEGRATED LEARNING WORKSHOP DATES FOR POST GRADUATE DIPLOMA IN OCCUPATIONAL HEALTH NURSING (ON CAMPUS)			
Semester	Subject Description	2026 Dates	Time
Semester 1 (January – June)	Management and Leadership	25 March 26 March 27 March (Seminar)	08h30 – 16h30
	Professional Practice	12 May 13 May	
Semester 2 (July – December)	Professional Development	13 August 14 August	08h30 – 16h30

TABLE 6: COMPULSORY TUESDAY SIMULATION LABORATORY SESSIONS FOR FULL TIME STUDY		
SIM LAB	09h00 – 13h00 Dynamics of Occupational Health Nursing Management Trends in Occupational Health Nursing	09h00 – 13h00 Practice of Occupational Health Nursing Risk Management in Occupational Health
Semester		
Semester 1 (January – June)	17 February 24 March 21 April 19 May	10 February 3 March 10 March 31 March 14 April 28 April 12 May 26 May
Semester 2 (July – December)	28 July 25 August 15 September 27 October	21 July 4 August 18 August 1 September 29 September 13 October 20 October 3 November

TABLE 7: COMPULSORY TUESDAY SIMULATION LABORATORY SESSIONS FOR PART TIME STUDY		
SIM LAB	09h00 – 13h00 Dynamics of Occupational Health Nursing Management Trends in Occupational Health Nursing	
Semester		
Semester 1 (January – June)	17 February 24 March 21 April 19 May	
Semester 2 (July – December)	28 July 25 August 15 September 29 October	

3.4 Teaching Approach

- 3.4.1 This programme follows a blended learning approach where face-to-face learning activities are combined with online learning components.
- 3.4.2 The main mode of teaching and learning is through face-to-face sessions, online lectures and presentations, face-to-face and online group work and discussions, and self-directed learning.
- 3.4.3 When conducted, face-to-face sessions will take place on the CPUT, Bellville campus.
- 3.4.4 As part of the WIL component, students are required to complete the required hours and tasks in the accredited facilities where you will be placed on a weekly basis; other WIL sessions will include compulsory workshops and simulation laboratory sessions as reflected in the previous tables.

3.5 Contact Sessions

The contact days and times for the theoretical component of subjects for all students is listed in Table 8 on page 7.

3.6 Assessments

- 3.6.1 The Post Graduate Diploma in Occupational Health Nursing consists of compulsory assessments to be completed for all subjects.
- 3.6.2 The assessment dates are available in the respective subject guides and assessments will be conducted online, on campus and face-to-face as prescribed by the department.

The Department of Nursing Science aims to create a positive learning experience for students, and we trust that you will enjoy the programme we offer as we strive towards equipping you with the ability to ensure excellence in quality care offered to our communities.



Kind regards

Dr Odilia M M Sehume
Head of Department
Department of Nursing Science

To whom it may concern,

It is my pleasure to write you a short motivation for Mr Wilbur William Matthee. My Name is Dr Priscilla MiJeong Ohm currently working with Mr Matthee within the Mitchell's Plain District hospital Emergency unit. The emergency unit is one that requires timeous action, fast thinking and definitive action as it has many moving parts including a high patient load, critically ill patients with undifferentiated pathology all needing urgent management. Mr Matthee carries all the above qualities. As a shift leader he allocates staff to needed areas with good skills distribution across the floor, furthermore he often fills in gaps needed as they arise to allow for continued workflow and patient care allowing for staff breaks without disruption of patient care. All shift related issues including high risk patients who have absconded, drug ordering, equipment issues as well as family communication as needed are always appropriately done or escalated with timeous feedback given to the doctor team leader. When students are allocated whether nursing, EMS or medical are within the unit during his work he allocates tasks to aid their learning and supervises each student within their patient care.

When it comes to patient care within the emergency unit, Mr Matthee shows depth of clinical knowledge which allows him to identify critically ill patients, as well as pre-empt needed management which allows for improve the efficiency of patient care within the unit. He is well versed in the management protocols commonly used within the unit and is able to remind doctors of when the next blood gases or ECGs are needed as well as the next step in management on the protocol. All emergency room doctors leave the resus patients once stabilised in Mr Matthee's care till next review time with no doubt that they will be correctly monitored, cared for and that they will be timeously informed if there is a change in condition.

One particular point of excellence for Mr Matthee is the way he is able to de-escalate any heated situation within the emergency unit whether it be with family, staff or patients with a calm tone and demeanour. He approaches all his patients with the same respect and as much as possible protects their dignity with simple actions as always closing curtains, giving patients privacy when needed and answering patient queries as well as informing family members of needed standard procedures. His method of communication and interaction with his patients has been a lesson and reminder to me to give good, individualised care and communication that maintains patient dignity and autonomy.

Regards,

Dr PMJ Ohm

A handwritten signature in black ink, appearing to be 'PMJ Ohm', written in a cursive style.

ANNEXURE

"U"

uFiling portal screenshot confirming 'No Information' for June 2025 UIF period — evidence of
UIF non-declaration

Date: 2026

*Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*

MEDSCHEME HOLDINGS PTY LTD (202103 - 202702)	
March 2025	
Salary:	R40589.15
Hours:	160
Start Work Date:	01-05-2024
End Work Date:	
Termination Reason:	
April 2025	
Salary:	R42537.43
Hours:	160
Start Work Date:	01-05-2024
End Work Date:	
Termination Reason:	
May 2025	
Salary:	R34637.82
Hours:	160
Start Work Date:	01-05-2024
End Work Date:	
Termination Reason:	
June 2025	No Information
July 2025	
Salary:	R27725.66
Hours:	160
Start Work Date:	01-05-2024
End Work Date:	
Termination Reason:	
August 2025	
Salary:	R40562.53
Hours:	160
Start Work Date:	01-05-2024
End Work Date:	
Termination Reason:	
September 2025	
Salary:	R42537.43
Hours:	160
Start Work Date:	01-05-2024
End Work Date:	
Termination Reason:	
September 2025	
Salary:	R42537.43
Hours:	160
Start Work Date:	01-05-2024
End Work Date:	
Termination Reason:	
October 2025	
Salary:	R35625.28
Hours:	160
Start Work Date:	01-05-2024
End Work Date:	
Termination Reason:	
November 2025	
Salary:	R36612.73
Hours:	160
Start Work Date:	01-05-2024
End Work Date:	
Termination Reason:	
December 2025	
Salary:	R19490.32
Hours:	160
Start Work Date:	01-05-2024
End Work Date:	12-12-2025
Termination Reason:	Resigned
January 2026	No Information
February 2026	No Information
(5 of 6) << < > >> 12	

ST LUKES HOSPICE (202103 - 202702)	
March 2021	No Information

MEDSCHEME HOLDINGS PTY LTD (202103 - 202702)					
March 2025		April 2025	May 2025		
Salary:	R40589.15	Salary:	R42537.43	Salary:	R34637.82
Hours:	160	Hours:	160	Hours:	160
Start Work Date:	01-05-2024	Start Work Date:	01-05-2024	Start Work Date:	01-05-2024
End Work Date:		End Work Date:		End Work Date:	
Termination Reason:		Termination Reason:		Termination Reason:	
June 2025		July 2025		August 2025	
No Information		Salary:	R27725.66	Salary:	R40562.53
		Hours:	160	Hours:	160
		Start Work Date:	01-05-2024	Start Work Date:	01-05-2024
		End Work Date:		End Work Date:	
		Termination Reason:		Termination Reason:	
September 2025		October 2025		November 2025	
Salary:	R42537.43	Salary:	R35625.28	Salary:	R36612.73
Hours:	160	Hours:	160	Hours:	160
Start Work Date:	01-05-2024	Start Work Date:	01-05-2024	Start Work Date:	01-05-2024
End Work Date:		End Work Date:		End Work Date:	
Termination Reason:		Termination Reason:		Termination Reason:	

ANNEXURE



Certificate of Service (disputed 'Care Coordinator' designation); request for 'Naming Convention' meeting recording; refusal to correct — contravention of BCEA s42

Date: January 2026

*Mathee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*

37 Conrad Street
Florida North
Roodepoort

Switchboard: 011 671-2000

Website: www.medscheme.co.za

19 January 2025

To Whom It May Concern

RE: CERTIFICATE OF SERVICE FOR WILBUR MATTHEE

Medscheme Holdings (Pty) Ltd hereby certify that **Wilbur Matthee**, Identity Number **8909035151087**, was employed by us for the period **01 May 2024 to 12 December 2025**.

The last position held was **Care Coordinator** in the **GEMS Disease Management** department.

At termination of service, Wilbur's Total Cost to Company Remuneration was **R42796.13** per month.

Sincerely,



AFROCENTRIC HEALTH
SALARIES DEPARTMENT
P.O. BOX 1101
FLORIDA GLEN, 1708

Keorapetse Moiloa
TSS Admin Support Officer
Human Capital Management
(011) 671 4715



Medscheme Holdings (Pty) Ltd Reg No 1970/015014/07
PO Box 1101, Florida Glen 1708 | 37 Conrad Street | Florida North | Roodepoort 1709



DIRECTORS:
G Van Wyk, KT Moloele, MJ Madungandaba, AA Mahmood, FV Nompumza, AD Schwulst (CEO), BS Madikiza, MS Bray (COO)

Group Company Secretary: LMS Mpumlwana



Correction Required – Certificate of Service Misrepresentation

4 messages

Wilbur William Matthee <info@ethichawks.co.za>
To: Monwabisi Kula <monwabisi.kula@afrocentrichealth.com>, Lindiwe.Miyambu@afrocentrichealth.com
Cc: Sipokazi Makaula <Sipokazi.Makaula@afrocentrichealth.com>, Cheryl-Dawn Modern <Cheryl-DawnM@afrocentrichealth.com>, Tebogo.Makoe@afrocentrichealth.com, nthabisengn@medscheme.co.za
Bcc: wilburmat@gmail.com

Mon, 19 Jan 2026 at 12:25

Dear Sipokazi,

I acknowledge receipt of the Certificate of Service. However, I must formally object to the fact that my role has been adjusted to align with the Opposing Affidavit dated 10 December 2025, rather than reflecting my actual employment records.

The Certificate of Service is a statutory document required for UIF and fund processing. It must be accurate and consistent with payroll and HR records, not tailored to litigation pleadings. Adjusting my role to fit an affidavit constitutes misrepresentation and undermines compliance with the Basic Conditions of Employment Act.

I further note that both you and Nthabiseng were present in the recorded Teams meeting titled “Naming Convention”, where the Operational Manager encouraged us to update our statistics. That meeting provides direct evidence that the employer had full knowledge of my role, identity, and responsibilities. Any deviation from those facts in the Certificate of Service is therefore unjustifiable.

I therefore request that the Benefits Team urgently issue a corrected Certificate of Service that reflects my true role and employment details, as per payroll and HR records and the recorded meeting. As a result, of this failure, that is evedince require you have not met your 12:00 deadline, I require correct Certificate of Service by close of business 19 January 2026.

Should this not be provided within the stipulated timeframe, I will proceed to file a formal complaint with the Department of Employment and Labour (DEL) and request a statutory audit of the employer’s compliance.

Please confirm by return email before close of business.

Kind regards,

Wilbur William Matthee
RN, BCur (HMN), PG Dip HSM (NQF)

<postmaster@medscheme.co.za>
To: info@ethichawks.co.za

Mon, 19 Jan 2026 at 12:26

Delivery has failed to these recipients or groups:

tebogo.makoe@afrocentrichealth.com

A communication failure occurred during the delivery of this message. Please try resending the message later. If the problem continues, contact your email admin.

The following organization rejected your message: AM4PEPF00027A62.mail.protection.outlook.com.

X-OriginatorOrg: AfroCentricHealth.onmicrosoft.com
X-OrganizationHeadersPreserved: ZABDCEXCAMBX06.medscheme.com
X-CrossPremisesHeadersFilteredByDsnGenerator: ZABDCEXCAMBX06.medscheme.com

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This email has been scanned for email related threats and delivered safely by Mimecast.
For more information please visit <http://www.mimecast.com>

[Quoted text hidden]

Cheryl-Dawn Modern <Cheryl-DawnM@afrocentrichealth.com>
To: info@ethichawks.co.za <info@ethichawks.co.za>
Cc: Nthabiseng Nkatane <nthabisengn@medscheme.co.za>, Sipokazi Makaula
<Sipokazi.Makaula@afrocentrichealth.com>

Mon, 19 Jan 2026 at 15:40

Dear Wilbur

Thank you for your e-mail,

Ito correspondences, from an HR perspective, I would like to confirm that we responded within the stipulated timeframe and provided all information required from our side. Furthermore the Certificate of Service issued is correct and fully aligned with our employment records as well as statutory requirements.

With regard to the naming-convention changes, please note that this business process is still in progress, and I can confirm that no changes to the Care Co-Ordinator job title for your department have been implemented at this stage. Your title is therefore accurately and correctly reflected in the Certificate of Service.

Should you require any additional documentation, we are available to assist; however, our position regarding the accuracy of the certificate remains unchanged.

Kind Regards

Cheryl-Dawn Modern

Senior HR Business Partner

📞 +27 (011) 671 4819

✉ cheryl-dawnm@afrocentrichealth.com

📍 [37 Conrad St, Florida North, Roodepoort](#) | PO Box 1101, Florida Glen, 1709

🌐 www.afrocentric.za.com



Please consider the environment before printing this email

Always aligning our leaders & people with the business purpose.

CONFIDENTIALITY:

This email contains confidential information. The information on this email and attachments thereto (if any) is only for the use of the intended recipient. If you are not the intended recipient, any disclosure, copying and/or distribution of the content of this email to any person other than the sender (as specified in the email), or the taking of any action thereon, is strictly prohibited. Interception of this email is also prohibited. If you have received this email in error please notify the sender (as specified in the e-mail) using the contact details contained in such e-mail and delete this email and any attachments thereto, once you have notified the sender.

From: Wilbur William Matthee <info@ethichawks.co.za>

Sent: Monday, January 19, 2026 12:25 PM

To: Monwabisi Kula <Monwabisi.Kula@afrocentrichealth.com>; Lindiwe Miyambu <Lindiwe.Miyambu@afrocentrichealth.com>

Cc: Sipokazi Makaula <Sipokazi.Makaula@afrocentrichealth.com>; Cheryl-Dawn Modern <Cheryl-DawnM@afrocentrichealth.com>; Tebogo Makoe <Tebogo.Makoe@afrocentrichealth.com>; Nthabiseng Nkatane <nthabisengn@medscheme.co.za>

Subject: Correction Required – Certificate of Service Misrepresentation

This message originated from outside your organization

Dear Sipokazi,

[Quoted text hidden]

[Quoted text hidden]

 **image014.png**
29 KB

Wilbur William Matthee <info@ethichawks.co.za>

Mon, 19 Jan 2026 at 16:30

To: Cheryl-Dawn Modern <Cheryl-DawnM@afrocentrichealth.com>

Cc: Nthabiseng Nkatane <nthabisengn@medscheme.co.za>, Sipokazi Makaula

<Sipokazi.Makaula@afrocentrichealth.com>

Bcc: wilburmat@gmail.com

Dear Cheryl-Dawn,

Thank you for the information provided.

You have stated that the company was compliant with the statutory timeline. However, the documents I received were incomplete, which means the company is in fact not compliant with statutory requirements.

For the record:

- I verbally accepted the updated role during the recorded Teams meeting titled "Naming Convention", attended by Sipokazi Makaula and Nthabiseng Nkatane, where the Operational Manager informed me to update my identifiers.
- I subsequently updated my professional signature to reflect the new title.
- Both the recording and my updated signature constitute legally binding confirmation of the naming convention.

Despite this, the Certificate of Service issued contradicts both the recorded meeting and my updated signature, and has been aligned with the employer's Opposing Affidavit dated 10 December 2025. This alignment is illegal, as statutory documents must reflect factual employment records, not be tailored to litigation pleadings.

As a result, the current Certificate of Service will form part of my legal proceedings. Unless a corrected version is provided by close of business today, 19 January 2026, I will proceed to file a formal complaint with the Department of Employment and Labour (DEL) and include this matter in my Labour Court affidavit.

Kind regards,



Wilbur Mat... 2025/10/15



to me ▾



Medscheme

Sr. Wilbur W. Matthee

*(Nurse Specialist PGrad HSM, Btech Gen P
(Midw, Psych, Comm))*

Case Manager: GEMS Active Disease Risk
Management

WITNESS

- +27 12 466 1332
- wilbur.matthee@medscheme.co.za
- The Boulevard Building F & G, Searle street, Woodstock, 7925
- www.medscheme.com



Consent before printing this email

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Reply



Forward





Request for Disclosure of Recorded Teams Meeting – “Naming Convention”

1 message

Wilbur William Matthee <info@ethichawks.co.za>
To: Cheryl-Dawn Modern <Cheryl-DawnM@afrocentrichealth.com>
Cc: Nthabiseng Nkatane <nthabisengn@medscheme.co.za>, Sipokazi Makaula <Sipokazi.Makaula@afrocentrichealth.com>
Bcc: wilburmat@gmail.com

Mon, 19 Jan 2026 at 17:24

Dear Cheryl, Sipokazi, and Nthabiseng.

I hereby formally request disclosure of the full and unaltered Microsoft Teams recording titled “Naming Convention”, attended by Sipokazi Makaula and Nthabiseng Nkatane. This recording is in the Respondent’s possession and constitutes material evidence directly relevant to the dispute concerning my designation and the accuracy of the Certificate of Service.

Please provide the recording by [reasonable deadline, e.g., 5 working days]. Should the Respondent fail to disclose the recording, I will proceed to seek an order compelling production in terms of Rule 6(7) of the Labour Court Rules and invite the Court to draw an adverse inference from any refusal to disclose evidence within your possession.

Kind regards,

Wilbur Matthee

Specialist Nurse Health Services Manager
SRN | BTech Psych, Midw, Community | BCur HNM | PG Dip HSM (NQF 8)

ANNEXURE



CMS referral letter identifying 'Administrative Misconduct and Ethical Failure' in relation to
Gexus/DMS integration gap — independent third-party corroboration

Date: 2026

*Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*



Administrator Misconduct, Ethical Failure & Retaliation – Request for Investigation

6 messages

Wilbur William Matthee <info@ethichawks.co.za>
To: complaints@medicalschemes.co.za
Bcc: wilburmat@gmail.com

Tue, 23 Dec 2025 at 13:02

CMS – Council for Medical Schemes

Subject: Administrator Misconduct, Ethical Failure & Retaliation – Request for Investigation

Dear CMS Compliance & Investigations Unit,

I request CMS intervention regarding:

- discrimination
- retaliation
- unlawful deductions
- institutional silence
- executive de-escalation of my PDA
- board-level governance failure

These failures compromise:

- ethical administration
- beneficiary protection
- scheme governance

I request:

- whistleblower protection
- independent investigation
- oversight of Medscheme's governance practices

Sincerely,

Wilbur William Matthee
RN, BCur (HMN), PG Dip HSM (NQF 8)

2 attachments

-  **Sworn Affidavit evidence bundle.pdf**
342 KB
-  **Regulatory Full Master bundle ready for regur final-compressed (1).pdf**
4.4 MB

Complaints <complaints@medicalschemes.co.za>
To: Wilbur William Matthee <info@ethichawks.co.za>

Fri, 09 Jan 2026 at 11:22

Good day,

We acknowledge receipt of your email.

Please note that in order for your matter to be formally registered it is required that you complete and submit the documentation and/or correspondence as seen below. We request that you submit your complaint together with the following documents, if applicable: In a single email.

- A completed complaint form which we have attached herein for your attention;
- If acting on behalf of a member/beneficiary, please attach a letter of consent, written and signed by the member /beneficiary of the medical scheme.

Your matter will only be attended to upon receipt of all the requested documents.

The complaint should be e-mailed to complaints@medicalschemes.co.za

We are looking forward to your response.

Kind Regards,

Polite

Complaints

Council for Medical Schemes

t:
e: complaints@medicalschemes.co.za
Disclaimer

[Quoted text hidden]

3 attachments

-  **Sworn Affidavit evidence bundle.pdf**
342 KB
-  **Regulatory Full Master bundle ready for regur final-compressed (1).pdf**
4.4 MB
-  **UPDATED-COMPLAINT-FORM-AND-CHECKLIST_APRIL-2024-ft.docx**
706 KB

Wilbur William Matthee <info@ethichawks.co.za>
To: Complaints <complaints@medicalschemes.co.za>

Mon, 12 Jan 2026 at 21:12

Dear Polite,

I acknowledge receipt of your email and the attached documents.

As a direct result of my employer's conduct and ongoing retaliation, I am currently in a humanitarian crisis and destitute. Due to an unlawful 40% salary deduction, I was unable to pay the necessary fees to register as a specialist nurse, legally barring me from practicing in 2026.

Furthermore, the conciliation meeting on January 9, 2026, resulted in an insulting offer of only one month's remuneration. This follows a year of discrimination, victimization, and retaliation after I reported P1 risks and made protected disclosures. I am now facing severe prejudice, imminent eviction, and homelessness.

I will complete and return the required documents as soon as possible.

Kind regards,

Sr. Wilbur Matthee

Specialist Nurse Health Services Manager
SRN | BTech Psych, Midw, Community | BCur HNM | PG Dip HSM (NQF 8)

Sr. Wilbur Matthee

Specialist Nurse Health Services Manager
SRN | BTech Psych, Midw, Community | BCur HNM | PG Dip HSM (NQF 8)

[OBJ]

[Quoted text hidden]

Wilbur William Matthee <info@ethichawks.co.za>
To: Complaints <complaints@medicalschemes.co.za>

Tue, 20 Jan 2026 at 15:47

Dear Polite,

I am writing in my capacity as a registered nurse, in strict adherence to my professional and ethical obligations as outlined by the South African Nursing Council (SANC).

Pursuant to our earlier correspondence, please find attached the following requested documents in support of the formal complaint being lodged:

1. Facts of the complaint – including supporting email evidence.
2. Dispute resolution steps undertaken – with proof of escalation and correspondence.
3. Recourse required – as outlined in the attached summary.

These documents have been prepared in compliance with SANC's Code of Ethics and Professional Standards, which mandate the reporting of conduct that jeopardises patient safety, data integrity, or ethical healthcare administration.

Should you require any further information or clarification, please do not hesitate to contact me.

Thank you for your attention to this serious matter.

Yours sincerely,

Attachments:

- Complaint Summary & Evidence.pdf
- Escalation Proof.pdf
- Required Recourse.pdf

Sr. Wilbur Matthee

Specialist Nurse Health Services Manager
SRN | BTech Psych, Midw, Community | | PG Dip HSM (NQF 8)

[Quoted text hidden]

2 attachments

-  **UPDATED-COMPLAINT-FORM-AND-CHECKLIST_APRIL-2024-ft.pdf**
243 KB
-  **General complaint was called insurbordanation .pdf**
235 KB

Complaints <complaints@medicalschemes.co.za>
To: Wilbur William Matthee <info@ethichawks.co.za>

Wed, 04 Feb 2026 at 12:11

Good day,

We acknowledge receipt of your email,

Regrettably, we will not be able to proceed with the registration and investigation of your complaint because it does not fall within our jurisdiction as the Council for Medical Scheme (CMS). The CMS is mandated by the Medical Schemes Act register and investigate complaints against Medical Schemes, Administrators, Brokers and Managed Health Care Facilities.

Please note that complaints against Health Professionals are regulated by the Health Professions Council of South Africa (HPCSA). We hereby recommend that you refer the attached complaint to the Health Professions Council of South Africa (HPCSA) in writing. Please see below the HPCSA contact details: <https://www.hpcsa.co.za/>

Client Care Centre

Professionals and members of the public are encouraged to call the HPCSA Client Care Centre for all general enquiries including annual fees, registrations, compliments, service delivery, certificates of status, CPD, ethical queries and undesirable business practices.

Telephone	Postal Address	Physical Address	Self-Service
(+27) 12 338 9300/01	PO Box 205 Pretoria 0001	553 Madiba Street Madiba Street Arcadia 0083	Visit our online portal on the following link: https://hpcsaonline.custhelp.com/
NATURE OF QUERY		E-mail	
Certificate of status		hpcsaacs@hpcsa.co.za	
Complaints against practitioners		Legalmed@hpcsa.co.za	
Education & Training		Education&Training@hpcsa.co.za	
Service Delivery		servicedelivery@hpcsa.co.za	

Kind regards,

[Quoted text hidden]

Wilbur William Matthee <info@ethichawks.co.za>
To: Complaints <complaints@medicalschemes.co.za>

Fri, 06 Feb 2026 at 10:54

Thank you for the feedback.

Kind regards

Sr. Wilbur Matthee

[Quoted text hidden]



image001.png

41 KB

ANNEXURE

"X"

SAHRC acknowledgment of seriousness of allegations of victimisation and financial prejudice; referrals to CCMA, Labour Court and Department of Employment and Labour

Date: 2026

*Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*



RE: Request for Intervention on Procedural and Human Rights Concerns

4 messages

Lwethu Yoli <LYoli@sahrc.org.za>
To: info@ethichawks.co.za <info@ethichawks.co.za>

Tue, 10 Feb 2026 at 09:45

Dear Mr Matthee,

Thank you for your correspondence and for submitting your documentation to the South African Human Rights Commission("Commission").

The Commission has considered the nature of the issues raised in your submission. While the Commission is mandated to promote, protect, and monitor the observance of human rights, certain aspects of your complaint relate to alleged victimization, discrimination, and occupational detriment arising within an employment relationship.

In terms of existing legal frameworks:

1. Allegations relating to unfair labour practices and discrimination in the workplace are generally adjudicated by the CCMA and/or the Labour Court; and
2. Complaints specifically concerning protected disclosures and alleged reprisals may be pursued with the Public Protector in terms of the Protection of Disclosures Act.

Accordingly, the Commission is not best placed to provide direct intervention in respect of those aspects of the matter.

This does not diminish the seriousness of the concerns you have raised, nor the personal impact described in your correspondence. Should you wish to pursue the matter further, you are encouraged to approach the above institutions, which have the necessary remedial powers.

Thank you for engaging the Commission and wish you well in pursuing the appropriate avenues available to you.

Kind regards,

Lwethu Yoli

Western Cape PO

Legal Officer
Accredited Mediator



Together, respecting and realising human rights.

Follow the SAHRC:     

From: Wilbur William Matthee <info@ethichawks.co.za>

Sent: Tuesday, December 23, 2025 1:48 PM

To: Busisiwe Dlamini <bdlamini@sahrc.org.za>

Subject: Request for Intervention on Procedural and Human Rights Concerns

Dear South African Human Rights Commission,

I am writing to formally bring to your attention serious concerns regarding systemic governance failures and procedural obstruction within Afrocentric Health.

Find attached my full evidence dossier, compiled between 7–22 December 2025, which documents these risks in detail. The scale of this dossier reflects both the seriousness of the issues and the extraordinary psychological strain endured during its preparation.

I must also notify the Commission that I am experiencing a **humanitarian crisis** as a direct result of these circumstances. The combination of procedural obstruction, exhaustion, and the emotional toll of whistleblowing has placed me in a position of acute vulnerability.

I respectfully request that the SAHRC:

- Review the attached dossier and assess whether rights have been compromised.
- Provide guidance or intervention to ensure trauma-informed governance and corrective action.
- Recognize the ethical and psychological dimensions of whistleblowing as integral to systemic reform.
- Consider my humanitarian crisis as part of the urgency of this matter.

I am available to provide further documentation or testimony as required.

Thank you for your attention to this matter.

Sincerely,

Wilbur William Matthee
RN, BCur (HMN), PG Dip HSM (NQF 8)

Wilbur William Matthee <info@ethichawks.co.za>
To: Lwethu Yoli <LYoli@sahrc.org.za>

Tue, 10 Feb 2026 at 11:41

Dear Lwethu Yoli,

Thank you for your correspondence regarding my submission. I appreciate the Commission's formal recognition of the "seriousness of the concerns" and the "personal impact" of the victimization I have described.

While I acknowledge your referral to the CCMA and the Public Protector regarding the employment aspects of my case, I would like to provide an update that reinforces the human rights dimension of this matter.

The Council for Medical Schemes (CMS) has officially responded to my report regarding the "System Integration Gap" I identified. They have formally referred the matter to the HPCSA and SANC for investigation into "Administrative Misconduct and Ethical Failure." This confirms that my disclosures involved a verifiable risk to public health and patient safety, which was met with institutional retaliation.

As noted in my initial submission, the retaliation against me included a 40.47% salary deduction (unlawful in terms of BCEA Section 34) and the deliberate inducement of destitution during a documented psychiatric crisis. I believe these actions fall under the Commission's mandate to monitor the observance of human rights, as they constitute a direct assault on the right to dignity, housing, and administrative justice.

I will be following your guidance by updating my filings with the Labour Court and approaching the Public Protector regarding the protection of disclosures. I request that the Commission keeps my file on record as a reference point for the systemic victimization of whistleblowers within the South African healthcare sector.

I thank the Commission for its time and for validating the gravity of this situation.

Sincerely,

Wilbur William Matthee

[Quoted text hidden]

 **image002.png**
177 KB

Lwethu Yoli <LYoli@sahrc.org.za>
To: Wilbur William Matthee <info@ethichawks.co.za>

Tue, 10 Feb 2026 at 11:56

Dear Mr Matthee,

Thank you for your further correspondence.

With specific regard to the financial aspects you raise, including allegations relating to salary deductions and associated financial prejudice, the Commission notes that these matters fall within the statutory mandate of the Department of Employment and Labour, as well as the labour dispute resolution framework established under labour legislation.

The Department of Employment and Labour, together with the CCMA and the Labour Court, are institutions specifically created to protect and enforce the rights implicated in such matters, including rights relating to fair labour practices, remuneration, and compliance with the Basic Conditions of Employment Act. These bodies are accordingly best placed to investigate, determine compliance, and provide effective remedies in relation to the financial issues raised.

The Commission's referral is therefore not a dismissal of the seriousness of the allegations, but is based on the principle that matters should be dealt with by the institutions vested with the appropriate legal powers and remedial authority to address the rights affected.

You are encouraged to pursue the financial aspects of your complaint with the Department of Employment and Labour and the relevant labour institutions, which are empowered to investigate and take action where contraventions are established.

[Quoted text hidden]

Wilbur William Matthee <info@ethichawks.co.za>

Tue, 10 Feb 2026 at 12:44

To: Lwethu Yoli <LYoli@sahrc.org.za>

Dear Mr/Mrs Lwethu Yoli

I hereby acknowledge receipt of your correspondence dated 10 February 2026.

I thank the Commission for its guidance in ensuring these human rights violations are addressed by the institutions vested with the appropriate remedial authority.

I request that this matter remains on your records as a secondary reference should further systemic human rights breaches arise during the labor litigation process.

Thank you for your assistance.

Sincerely,

Wilbur William Matthee
RN, BTech, PG Dip HSM (NQF 8)

[Quoted text hidden]

 **image002.png**
179 KB

ANNEXURE

"Y"

CCMA Certificates of Non-Resolution: WECT23564-25 and WECT24486-25 — jurisdictional
basis for this Statement of Claim

Date: 9 January 2026

*Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*

CERTIFICATE OF OUTCOME OF DISPUTE REFERRED TO CONCILIATION

CASE NUMBER: WECT23564-25

I certify that the dispute between:

Wilbur Matthee

and

AgroCentric Distribution
Services (Pty) Ltd

(referring party)

(other party/parties)

Referred to conciliation on:

5/12/25
(give date)

Concerning

S187 (h) Protected Disclosure

☐

Was resolved on the _____ or

(give date)

☒

Remains unresolved as at 9/1/26

(give date)

Condonation:

Granted

Not applicable

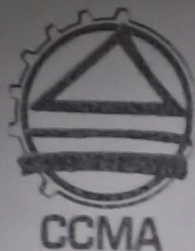
If this dispute remains unresolved, the
following steps may be taken

Refer to
Arbitration

Refer to interest /
Advisory
Arbitration

Strike/
Lockout

Refer to
Labour
Court



N Ngweni
Name of Commissioner

[Signature]
Signature of Commissioner

Lape Town
Place

9/1/26
Date

CERTIFICATE OF OUTCOME OF DISPUTE REFERRED TO CONCILIATION

CASE NUMBER: ~~WECT 23564-25~~ 24486-25 #1

I certify that the dispute between:

Wilbur Matthee

and

Afro Centric Distribution
Services (Pty) Ltd

(referring party)

(other party/parties)

Referred to conciliation on:

5/12/25
(give date)

Concerning

S187(f) Automatically Unfair Dismissal.

☐ Was resolved on the _____ or ☒ Remains unresolved as at 9/1/2026
(give date) (give date)

Condonation:

Granted

☒ Not applicable

If this dispute remains unresolved, the
following steps may be taken

Refer to
Arbitration

Refer to interest /
Advisory
Arbitration

Strike/
Lockout

Refer to
Labour
Court ☒



N Ngwane
Name of Commissioner

[Signature]
Signature of Commissioner

Cape Town
Place

9/1/2026
Date

ANNEXURE

"Z"

EthicHawks Full Master Chronological Register (148 entries, 11 January 2024 – 2 April 2026)
— comprehensive evidentiary timeline

Date: 3 April 2026

*Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*

2024 08:06	AfroCentric/Medschem e	Specialist: Care Manager role (reminder).				Nkatalane, Gretchen Veldman
15 Jan 2024 12:49	Internal – AfroCentric/Medschem e	Third interview invitation – rescheduled to 18 Jan 2024 at 11:00.	Matthee confirms attendance.	Complete	Not applicable.	Neo Sekgoka, Nthabiseng Nkatalane, Gretchen Veldman
29 Jan 2024 08:36	Internal – AfroCentric/Medschem e	Assessment for Specialist Care Manager position – instructions and link sent.	Matthee completes assessment.	Complete	Not applicable.	Neo Sekgoka, W. Matthee
30 Jan 2024 13:12	Internal – AfroCentric/Medschem e	Request for MIE consent form for fingerprint/background check.	Matthee submits consent form.	Complete	Not applicable.	Neo Sekgoka, W. Matthee
21 Feb 2024 09:00	Internal – MIE/AfroCentric	Fingerprint criminal check booking for pre-employment screening.	Reminder emails sent; Matthee attends.	Complete	Not applicable – standard pre-employment.	MIE, Lerato Maimane, W. Matthee
02 Apr 2024 07:53	Internal – Medscheme	BAIT AND SWITCH: Recruited for 'Specialist: Care Manager' but offered 'Care Coordinator'	Matthee accepts offer; Nthabiseng Nkatalane confirms start date 1 May 2024.	Complete	KEY EVIDENCE: Title suppression – recruited for Specialist,	Nthabiseng Nkatalane, W. Matthee

EthnicHawks | Matthee v AfroCentric/Medscheme | Master Register | Prepared: 03 April 2026

17 Mar 2025	Medical – Akeso Stepping Stones	Matthee seeks admission for mental health treatment (major depression, GAD). Admission for inpatient treatment (severe depression, PTSD); 3–4 weeks.	admission for 18 March 2025.	Complete	Not applicable – medical event.	Kezian Quimpo, W. Matthee
18 Mar 2025	Medical – Akeso Stepping Stones		Admitted to Akeso Stepping Stones Clinic.	Complete	Not applicable – medical event.	Dr Jacques Malan, W. Matthee
01 Apr 2025	Internal – AfroCentric	Annual salary increase of 4.8% effective 1 April 2025. New CTC: R42,796.13/month.	CEO Gerald Van Wyk confirms increase.	Complete	Not applicable.	Gerald Van Wyk, W. Matthee
01 Apr 2025	Medical – Dr Jacques Malan	Sick note issued: unfit for work from 18 March 2025; admitted for 3–4 weeks inpatient treatment.	Dr Malan certifies medical condition (confidential).	Complete	Not applicable – medical documentation.	Dr Jacques Malan, W. Matthee
16 Apr 2025 12:59	Internal – Medscheme	Jones states in meeting: 'we do not apply staggering of money owed' – critical evidence used to justify later unlawful deduction.	Matthee given accommodations (LACE cases, reduced hours, 3 hours for doctor visits).	Complete	Employer's written position on staggering – contradicts own policy.	Mary-Ann Jones, W. Matthee

14 Jul 2025 13:27	Internal – Medscheme	Special leave approved for 10 days; remainder of absence as unpaid leave.	Jones confirms special leave approved.	Complete	Not applicable.	Mary-Ann Jones, W. Matthee
23 Jul 2025	Internal – Medscheme	UNLAWFUL SALARY DEDUCTION: 40.47% (R14,811.77) deducted without consultation while between psychiatric admissions. Net pay R15,679.29.	Employer invokes 'no work no pay'; manager states company 'does not allow staggering' and Matthee 'wouldn't feel the deduction' due to online deliveries.	Complete	EXCEEDED BCEA s34(2) 25% cap. No consultation. Employer held Dr Maian's letter confirming medical necessity.	Cheryl-Dawn Modern, Mapule Mashele, Mary-Ann Jones, W. Matthee
24 Jul 2025 10:35	Internal – Medscheme	Jones shares AOD policy: 'monthly deductions must not be more than 25% of employee's CTC as per legislation.'	Matthee: 'they are allowed to deduct more than 50%?' Jones: 'This is for disability.'	Complete	CRITICAL: Employer acknowledged 25% cap but deducted 40.47%. Evidence of wilful non-compliance.	Mary-Ann Jones, W. Matthee

29 Sep 2025 12:27	Internal – Medscheme	RETALIATORY UNPAID LEAVE: Jones loads double punitive unpaid leave entry on exact same day Matthee files formal discrimination complaint.	Mapule Mashele reverses the entry on 1 Oct 2025.	Complete	RETALIATORY CONDUCT – documented and reversed. Written evidence of discretionary punitive action.	Mary-Ann Jones, Mapule Mashele, W. Matthee
01 Oct 2025 11:09	Internal – Investigation	Matthee provides formal update after meeting with Mashele; acknowledges shared understanding of seriousness.	Matthee agrees to speak directly with Jones in Mashele's presence.	Complete	Not applicable – cooperation with investigation.	Mapule Mashele, Mary-Ann Jones, W. Matthee
01 Oct 2025 13:28	Internal – Investigation	Mashele closes discrimination complaint. Jones's 'Wilma' was a 'typo'; apology accepted; matter closed. Matthee remains in Jones's reporting line.	Mashele issues closure report.	Complete	WHITEWASH – serious complaint dismissed as 'typo'. May 2025 disclosure of Jones's conduct ignored. Matthee not removed.	Mapule Mashele, Mary-Ann Jones, Cheryl-Dawn Modern, W. Matthee

02 Nov 2025	Internal – HR/Union	ADMISSION: Jones states: 'You made a case of harassment against me, so I can make a case of harassment against you. You misspelled my name in an email – go look'. Also warns: 'think of the ripple effect' of his actions.	No response documented.	Pending	evidence of PDA breach, retaliatory intent, and veiled threat.	Cheryl-Dawn Modern, Sipokazi Makaula, Thando Mandondo, Tebogo Dikobe, W. Matthee
02 Nov 2025	Internal – HR/Union	Formal 3-page grievance against Jones: malicious intent, gaslighting, weaponising past issues, orchestrated provocation, campaign for removal, retaliation. Demands investigation, removal from team, written guarantee against retaliation.	No response documented.	Pending	30+ DAYS INSTITUTIONAL SILENCE BEGINS.	Cheryl-Dawn Modern, W. Matthee
02 Nov 2025 22:55	Internal – HR	Request for review of July 2025 salary deduction – lack of written consent, staggering refused,	No response documented.	Pending	Ignored – part of institutional silence.	Cheryl-Dawn Modern, W. Matthee

05 Dec 2025 16:42	Internal – Medscheme	Matthee requests meeting conditions: recording, agenda, written responses. Bongani Luvuno claims only recently made aware.	Luvuno proposes engagement on 10 December.	Pending	Procedural delay – executive claims ignorance despite 30+ days silence.	Bongani Luvuno, Mapule Mashele, Gretchen Veldtman, W. Matthee
09 Dec 2025	Board of Directors – AfroCentric	Board escalation (38 attachments): discrimination, retaliation, unlawful deduction, 30+ days silence, King IV breaches. Requests independent investigation, manager suspension, board settlement.	Emails to Shadi Chauke and Lebohang Mpumiwana (typo) fail – invalid addresses.	Partial failure	Technical deflection – invalid email addresses for key directors.	Nkateko Munisi, Alice le Roux, Bruno Fernandes, Shadi Chauke (invalid), Lebohang Mpumiwana, W. Matthee
09 Dec 2025	Chief Risk Officer – Monwabisi Kula	Urgent plea: paid special leave, removal from manager's line, hardship payment.	Kula: asks for patience; 48-hour deadline unreasonable; acknowledges matter; commits to feedback 'in reasonable time'.	In progress	Delay tactic – 'takes time/other critical things'.	Monwabisi Kula, W. Matthee

2023		employer's opposition.	QuotaExceededException).	tails – mailbox full	CCWA mailbox full.	
12 Dec 2025	Internal – Medscheme	Resignation effective – employment terminated due to intolerable conditions.	Resignation submitted.	Complete	Not applicable – constructive dismissal complete.	W. Matthee
16 Dec 2025	Board Audit & Risk Committee	Maximum Severity GRC Escalation: suppression of findings, retaliation against investigator. King IV/ISO/OHSALRA breaches. 24-hour compliance window.	CRO Kula (17 Dec): asks Matthee to stop emailing directors; warns impatience 'does not serve you well'.	Complete	GATEKEEPING – CRO blocks Board-level communication.	Monwabisi Kula, Nkateko Munisi, Alice le Roux, Bruno Fernandes, W. Matthee
17 Dec 2025	Board/Executives	Matthee retracts GRC escalation (sent on public holiday, invalidating timeline). Underlying concerns remain.	No response documented.	Complete	Not applicable – retracted by Matthee.	W. Matthee, Board members
18 Dec 2025	Board of Directors	CRO: 'can I please ask you to stop sending these e-mails to our directors? Sending further e-mails	Matthee declines, asserting right to escalate material risks to oversight committees.	Complete	GATEKEEPING – CRO attempts to suppress	Monwabisi Kula, W. Matthee

23 Dec 2025	Council for Medical Schemes (CMS)	Administrator misconduct, ethical failure, retaliation, Gexus/DMS integration gap generating false non-adherence reports to GEIMS patients.	CMS acknowledges; requests complaint form and consent letter.	Pending	Not yet deflected at initial stage.	W. Matthee, Polite (CMS Complaints)
23 Dec 2025	SAHRC – Initial	Systemic governance failures, procedural obstruction, humanitarian crisis, right to dignity, housing, administrative justice.	No formal acknowledgement initially. Busisiwe Dlamini later responds requesting province.	Pending	Not yet deflected.	W. Matthee, Busisiwe Dlamini
23 Dec 2025	DEL – Initial	Unlawful salary deductions (>40%), gender discrimination, retaliation, constructive dismissal.	No immediate response documented.	Pending	Not yet deflected.	W. Matthee, DEL IES
23 Dec 2025	JSE – Initial	Board governance failure, P1 Material Risk Report ignored, retaliation via affidavit, King IV breaches.	Ncumisa Sinyanya acknowledges receipt same day.	Pending	Engaged – acknowledged.	Ncumisa Sinyanya (JSE Investigations), W. Matthee

15 Jan 2026 20:43	Employer – HR/Leadership	Matthee rejects stalling; provides notice of UIF non-compliance (June 2025 gap); demands docs by 19 Jan 12:00.	Email to Tebogo Makoe@ fails – 'Access denied'.	Partial failure	Email blocked – technical deflection.	Sipokazi Makaula, Tebogo Makoe, Lindiwe Miyambu, Monwabisi Kula, W. Matthee
18 Jan 2026	Employer – HR	Employer responds: UIF declared (screenshot), UI-19 attached, Certificate of Service attached. Makaula: UI-19 only if resignation qualifies.	Documents provided but Certificate disputed – role downgraded.	Complete	Documents provided but disputed for accuracy.	Sipokazi Makaula, Sebatatso Sebotsa, W. Matthee
19 Jan 2026 12:25	Employer – HR/Leadership	Matthee formally disputes Certificate of Service: 'Care Coordinator' vs 'Specialist: Care Manager' (NQF 8); refers to recorded 'Naming Convention' Teams meeting.	Cheryl-Dawn Modern (WRITTEN ADMISSION): Certificate correct; naming convention changes 'still in progress'; no changes implemented for Matthee's department.	Complete	EMPLOYER REFUSES CORRECTION – written admission of ongoing naming convention process.	Cheryl-Dawn Modern (WRITTEN ADMISSION), Nthabiseng Nkalane, Sipokazi Makaula, W. Matthee

2026	Initial	Mary-Ann Jones and Mapule Mashele (nurses).	No acknowledgement for 6 days.	Pending	SANC. SANC ignored complaint.	David Esau, Fezeka Ngalo, Sipokazi Makaula
10 Feb 2026	DEL – UIF & BCEA	False UIF declaration (June 2025 gap), fraudulent Certificate of Service, suppression of OHSA duties.	No acknowledgement for 6 days.	Pending	Ignored until follow-up.	David Esau, Fezeka Ngalo, Sipokazi Makaula
11 Feb 2026	HPCSA – Second Complaint	Complaint against HPCSA-registered Medical Directors and Lisa Mari (pharmacist).	HPCSA: states Lisa Mari falls under SAPC. Email to info@hpcsa.co.za blocked by mail flow rule.	Complete	Passed to SAPC. Blocked email – institutional access barrier.	LegalMed, W. Matthee
16 Feb 2026	DEL – Follow-up	48-hour deadline expired; June 2025 UIF gap still 'No information'; no case reference or inspector assigned.	Fezeka Ngalo responds same day: requests physical address, then allocates integrated inspection.	In progress	Engaged – Ngalo acted promptly.	Fezeka Ngalo, David Esau, Buziswa Ngqweleka, Bongile Binase, Hlanganisa Dalesile
16 Feb 2026	Minister of Health / NDOH	Ministerial escalation re SANC's non-response.	Emsie Grundling acknowledges (17 Feb). Veronica Taschl requests full correspondence	In progress	NDOH actively requesting documents – engaged.	Minister Aaron Motsoaledi, Veronica Taschl, Emsie Grundling

2026	Commissioner (WC)	Station Commander: unlawful refusal, failure to examine evidence, usurpation of NPA function, intimidation.	wc-provcomm@saps.gov.za successful.	progress	multiple invalid SAPS email addresses.	SAPS), W. Matthee
02 Mar 2026	Schroter Law / TWBH	Van der Walt provides preliminary legal opinion: Matthee does not qualify for PDA protection; constructive dismissal threshold not met.	Matthee provides detailed point-by-point rebuttal. TWBH eventually declines: 'does not meet eligibility criteria'.	Complete	TWBH declined assistance based on Schroter preliminary opinion.	Marco van der Walt, Tsholofelo Mahlangu, W. Matthee
04 Mar 2026	OCJ Court Online	Account activation for eRegistry.	Account activated; password reset confirmed.	Complete	Not applicable.	OCJ Court Online, W. Matthee
05 Mar 2026	Labour Court – OCJ Portal	Urgent application: BCEA s77A – unlawful salary deduction (40.47%); urgent interim relief sought.	Manual filing via Labour Court online portal.	Pending	Not yet deflected.	Farzaana Ismail, Tsakani Soundy, W. Matthee

17 Mar 2026	DEL – UIF Referral	UJF non-compliance at Medscheme.	authority under Section 70(b) – earnings above Minister's threshold. Suggests CCMA.	Complete	passed to CCMA.	Matthew Holmes, Granville Van Wyk, Thinabethu Mazibuko
17 Mar 2026	DEL – UIF Referral	UJF non-compliance at Medscheme.	Matthew Holmes: referred to UIF unit; Granville Van Wyk assigned as contact.	In progress	Passed internally – BCEA team to UIF unit.	Matthew Holmes, Granville Van Wyk, Thinabethu Mazibuko
17 Mar 2026	Board of Directors – Section 162 Notice	Formal service of Section 162 Director Delinquency Application. Respondents: B. Fernandes, N. Munisi, A. Le Roux, G. Van Wyk, LSM Mpumwana. Urgent High Court proceedings Rule 6(12).	No response documented. Board reminded of SENS announcement obligation.	Pending	Ignored – no response. Now litigation respondents.	Bruno Fernandes, Nkateko Munisi, Alice le Roux, Gerald van Wyk, Lebohlang Mpumwana, W. Matthee
19 Mar 2026	SAPS – Delft Police Station	Criminal case registration following Brigadier Naidoo's intervention.	SMS: 'Case registered on 2026-03-19 at DELFT Station. Ref nr CAS 705/3/2026'.	Complete	Case registered after Brigadier Naidoo's intervention.	SAPS Delft, Brigadier Naidoo, W. Matthee

26 Mar 2026	Inexo – Transcription Service	LPC complaints. To Nicola Hanekom (Acting Chief Registrar) and Nosiselo Chwethiso.	Parallel request for audio recording and transcript.	Belinda Botes acknowledges: states 'I'll request the audio and revert'. Then goes on bereavement leave 31 Mar – 6 Apr.	In progress	Engaged – but no backup system when Botes goes on leave.	Belinda Botes, Tina Jorgenson, W. Matthee
01 Apr 2026	Director of Court Operations – R David	Formal complaint: Registrar's office non-response; Inexo breakdown; no backup system; prejudice to self-represented litigant.	Awaiting response.		Pending	Not yet deflected.	R David (Director of Court Operations), W. Matthee
02 Apr 2026	OCJ Court Online	New BCEA case filed. Case No: 2026-077546. Matthee v Afrocentric Health (RF) (PTY) LTD.	Initiating Document approved and filed.		Complete	Not applicable.	OCJ Court Online, W. Matthee

23 Jul 2025	Mapule Mashele. UNLAWFUL SALARY DEDUCTION: 40.47% (R14,811.77) without consultation while between admissions.	Employer invokes 'no work no pay'; manager denies staggering.	BCEA s34(2) violated. Employer held Dr Malan's letter. Bad faith established.
24 Jul 2025	Jones shares AOD policy showing 25% cap; acknowledges cap exists but defends 40.47% deduction.	Jones: 'This is for disability.' Matthee: what is illegal?	Critical admission: employer knew of statutory cap and violated it knowingly.
23 Sep 2025	GENDER IDENTITY DISCRIMINATION: Jones posts 'Wilma' in Teams channel with 'NO FILTERING' in red, immediately after Matthee proposed filtering strategy.	HR investigates. Matthee NOT removed from Jones's team.	EEA s6 violation. HR promises not fulfilled. Matthee left in hostile environment.
25 Sep 2025	Formal 11-page discrimination complaint: requests neutral investigator, suspension of accused, removal from team.	HR promises protection but does not act on removal request.	Protected activity under LRA s187(1)(f). Employer's failure to act = constructive complicity.
29 Sep 2025	RETALIATORY UNPAID LEAVE: Jones loads double punitive unpaid leave on exact same day Matthee files formal complaint.	Reversed by Mashele 1 Oct 2025.	Direct evidence of retaliation. Written proof of discretionary punitive conduct.

12 Dec 2025	RESIGNATION (Constructive Dismissal): employment terminated due to objectively intolerable conditions.	Resignation submitted.	Section 186(1)(e) LRA: objectively intolerable = constructive dismissal. PDA s7 protection applies.
22-23 Dec 2025	Final humanitarian notice to Board and Lead Independent Director (Bruno Fernandes). No response → regulatory escalation triggered.	No response. Mathee proceeds with all regulatory complaints.	Board's silence = final confirmation of institutional failure. Triggers regulatory chain.

4. 30+ Days Institutional Silence	Formal grievance of 2 November 2025 received no response for over 30 days, constituting a structural failure of internal remedy.	02 Nov 2025 grievance → no response; 02 Nov salary deduction dispute → no response; 11 Nov escalation → only acknowledged without resolution	Constructive dismissal ingredient (LRA s186(1)(e)); demonstrates futility of internal remedies for purposes of urgent Labour Court relief.
5. Executive Gatekeeping	GRO Monwabisi Kula actively blocked Board-level escalation, requesting Matthee stop emailing directors and characterising governance escalation as 'impatience'.	17 Dec 2025: 'can I please ask you to stop sending these e-mails'; 18 Dec 2025: gatekeeping email repeated	King IV governance failure; suppression of protected escalation to oversight committees; evidence for Section 162(5)(c) delinquency application.
6. Regulatory 'Passed Buck' Carousel	Every regulatory body except NDOH referred Matthee to another forum, creating a systemic access-to-justice failure.	CMS→HPCSA; HPCSA→SANC; HPCSA→SAPC; JSE→CCMA/Labour Court; SAHRC→CCMA/Labour Court/PP/DEL; DEL (BCEA)→CCMA; DEL (salary)→no action	Systemic failure of regulatory oversight; supports Blowatch challenge to costs order; documents futility argument.
7. Technical Barrier Pattern	Invalid email addresses, full CCMA mailboxes, inaccessible SAPS channels, and failed deliveries systematically delayed or prevented engagement.	CCMA mailbox full (3 occasions Dec 2025); FSCA email invalid; JSE regulation@ blocked; HPCSA email blocked; SAPS multiple invalid addresses;	Constitutes systemic obstruction of access to justice for self-represented litigants; relevant to rescission application and access-to-justice argument under Constitution s34.

DEL – UIF Unit (Granville Van Wyk)	17 Mar 2026 – referred internally by Holmes	Awaiting UIF assessment	HIGH	Follow up directly with Granville Van Wyk; provide uH-ling June 2025 gap extract
SAHRC (Diamini / Yoli)	13 Mar 2026 – forensic payroll update sent	Awaiting feedback	MEDIUM	Follow up; reference Labour Court proceedings and imminent eviction
CMS Commissioner	02 Apr 2026 – auto-reply only	Awaiting substantive response	HIGH	Escalate with HCGS951842 reference; request formal investigation under MSA s47
SAPC (Nkanyane / Ngoveni)	26 Mar 2026 – asked to resend attachment (Ref 5211-7743)	Awaiting further response	MEDIUM	Resend with read-receipt; note historical email timeout pattern
SANC Registrar	16 Feb 2026 – complaint filed	No response (ignored)	MEDIUM	Escalate to National Chief Nursing Officer and Provincial Director of Nursing
JSE (Jowdiene van Schalkwyk)	Post-6 Mar 2026 – JSE engaging company	Awaiting company response	HIGH	Monitor SENS announcements by AfroCentric; provide updated court filing details
B-BBEE Commission (Case 5/02/2026)	17 Mar 2026 – cc'd on Section 162 notice	Awaiting investigation progress	HIGH	Provide Cheryl-Dawn Modern admission (19 Jan 2026) and Certificate of Service as key exhibits

HCBP Policy (employer's own policy)	Employer's own policy treats recovery as deduction subject to 25% cap and mandatory prior consultation – contradicts employer's position.	Filed – Contradicts employer's defence
AOD Policy (25% cap acknowledgement, 24 Jul 2025)	Jones shares policy confirming 25% cap; then defends 40.47% deduction – direct evidence of wilful non-compliance.	Filed – Critical: employer knew of cap
Payslip showing 40.47% deduction (Jul 2025)	Net pay R15,679.29 – confirms deduction exceeded BCEA s34(2) cap. Amount: R14,811.77.	Filed – KEY EXHIBIT for BCEA s77A claim
First Alder roster (25 Jul 2025)	Mathee drafts and implements management-level roster – direct contradiction of 'Care Coordinator' designation; evidences Specialist-level responsibility.	Filed – Management role evidence
'Wilima' Teams message (23 Sep 2025)	Gender identity discrimination incident – screenshot/system evidence required.	Filed – EEA s6 violation evidence
Formal discrimination complaint (25 Sep 2025)	11-page complaint with specific procedural requests – establishes PDA chain and employer's obligations.	Filed – Anchor document for retaliation chronology

uFiling extract showing June 2025 gap	Confirms UIF contributions deducted but not declared; 'No Information' for June 2025.	Filed – Active SAPS case CAS 705/3/2026
SMS confirmation CAS 705/3/2026 transfer (25 Mar 2026)	SAPS SMS confirming case transfer from Delft to Woodstock – active criminal investigation.	Filed – Criminal case confirmation
Audio recording of Delft Station Commander (23 Feb 2026)	Records unlawful refusal to open criminal case; Station Commander usurping NPA prosecutorial function.	Filed – Grounds for IPID complaint
High Court audio recording (Case 2026-066952 – 25 Mar 2026)	Official record required for Rule 42 rescission application and Judicial Conduct Tribunal complaint.	URGENT – Audio not yet obtained; Registrar non-responsive; Inlexso request pending
Director of Court Operations complaint (01 Apr 2026)	Documents Registrar non-response and Inlexso backup system failure – relevant to Rule 42 rescission application.	Filed – Supports access-to-justice argument

of each investigation, disclosure and general public interest, attention to s1(1) and the employer's obligations to respond to the employee's complaint, and the employer's failure to investigate protected disclosures, the Whitewash investigation outcome, and the opposing affidavit's personal attacks further constitute detriment under the Act.

3. Employment Equity Act (EEA), Act 55 of 1998

Applicable Sections: s6(1) (prohibition of unfair discrimination); s6(3) (harassment as unfair discrimination); s11 (burden of proof); s10 (obligation to eliminate unfair discrimination)

Institutional Failure Analysis: The 'Wilma' incident (23 September 2025), Jones's documented pattern of hostile conduct from March 2025, and the employer's whitewash investigation (1 October 2025) constitute unfair discrimination on the grounds of gender identity and sexual orientation under s6(1). The investigation's failure to remove Matthee from the hostile reporting line after a formal complaint — despite HR's promise that he would not be 'required to meet with the manager' — constitutes a failure to take all reasonable steps to prevent harassment under s60. The title suppression (recruited as Specialist, contracted as Coordinator) may also constitute discrimination in the terms and conditions of employment. Under s11, the burden shifts to the employer to show that the discrimination was fair; given the documented conduct, no such justification is available.

4. Basic Conditions of Employment Act (BCEA), Act 75 of 1997

Applicable Sections: s34 (deductions from remuneration); s34(2) (25% cap on deductions); s35 (employer duties re deductions); s77A (civil claim for statutory breach)

Institutional Failure Analysis: The July 2025 salary deduction of 40.47% (R14,811.77) exceeded the statutory 25% cap in s34(2) without consultation, without written consent, and in full knowledge of Matthee's disability. Jones's contemporaneous acknowledgment of the 25% cap (24 July 2025: 'monthly deductions must not be more than 25%') while

OVERALL SYSTEMIC CONCLUSION

The pattern documented across these tables reveals that Wilbur Matthee faced not a single institutional failure but a cascading, interlocking series of structural obstructions. At the internal level, the employer deployed the full arsenal of corporate deflection: whitewash investigations, institutional silence, gatekeeping, retaliatory conduct, document misrepresentation, and bad faith litigation. At the regulatory level, every external body except NDOH deflected to another forum, creating an access-to-justice vacuum. At the judicial administration level, OCJ portal rejections, Registrar silence, and the Inlexso backup system failure compounded the prejudice to a self-represented litigant navigating unprecedented governance terrain without legal aid.

The whistleblower protection framework of the PDA exists precisely for cases such as this: where a person with direct clinical and governance knowledge identifies a material risk, attempts to escalate it through every available channel, and is systematically punished for doing so. The documented causal chain — disclosure → detriment → institutional silence → regulatory carousel → judicial obstruction — constitutes a case study in the structural vulnerabilities that the PDA, LRA, EEA, and Constitution were designed to address.

The active SAPS criminal investigation (CAS 705/3/2026), the pending Labour Court BCEA claim (Case 2026-077546), the B-BBEE Commission investigation (Case 5/02/2026), the intended Rule 42 rescission application in High Court Case 2026-066952, and the ongoing JSE SENS disclosure review represent the remaining institutional pathways through which the record documented in these tables may achieve its intended purpose: accountability, remedy, and systemic correction.

ANNEXURE

"AA"

CPD programme developed by Applicant and presented to management (April 2025); evidence of independent authorship and management rejection; evidence of subsequent implementation without attribution (October 2025) — in support of Part H (Misappropriation of Intellectual Property)

Date: April 2025 / October 2025

*Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Labour Court, Cape Town | Statement of Claim (Constructive Dismissal)*

FW: CPD, SANC Regulation no: 1569, EMguidance courses, ect.

1 message

Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>
To: Mary-Ann Jones <Mary-AnnJ@medscheme.co.za>

Tue, 23 Sept 2025 at 09:55

Hello Mary-Ann

Please find attached pdf sanc regulation no1569,

Kind regards

From: Wilbur Matthee
Sent: Tuesday, April 22, 2025 11:32 AM
To: Mary-Ann Jones <Mary-AnnJ@medscheme.co.za>
Subject: CPD, SANC Regulation no: 1569, EMguidance courses, ect.

Dear Mary- Ann,

I hope this mail finds you well. As per our brief conversation this morning, we discussed the following:

- GEMS upskilling content.

The process that is required for approval of content, especially content regarding upskilling information.

- SANC CPD policy.

According to the Nursing Act 2005, every practitioner registered with SANC is to undertake CPD activities. A minimum of 15 CPD points will be required each year for the renewal of the Annual Practicing Certificate (APC). The CPD System will be implemented in a phased manner, starting with practicing midwives working in selected districts across all nine provinces. A wide range of formal and informal learning activities/events will generate CPD points. Each practitioner is expected to accrue 15 CPD points annually.

- Online courses via EM guidance that are free some paid content, the provision of a certificate of completion if marks are above 85% but a certificate for results above 95%.

Current range of courses available some paid and free.



- Current Flu vaccine (another course: **Overcoming barriers to influenza vaccination in pregnancy and role of OBGYN in advocating for vaccination**)

In this CPD activity, you will gain insights into influenza vaccination in high-risk groups, with a focus on cardiovascular disease and stroke.

Earn 2 CPD points by watching the provided video and reviewing the accompanying information card on: Influenza vaccination after myocardial infarction and then proceed to answer the 15 multiple-choice questions to successfully complete this CPD activity.

Learning objectives:

Upon completion of this CPD activity, you will:

Understand influenza and its associated risks in the context of patients with cardiovascular comorbidities.

- Adult Vaccination Guideline:

Such as pneumococcal immunization and others required for adults as per our preventative benefit

- Current Managing type 2 Diabetes: A Proactive approach to T2D

In this CPD activity, distinguished endocrinologists Dr. Z. Bayat and Dr. R. Daya explore proactive strategies for managing Type 2 Diabetes, providing valuable insights and practical approaches.

:Learning Objective

Proactive approaches to managing Type 2 Diabetes.

Identifying and addressing key factors that impact diabetes management.

Implementing practical strategies to enhance patient outcomes in Type 2 Diabetes management.

Thank you very much for your assistance and consideration.

Best regards



Sr. Wilbur W. Matthee

(Nurse Specialist PGrad HSM, Btech Gen Pn (Midw,Psych,Comm))

Care coordinator: GEMS Active Disease Risk Management

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